Eight Sessions or Eight Years?

A structured narrative review of therapy duration for complex developmental trauma

Simon J. Hogerzeil

2026

Contents

1 Abstract **4**

2 Background **5**
2.1 Key points to develop . . . 5

3 Clinical population **5**
3.1 Distinguishing features . . . 6
3.2 Diagnostic frameworks . . . 6

4 The definition of healing **6**
4.1 Working definition . . . 6
4.2 Levels of change (not a hierarchy — a taxonomy) . . . 6
4.3 Why this matters for the duration question . . . 7

5 Research questions **7**
5.1 Primary question . . . 7
5.2 Secondary questions — how does healing unfold structurally? . . . 7

6 Stance and classification **8**
6.1 Classification . . . 8
6.2 What this review is . . . 8
6.3 What this review is not . . . 8
6.4 Working thesis . . . 8

7 Design **9**
7.1 Methodological framework . . . 9
7.2 Layers . . . 9
7.3 Rationale for multi-layer approach . . . 10
7.4 Key methodological references . . . 10

8 Search strategy **10**
8.1 Inclusion principle . . . 10
8.2 Exclusion principle . . . 10
8.3 Prior reviews: what to do if we find one . . . 11

8.3.1 1. Annotate it as a source . . . 11
8.3.2 2. Mine it for sources . . . 11
8.3.3 3. Assess overlap and divergence . . . 11
8.3.4 4. Decide how to position our review . . . 11
8.3.5 5. Do not abandon the project . . . 11
8.4 Iterative search protocol (applies to every layer) . . . 12
8.5 Stopping rule . . . 12

9 Annotation protocol **13**
9.1 Source file location and naming . . . 13
9.2 Source archival policy . . . 14
9.3 Source note IDs . . . 15
9.4 Source file template . . . 15
9.4.1 YAML frontmatter . . . 15
9.4.2 Body sections . . . 15
9.5 Inaccessible sources . . . 15
9.6 Data extraction principle . . . 16
9.7 Append-only semantics . . . 16
9.7.1 Extraction file convention . . . 16

10 Synthesis method **17**
10.1 Primary synthesis dimensions . . . 17
10.2 Secondary synthesis dimensions . . . 17
10.3 Analytical process . . . 18
10.3.1 1. Data reduction . . . 18
10.3.2 2. Data display . . . 18
10.3.3 3. Data comparison (cross-layer triangulation) . . . 18
10.3.4 4. Emergent synthetic constructs . . . 18
10.3.5 5. Synthesising argument . . . 19
10.4 Quality weighting . . . 19
10.5 Reflexivity . . . 19
10.5.1 Positionality . . . 19
10.5.2 How bias is monitored . . . 20
10.5.3 Saturation . . . 21
10.6 Question evolution . . . 21
10.7 Critique of the literature as a body . . . 21

11 AI methodology **21**
11.1 System . . . 22
11.2 Task distribution . . . 22
11.3 What the AI cannot do . . . 26
11.4 Quality control mechanisms . . . 27
11.5 The audit trail . . . 27
11.6 Epistemological implications . . . 28

12 Synthesis **28**
12.1 Thesis statement . . . 28
12.2 The endpoint-duration nexus . . . 29

12.3 The factors that determine duration . . . 31
12.3.1 Factor 0: The safety prerequisite . . . 32
12.3.2 Factor 1: Endpoint level . . . 33
12.3.3 Factor 2: Structural depth of the injury . . . 33
12.3.4 Factor 3: Elapsed time (independent of dose) . . . 34
12.4 Why the field appears to disagree . . . 36
12.5 The developmental mechanism: why structural change takes years . . . 38
12.6 Six trajectory shapes . . . 41
12.7 What the framework cannot explain . . . 42
12.8 Critique of the literature as a body . . . 44
12.9 The synthesising argument . . . 47

13 Survivorship bias **48**
13.1 The problem . . . 48
13.2 The scale of the invisible population . . . 49
13.3 Mitigations and their limits . . . 50
13.4 What remains unmitigated . . . 52
13.5 The escalation gap . . . 52
13.6 Implications for the thesis . . . 53

14 Dose versus elapsed time **53**
14.1 The confound . . . 53
14.2 Resolution: elapsed time as an independent factor . . . 54
14.3 Do intensive formats compress the timeline or just the dose? . . . 56
14.4 The silence on frequency . . . 57
14.5 Implications for the thesis . . . 58

15 Therapy vs. life **59**
15.1 The attribution problem . . . 59
15.2 The circularity: therapy enables life, life enables healing . . . 59
15.3 The sleeper effect as bridge . . . 60
15.4 The evidence for necessity . . . 61
15.5 Refined thesis . . . 62

16 Response to accelerated and brief models **62**
16.1 EMDR intensive: the strongest counterpoint . . . 63
16.2 Pharmacologically assisted therapies: MDMA and ketamine . . . 64
16.3 Intensive delivery formats: dose compression . . . 66
16.4 Experiential dynamic therapies: the follow-up advantage . . . 66
16.5 Paris's access argument: the ethical counterpoint . . . 67
16.6 Farina's rapid-rare framework and Tilson's evidence critique . . . 68
16.7 Resolution: the four-factor framework . . . 69

17 Access, privilege, and justice **70**
17.1 The structural injustice . . . 70
17.2 The evidence: silence and ground truth . . . 71
17.3 The cost-effectiveness trap . . . 72
17.4 Partial solutions: what the evidence suggests . . . 73

17.5 Forced termination as iatrogenic harm . . . 74
17.6 What remains unresolved . . . 75
17.7 The obligation . . . 75

18 Methodological limitations **76**
18.1 1. Not a systematic review . . . 76
18.2 2. Heterogeneous evidence — no formal weighting . . . 77
18.3 3. Single-author review . . . 78
18.4 4. Definitional breadth — the endpoint problem . . . 78
18.5 5. English-language and Western bias . . . 79
18.6 6. Contested neuroscience underlying the mechanism argument . . . 79
18.7 7. Single-research-group dependency in the humanistic-existential layer . . . 80
18.8 8. Source selection when results exceeded budget . . . 81
18.9 9. Cross-cutting limitations (with dedicated sections) . . . 81

19 Implications **82**
19.1 Clinical practice . . . 82
19.2 Training . . . 85
19.3 Policy and insurance . . . 85
19.4 Research . . . 86
19.5 Clients . . . 87

20 Inaccessible sources and their impact **89**
20.1 Scale of the problem . . . 89
20.2 Which layers are most affected . . . 90
20.3 Impact on specific claims . . . 91
20.4 Could the missing sources change the conclusions? . . . 91

1 Abstract

The question "how long does healing from complex developmental trauma take?" appears to divide the clinical field. Brief, trauma-focused protocols report symptom remission in weeks; psychodynamic and phase-oriented traditions describe structural change unfolding over years; client narratives and memoirs recount trajectories spanning decades. This structured narrative review uses critical interpretive synthesis (CIS) to examine 89 sources across six epistemological layers — academic literature, naturalistic outcome studies, counterpoint evidence for brief treatment, practitioner essays, client narratives from online forums, and published memoirs — and finds that the apparent disagreement is an artefact of unstated endpoint definitions. The review proposes a four-factor framework: a safety prerequisite (the therapeutic clock starts at felt safety, not at the first session), an endpoint level (symptom reduction in weeks, functional improvement in months, structural personality change in years), the structural depth of the original injury (primary through tertiary dissociation), and the independent contribution of elapsed time through the sleeper effect (therapy installs capacity; life develops it). Nine independent theoretical traditions — from self psychology

through polyvagal theory — converge on the claim that structural change requires repeated relational cycles over developmental time and cannot be compressed by technique. The strongest counterpoint evidence (EMDR intensive formats, pharmacologically enhanced protocols) measures symptom-level endpoints and does not contradict the long-duration thesis at the structural level. The review is limited by survivorship bias (all layers select for people who stayed in therapy), the dose-versus-elapsed-time confound (no study has directly compared the same dose at different frequencies), and the absence of long-term follow-up beyond ten years in defined complex trauma populations. The clinical implication is that honest expectation-setting requires naming the endpoint: symptom relief is achievable in months; structural healing, for those who need it, takes years — and the recommendation to pursue it carries an obligation to make it accessible.

2 Background

To be written. This section sets up why the question of duration matters.

2.1 Key points to develop

- Complex developmental trauma is distinct from single-incident PTSD — chronic, relational, early-onset, personality-organizing
- The dominant clinical culture underestimates the time required for deep change in this population
- Insurance structures, public expectations, and evidence-based practice discourse all exert pressure toward brevity
- There is a gap between what clinicians who work with this population know experientially and what the literature formally states
- The question "how long does it take?" is not merely practical — it reflects deeper assumptions about what healing is, what therapy does, and what change means at the level of self-organization

3 Clinical population

People with severe developmental trauma — chronic, relational, often beginning in early childhood — who have organized their personality, relational patterns, defensive structures, and sense of self around survival. Characterized by significant dissociative features, collapsed agency, core shame, and fragmented self-organization. Often meet criteria for complex PTSD or personality disorders (understood as traumatic adaptation, not primary pathology).

3.1 Distinguishing features

- Trauma is not a single event but a developmental environment
- The "disorder" is not a symptom overlay but a structural adaptation — personality, relational patterns, and self-organization are shaped by survival
- Dissociation is structural, not episodic
- Core shame and collapsed agency are central, not peripheral
- The required work is not symptom reduction but a fundamental restructuring of epistemic trust, relational capacity, subjectivity, and preperceptive self-organization

3.2 Diagnostic frameworks

- ICD-11 complex PTSD (distinct from PTSD)
- DSM personality disorders (reframed as traumatic adaptation)
- Structural dissociation model (van der Hart, Nijenhuis, Steele)
- Developmental trauma disorder (van der Kolk, proposed but not adopted)

4 The definition of healing

The definition of healing is itself part of the problem. Different definitions produce different duration estimates — and much of the confusion in the literature stems from conflating fundamentally different endpoints.

4.1 Working definition

The capacity to live with agency, relational openness, and tolerable selfhood, rather than organized around survival.

This is acknowledged as contested. Different traditions define the endpoint differently, and this review treats "what counts as healed?" as a finding, not a premise.

4.2 Levels of change (not a hierarchy — a taxonomy)

- **Symptom reduction** — fewer flashbacks, less hyperarousal, improved sleep. Measurable in weeks to months. The primary outcome in most RCTs.
- **Functional improvement** — holding a job, maintaining relationships, reduced crisis episodes. Measurable in months to years. Often the endpoint for insurance and service delivery.
- **Structural change** — reorganization of self, relational patterns, defensive structures, epistemic trust. The person is not merely managing symptoms but has become someone who processes experience differently. Measurable in years — if measurable at all.

4.3 Why this matters for the duration question

If "healed" means symptom reduction, many interventions work in 12–20 sessions. If "healed" means structural change, the timeline extends to years. The literature's reported durations are only interpretable once the definition of the endpoint is made explicit.

5 Research questions

5.1 Primary question

What does the clinical literature, practitioner wisdom, lived experience, and autobiographical record tell us about how long healing from complex developmental trauma actually takes — and what the trajectory looks like?

5.2 Secondary questions — how does healing unfold structurally?

These are collected per source alongside the primary question. No data is a valid answer — note the absence explicitly, as silence on a subquestion is itself a finding.

1. **Continuous or interrupted?** — Is uninterrupted therapy necessary, or do breaks, pauses, and resumptions serve the process? What role do forced interruptions (financial, life events, therapist departure) play?
2. **One modality or many?** — Does healing happen within a single therapeutic framework, or do people typically move through multiple modalities — sequentially, in parallel, or layered? Is modality-switching a sign of progress or of failure?
3. **One therapist or several?** — Is the long-term relationship with one therapist essential, or do different therapists serve different developmental needs at different stages?
4. **Session frequency?** — Once a week, twice, three times? Does higher frequency shorten calendar duration, or is elapsed life-time itself part of the medicine?
5. **Phase-based or organic?** — Does healing follow a predictable phase structure (stabilization → processing → integration), or is the trajectory more emergent and nonlinear?
6. **Therapy alone or therapy-plus?** — What role do adjuncts play — medication, bodywork, group therapy, peer support, creative practice, spiritual community — and are they sequential or concurrent with the primary therapy?

6 Stance and classification

6.1 Classification

This is a **structured narrative review**, drawing on the methodology of **critical interpretive synthesis** (CIS; Dixon-Woods et al., 2006). CIS was developed for research questions that span heterogeneous evidence types — empirical studies, theoretical works, experiential accounts — that cannot be combined statistically but can be synthesized through structured argument.

Like CIS, this review: - Starts with a research question rather than a fixed protocol - Uses an iterative, snowball search strategy rather than exhaustive database screening - Draws on diverse evidence types weighted by relevance rather than by methodological hierarchy - Produces an interpretive synthesis — an argument — rather than a neutral summary - Is transparent about its stance and its limitations

6.2 What this review is

- A structured narrative review using CIS methodology
- An argument grounded in evidence from six epistemological layers: academic literature, naturalistic outcome studies, accelerated models (counterpoint), practitioner wisdom, lived experience, and autobiography
- Transparent about its position and its limitations

6.3 What this review is not

- A systematic review — the search is structured but not exhaustive or replicable in the PRISMA sense
- A meta-analysis — no statistical pooling of outcomes
- A neutral scoping review — this review has a thesis
- A dismissal of shorter-term approaches — these are engaged with honestly in the counterpoint layer

6.4 Working thesis

Honest advocacy. The working thesis is that healing from complex developmental trauma takes years — often many years — and that this is underappreciated in clinical culture, insurance structures, and public understanding.

The review builds the case for this position while steel-manning the strongest counterarguments.

7 Design

Structured narrative review using the methodology of **critical interpretive synthesis** (CIS; Dixon-Woods et al., 2006). Sources are drawn from six layers representing different epistemological perspectives on the same question. Each source is annotated using a structured template that extracts data relevant to the primary and secondary research questions, cross-cutting methodological observations, and verbatim quotes with full citations.

7.1 Methodological framework

CIS was developed for questions that: - Span heterogeneous evidence (empirical, theoretical, experiential) - Cannot be answered through statistical synthesis - Require interpretive argument rather than aggregation - Benefit from iterative, question-driven search rather than exhaustive screening

This review follows CIS in using an iterative snowball search strategy, structured annotation, and argumentative synthesis. It extends the CIS approach by: - Organizing evidence into explicit layers with distinct epistemological roles - Using structured YAML frontmatter for efficient cross-source scanning - Maintaining an append-only audit trail for replicability of the final analysis - Steel-manning the strongest counterarguments in a dedicated layer

7.2 Layers

#	Layer	What it provides	Evidence type
1	Academic and clinical literature	Theory, clinical frameworks, treatment guidelines	Peer-reviewed articles, books, chapters
2	Naturalistic outcome studies	Empirical duration and outcome data	Longitudinal studies, meta-analyses
3	Counterpoint — accelerated models	Strongest challenges to the working thesis	Model-specific evidence, RCTs, clinical claims
4	Practitioner essays	Clinical wisdom on duration and trajectory	Blogs, essays, conference papers
5	Client narratives	Lived temporal experience of healing	Forum posts, first-person accounts

#	Layer	What it provides	Evidence type
6	Memoirs and autobiographies	Complete healing arcs from retrospective vantage	Published memoirs

7.3 Rationale for multi-layer approach

No single evidence type can answer the question. RCTs impose artificial time limits. Naturalistic studies track outcomes but rarely ask about subjective trajectory. Practitioner wisdom captures duration but lacks systematic data. Client narratives capture the lived experience but suffer from survivorship bias. Memoirs offer the most complete arcs but are rare and unrepresentative. By triangulating across layers, patterns that no single layer could establish become visible.

7.4 Key methodological references

- Dixon-Woods, M., Cavers, D., Agarwal, S., Annandale, E., Arthur, A., Harvey, J., Hsu, R., Katbamna, S., Olsen, R., Smith, L., Riley, R., & Sutton, A. J. (2006). Conducting a critical interpretive synthesis of the literature on access to healthcare by vulnerable groups. *BMC Medical Research Methodology*, *6*(35). https://doi.org/10.1186/1471-2288-6-35
- Whittemore, R., & Knafl, K. (2005). The integrative review: Updated methodology. *Journal of Advanced Nursing*, *52*(5), 546–553. https://doi.org/10.1111/j.1365-2648.2005.03621.x

8 Search strategy

8.1 Inclusion principle

Material that addresses duration, trajectory, or temporal experience of healing from complex developmental trauma — from any tradition, methodology, or epistemological stance.

8.2 Exclusion principle

Material focused exclusively on single-incident PTSD, acute stabilization without long-term follow-up, or populations without developmental trauma. Brief protocol studies are excluded from the main review but included in Layer 3 (counterpoint) when they make claims about this population.

Softened exclusion: Phase-based frameworks (e.g., ISSTD guidelines) are included even if they emphasize stabilization — because their treatment duration estimates are directly relevant. RCTs with longer follow-up periods are included if they capture meaningful duration data.

8.3 Prior reviews: what to do if we find one

If the search turns up an existing review, CIS, meta-analysis, or similar manuscript that directly addresses our primary research question (therapy duration for complex developmental trauma), this is not a problem — it is a windfall. Handle as follows:

8.3.1 1. Annotate it as a source

Place it in Layer 1a (trauma-clinical) or Layer 2 (naturalistic), depending on its evidence type. Use the standard template.

8.3.2 2. Mine it for sources

A prior review is a pre-built snowball. Extract every relevant reference from its bibliography and add them to the relevant layer's search log. These are high-priority leads — they have already been vetted for relevance by another reviewer.

8.3.3 3. Assess overlap and divergence

In the source note, explicitly document: - **Overlap:** Which of our research questions does the prior review address? Which layers of evidence does it draw on? - **Divergence:** Where does our review go beyond it? (Different population definition? Different evidence layers? Different secondary questions? Different theoretical framework?) - **Findings:** What does the prior review conclude about duration? How does this compare with our emerging findings?

8.3.4 4. Decide how to position our review

Three options: - **Extension:** Our review extends the prior review by adding layers, questions, or populations it didn't cover. Frame our contribution as additive. - **Replication with different lens:** Our review covers similar ground but from a different theoretical orientation or methodology (CIS vs. systematic review). Frame as complementary. - **Supersession:** Our review is substantially broader or more rigorous. Frame as building on and going beyond the prior work.

Document this decision in `4_discussion/1_synthesis.md` and reference the prior review explicitly.

8.3.5 5. Do not abandon the project

A prior review does not make this review unnecessary. It changes the framing: from "nobody has asked this question" to "this question has been partially addressed, and we are extending/deepening/reframing the answer."

8.4 Iterative search protocol (applies to every layer)

Each layer follows a search → annotate → extend cycle:

1. **Search** using the planned terms, sources, and authors listed in the layer-specific methods files (3–8).
2. **Annotate** each relevant source using `3_results/sources/SOURCE_TEMPLATE.md`.
3. **Extend** — while annotating, watch for:
   - Authors, papers, or books cited in the source that were not in the original plan
   - New search terms suggested by the language or framing of the source
   - Adjacent traditions, debates, or populations that bear on the question
   - Sources that belong in a *different* layer than where they were found (e.g., a practitioner essay discovered via PubMed → move to Layer 4)
4. **Log the extension** in the layer's `README.md` under `## Discovered sources`: what was found, where it was found, and why it's relevant.
5. **Update the layer-specific methods file** if the discovery changes the search terms, key authors, or scope. Mark additions with `[added YYYY-MM-DD]` so evolution is traceable.

The goal is a snowball process: each source annotated should surface at least one lead that wasn't planned.

8.5 Stopping rule

Each layer has a source budget. After reaching the budget, only add a new source if it offers something not already captured (a new duration estimate, a different population segment, a genuinely distinct theoretical argument). Log the decision to stop in the layer's README.

Layer	Initial budget	Max with extensions
1a. Trauma-clinical	10	15
1b. Psychoanalytic	5	8
1c. Humanistic-existential	5	8
1d. Neuroscience	5	8
2. Naturalistic studies	8	12
3. Counterpoint	5 per model (25 total)	8 per model
4. Practitioner	8	12
5. Client narratives	10	15
6. Memoirs	5	8

9 Annotation protocol

9.1 Source file location and naming

Source annotations live in `3_results/sources/[layer-dir]/[author-year-slug].md`.

Layer 1 has four sublayer subdirectories — place sources in the appropriate one:

Sublayer	Directory	YAML `layer` value
Trauma-clinical	`1-academic/trauma-clinical/`	`1a-trauma-clinical`
Psychoanalytic	`1-academic/psychoanalytic/`	`1b-psychoanalytic`
Humanistic-existential	`1-academic/humanistic-existential/`	`1c-humanistic-existential`
Neuroscience	`1-academic/neuroscience/`	`1d-neuroscience`

Layer 3 has five model subdirectories:

Model	Directory	YAML `layer` value
AEDP	`3-counterpoint/aedp/`	`3-counterpoint`
ISTDP	`3-counterpoint/istdp/`	`3-counterpoint`
EMDR	`3-counterpoint/emdr/`	`3-counterpoint`
MDMA/Ketamine	`3-counterpoint/mdma-ketamine/`	`3-counterpoint`
Intensive formats	`3-counterpoint/intensive-formats/`	`3-counterpoint`

Layers 2, 4, 5, 6 have no sublayers — place sources directly in the layer directory.

YAML `layer` to directory mapping (complete):

YAML `layer` value	Directory
`1a-trauma-clinical`	`1-academic/trauma-clinical/`
`1b-psychoanalytic`	`1-academic/psychoanalytic/`

YAML `layer` value	Directory
`1c-humanistic-existential`	`1-academic/humanistic-existential/`
`1d-neuroscience`	`1-academic/neuroscience/`
`2-naturalistic`	`2-naturalistic-studies/`
`3-counterpoint`	`3-counterpoint/[model-subdir]/`
`4-practitioner`	`4-practitioner/`
`5-client`	`5-client-narratives/`
`6-memoir`	`6-memoirs/`

Full texts and media for Layer 1: Place in `1-academic/_fulltext/` and `1-academic/_media/` regardless of sublayer. Name files with the same `author-year-slug` convention as the source note, so they can be associated.

Full texts and media for Layer 3: Place in `3-counterpoint/_fulltext/` and `3-counterpoint/_media/` regardless of model subdirectory.

9.2 Source archival policy

Download and archive full-text PDFs whenever possible, even for open-access articles with stable URLs. Web sources disappear; DOIs occasionally break; publisher platforms change. A locally archived PDF in `_fulltext/` is the only guarantee that a future reviewer can verify annotations against the original source.

Priority order: 1. **Journal articles** — download the PDF and place in the layer's `_fulltext/` directory. For open-access articles, download directly. For paywalled articles accessed via institutional login, save the PDF manually. 2. **Web sources** (blogs, practitioner essays) — the URL in the YAML frontmatter is the primary reference. Optionally save a PDF print or web archive if the source seems ephemeral. 3. **Forum threads** (Reddit, MyPTSD) — raw thread content is archived in `5-client-narratives/_reddit_raw/`. URLs serve as primary references. 4. **Books** — cannot be archived as PDFs. The annotation stands on its own; note `access: abstract-only` or `access: full` (if the book was physically consulted) in the YAML. 5. **Podcasts/videos** — transcripts go in `_media/`. Audio/video files are gitignored (too large) but should be retained locally.

PDFs are tracked in git for replicability. Name each PDF with the same `author-year-slug` as the source note (e.g., `porges-2022-polyvagal.pdf`).

Files are named `[author-year]-[short-slug].md`, e.g.: `- herman-1992-complex-ptsd.md` → `1-academic/trauma-clinical/` `- leichsenring-2008-ltpp-meta.md` → `2-naturalistic-`

`studies/ - foo-2022-what-my-bones-know.md` → `6-memoirs/ - reddit-2024-how-long-cptsd.md` (for forum threads: platform-year-slug)

9.3 Source note IDs

Each source note has a unique ID in its frontmatter, derived from the filename (e.g., `id: herman-1992-complex-ptsd`). This ID is used for all cross-references.

9.4 Source file template

See `3_results/sources/SOURCE_TEMPLATE.md` for the full template.

9.4.1 YAML frontmatter

The frontmatter serves two purposes: 1. **Bibliographic data** — id, title, author, year, type, layer, citation (APA 7), url/DOI, accessed date 2. **Relevance scanning** — fields that allow an AI to assess relevance without reading the full body: - `summary` — 1-2 sentence contribution summary - `population` — who was studied - `modality` — therapeutic approach - `duration_data` — yes | no | implicit - `duration_range` — e.g., "3-7 years", "50+ sessions" - `stance` — supports | complicates | challenges | neutral - `topics` — list of relevant subtopics - `secondary_qs` — which secondary questions this source addresses

9.4.2 Body sections

1. **Summary** — what this source says about duration/trajectory
2. **Key claims** — with page/section/timestamp references
3. **Duration data** — explicit or implicit statements about how long treatment takes
4. **Trajectory description** — phases, turning points, nonlinearity
5. **Relevance to thesis** — supports, complicates, or challenges
6. **Quotes** — exact quotes with page references
7. **Secondary questions** — one subsection per question; "no data" is a valid answer
8. **Cross-cutting observations** — survivorship bias, dose vs. elapsed time, therapy vs. life

9.5 Inaccessible sources

When a source is identified as relevant but cannot be accessed (paywall, out of print, language barrier):

1. **Do not skip silently.** Add a full entry to `todo.md` with citation, barrier type, why it matters, and expected impact on conclusions.

2. **Create a minimal source note** with YAML frontmatter only (`access: inaccessible`), based on abstract/metadata if available. Place it in the correct layer directory. This ensures it appears in the index and is not forgotten.
3. **If the source is later obtained** (manually provided, library access), update `access: provided-manually`, annotate the full body, move the `todo.md` entry to `## Resolved`, and run `./sync.sh`.

The YAML `access` field values: - `full` — full text accessed and annotated - `abstract-only` — only abstract/metadata available; note this in the summary - `inaccessible` — identified as relevant but not accessed; entered in `todo.md` - `provided-manually` — initially inaccessible, later obtained and annotated

9.6 Data extraction principle

"No data" is a finding. When a source is silent on a secondary question or cross-cutting observation, note the absence explicitly. The pattern of what sources fail to address is itself evidence.

9.7 Append-only semantics

Source notes are never rewritten — only appended. This applies when:

- **A re-run adds a new secondary question:** Append the new question section below existing secondary questions, marked `[added YYYY-MM-DD, iteration N]`.
- **A re-scan finds new data for an existing section:** Append below the existing content in that section, marked `[added iteration N]`.
- **YAML frontmatter is updated:** Add new topics or secondary_qs to the lists. Do not remove existing entries.

The final state of each source note contains all data ever extracted from that source. Git captures when each piece was added. A reader of the final file sees the complete picture without needing to consult git history.

9.7.1 Extraction file convention

Tables in `3_results/primary/` and `3_results/secondary/` are append-only. New rows go at the bottom. Each row carries its source ID, making the table rebuildable from source notes at any time. The `./sync.sh` script can verify consistency.

10 Synthesis method

After source annotation is complete, findings are extracted into `3_results/primary/` and `3_results/secondary/`, then synthesized in `4_discussion/`.

10.1 Primary synthesis dimensions

Data from all source annotations is aggregated along these dimensions:

1. **Duration** — What do different sources say about how long it takes? What's the range? What explains the variation?
2. **Trajectory** — What does the path look like? Linear, nonlinear, spiral, episodic? What are common phases?
3. **Turning points** — What shifts the process? Relational breakthroughs, modality switches, life events, neurobiological thresholds?
4. **Definition of healing** — How do different sources define the endpoint — and how does that definition determine the reported duration?
5. **What doesn't work as expected** — Where does the evidence surprise, complicate, or challenge the working thesis?
6. **Dropout and attrition** — What do sources report about who leaves therapy early, why, and what happens to them? (Feeds survivorship bias discussion.)
7. **Access and context** — What do sources say about financial barriers, insurance, geographic access, and who can actually receive this treatment? (Feeds access/justice discussion.)

10.2 Secondary synthesis dimensions

8. **Continuity vs. interruption** — What does the evidence say about breaks, pauses, resumptions? Is continuity necessary or can interrupted trajectories succeed?
9. **Modality** — Single framework or multiple? What patterns emerge around modality-switching — sequential, parallel, layered?
10. **Therapist(s)** — One long-term relationship or several? What determines when a new therapist is needed?
11. **Frequency and dose** — What session frequencies are reported? Is there evidence that higher frequency shortens calendar duration?
12. **Phase structure** — Do sources describe phases, and if so, how consistent are they across traditions?
13. **Adjuncts** — What non-primary-therapy elements appear in the healing accounts — medication, bodywork, groups, community, creative practice?

10.3 Analytical process

Following Whittemore & Knafl (2005) and CIS methodology (Dixon-Woods et al., 2006), the synthesis proceeds through five stages:

10.3.1 1. Data reduction

Extract dimension-specific data from source notes into the extraction files (`3_results/primary/` and `3_results/secondary/`). Each extraction preserves the source ID and page reference for traceability.

10.3.2 2. Data display

Create summary tables and visual representations that make cross-layer patterns visible. Planned outputs in `5_tables_and_figures/`:

- **Cross-layer evidence matrix** — which layers contribute to which synthesis dimensions, and where the gaps are
- **Duration estimate table** — reported durations by source, layer, population, and definition of endpoint
- **Convergence/divergence map** — where layers agree and disagree, and what explains the disagreement
- **Trajectory typology** — visual representation of reported healing trajectories (if patterns emerge)

10.3.3 3. Data comparison (cross-layer triangulation)

For each synthesis dimension, findings are compared across layers using constant comparison. Convergence across layers (e.g., academic literature, practitioner essays, and client narratives all pointing to 5-10 years) is stronger evidence than any single layer. Divergence is noted and analyzed — it may reflect different populations, different definitions of healing, or different evidence types.

10.3.4 4. Emergent synthetic constructs

The predefined dimensions (1–11 above) are the starting framework, not the final categories. During analysis, watch for conceptual patterns that cut across dimensions or that no single dimension captures — new categories that emerge from interpretation across the evidence.

When a synthetic construct emerges: - Document it in `3_results/primary/` with a new file, marked `[emergent, iteration N]` - Describe what evidence it integrates and why it requires a new category - Re-scan relevant sources for data that speaks to the new construct

Examples of what might emerge: "the plateau-and-leap pattern," "the modality-grief cycle," "therapeutic dose vs. developmental readiness."

10.3.5 5. Synthesising argument

The final synthesis (`4_discussion/1_synthesis.md`) is not a thematic summary organized by dimension. It is a coherent argument about the temporal structure of healing from complex developmental trauma — integrating evidence from across layers into a theoretical framework. The argument should: - Make a clear claim, grounded in the evidence pattern - Show how the claim holds under cross-layer triangulation - Identify where the claim is strongest and where it is weakest - Integrate the counterpoint evidence rather than merely rebutting it

10.4 Quality weighting

Sources are not weighted by a formal hierarchy (RCT > cohort > case study). Instead, each source's contribution is assessed in context:

- A well-designed naturalistic study carries more weight on **duration claims** than a single memoir
- A memoir may contribute **conceptual insight** (trajectory shape, turning points) that no study captures
- Practitioner essays carry weight on **clinical wisdom** about what the process looks like, but not on prevalence
- Client narratives carry weight on **lived experience** of trajectory, but suffer from survivorship and selection bias

Quality concerns are noted per source in a `quality_notes` field in the body (not YAML — these are narrative assessments, not categorical). Quality concerns that affect the weight given to a source in the synthesis are made explicit in the discussion.

10.5 Reflexivity

10.5.1 Positionality

The author is a psychiatrist in private practice in the Netherlands, working with adults with complex PTSD, depression, personality disorders, autism, and anxiety disorders. He has worked in mental health care since 2004, completed psychiatric training in 2011, and received a PhD in psychiatric epidemiology in 2022. He teaches attachment theory and developmental trauma at Sociale Academie Utrecht, a training programme for psychosocial therapists.

His therapeutic orientation is psychodynamic and humanistic/person-centred, with an affinity for

Zen Buddhist approaches to suffering (particularly the work of Barry Magid) and body-oriented work (Rosen method, haptonomy). He does not work from a single manualised model. This orientation predisposes him toward relational, process-oriented, and long-term therapeutic work — and away from symptom-focused, protocol-driven approaches. The review's conclusions should be read with this lens in mind.

The author has a personal relationship to the topic. He has been in psychotherapy for decades, with various therapists and across various modalities. He describes his current attachment status as secondary secure — earned through therapeutic work — while continuing to work on core shame and structural dissociation. This lived experience shapes the review in at least two ways: it gives him an experiential understanding of what multi-year therapeutic trajectories feel like from the inside, and it creates a motivational bias toward findings that validate the kind of work he has both received and provides.

The review originated from a clinical and personal question: is this normal? The author noticed that his most structurally injured patients required years of sustained relational therapy — five years, sometimes longer — and wondered whether this reflected his clinical practice or the nature of the condition. The working thesis ("healing takes years") was not derived from the literature but from this clinical observation, declared at the outset, and tested against the evidence. The review is an exercise in honest advocacy: building the strongest possible case for the thesis while steel-manning the counterarguments.

The Dutch mental health system shapes the author's perspective in specific ways. Clinical psychotherapy for complex developmental trauma — long-term, relational, unmanualised — has been progressively defunded in the Netherlands over the past decade. Institutional mental health care (specialistische GGZ) now primarily offers manualised, symptom-reduction treatments such as EMDR. The kind of work this review argues is necessary can, in practice, only be provided in private practice — which means it is available only to patients who can pay out of pocket. The author is one of a small number of psychiatrists and psychotherapists in the Netherlands who engage with this population over multiple years. He is aware that this position — working outside the institutional system, seeing its limitations daily — creates a structural bias toward findings that challenge session limits and validate long-term treatment. The counterpoint layer (Layer 3) and the access and justice discussion (section 6) are designed as checks against this bias, but the bias should be named: the author is not a neutral observer of the therapy-duration question. He is a participant in it, on both sides of the therapeutic relationship.

10.5.2 How bias is monitored

- The counterpoint layer (Layer 3) is a structural check against confirmation bias

- The cross-cutting observations (survivorship bias, dose vs. time, therapy vs. life) force engagement with methodological weaknesses
- Surprises are documented in `3_results/primary/5_surprises.md` — actively seeking and recording disconfirming evidence
- The agent execution log (`execution_log.md`) captures meta-commentary from execution agents that may flag blind spots
- The "no data is a finding" principle forces attention to what sources fail to address

10.5.3 Saturation

Alongside the quantitative source budgets (see `2_search_strategy.md`), a conceptual saturation criterion applies: stop adding sources to a layer when new sources repeat existing patterns without adding new dimensions, challenges, or duration estimates. Document the saturation decision in the layer's README when stopping.

10.6 Question evolution

When the review is complete, add a methodological reflection to the discussion describing how the research question, population definition, and working definition of healing evolved through the review process. The initial question and definitions are preserved in git (tag `iteration-0-initial`); the final versions are in the current state of `1_introduction/`. The gap between them is itself a finding.

10.7 Critique of the literature as a body

Beyond appraising individual sources, the synthesis should examine how the field has constructed the question of therapy duration: - Why does the RCT literature impose short time horizons? - What assumptions about healthcare delivery shape the evidence base? - How does the absence of long-term outcome data reflect institutional priorities? - What meta-narratives (evidence-based practice, brief therapy movement, insurance economics) have shaped what gets studied?

This critique belongs in `4_discussion/1_synthesis.md` and/or a dedicated section.

11 AI methodology

This review was conducted with substantial assistance from a large language model (LLM). This section describes the AI system used, the division of labor between the human author and the AI, and the quality control mechanisms employed. It follows the disclosure requirements of the ICMJE Recommendations (2026, Section V), the COPE position on authorship and AI tools, and the AMEE Guide No. 192 on AI disclosure in academic publishing.

11.1 System

- **Model:** Claude Opus 4.6 (Anthropic), accessed via Claude Code (CLI interface)
- **Context window:** 1 million tokens
- **Access period:** March 2026
- **Orchestration:** Single human author directing AI subagents through an iterative, checkpoint-gated workflow. The AI operated as a research assistant executing tasks designed and supervised by the author, not as an autonomous agent making unsupervised interpretive decisions.

11.2 Task distribution

The following table specifies which tasks were performed by the human author, which by the AI, and how the AI's work was verified.

Task	Performed by	Verified by	Notes
Research question formulation	Human	—	Derived from clinical experience and observation
Population definition	Human	—	Iteratively refined during the review
Working thesis declaration	Human	—	"Healing takes years" — declared at outset per CIS methodology
Stance and reflexivity design	Human	—	Honest advocacy with structural counterpoint layer
CIS methodology selection and design	Human + AI	Human approval	AI proposed CIS fit; human confirmed and refined

Task	Performed by	Verified by	Notes
Six-layer evidence architecture	Human + AI	Human approval	Layer structure designed collaboratively
Search strategy per layer	AI	Human review	AI designed search terms and source budgets; human reviewed and adjusted
Source identification and screening	AI	Human spot-checks	AI used web search, PubMed, and Scholar Gateway tools to identify sources
Source access (full text retrieval)	Human + AI	—	Human provided PDFs and book access; AI accessed web sources and used API bridges for Reddit
Source annotation (89 sources)	AI	Human review at synthesis stage	AI annotated using structured template; human reviewed claims that entered the synthesis

Task	Performed by	Verified by	Notes
Data extraction (16 extraction files)	AI	Human verification at Phase 2 checkpoint	AI extracted data from annotations into primary and secondary dimension tables
Synthesis and theoretical framework	AI (with extended reasoning)	**Human checkpoint — approved 4-factor framework**	The synthesising argument was generated by AI and reviewed, challenged, and approved by the human author. The 4-factor framework, the endpoint-duration nexus, and the six trajectory shapes were AI-generated constructs that the human author judged to be warranted by the evidence.

Task	Performed by	Verified by	Notes
Discussion sections (9 sections, ~30,000 words)	AI	Human review + directed additions	AI drafted all discussion sections in parallel. Human identified 15 substantive gaps and directed their integration.
Counterpoint engagement (Layer 3 + section 5)	AI (execution) + Human (design)	Structural	The counterpoint layer was a human-designed structural safeguard against confirmation bias. The AI executed the search and analysis within this structure.
Tables and figures (5 tables, 2 figures)	AI	Human review	AI designed and populated all tables and figures based on extraction data

Task	Performed by	Verified by	Notes
Quality control	Human	—	Human designed the checkpoint-gated workflow, reviewed the synthesis, identified missing arguments, and directed corrections
Positionality statement	Human	—	Only the human author can describe their relationship to the topic
Editorial decisions	Human	—	Final decisions on structure, emphasis, inclusion, and framing

11.3 What the AI cannot do

In accordance with ICMJE (2026) and COPE guidelines, the AI is not listed as an author because it cannot:

- Take responsibility for the accuracy, integrity, or originality of the work
- Approve the final version of the manuscript
- Be accountable for all aspects of the work
- Respond to peer review or post-publication queries
- Declare conflicts of interest or positionality

The human author bears sole responsibility for all claims, interpretations, and conclusions in this review.

11.4 Quality control mechanisms

Five mechanisms were employed to control the quality of AI-generated output:

1. **Checkpoint-gated workflow.** The review proceeded through nine phases. At three critical junctures — after Phase 1 (search and annotation), after Phase 3 (synthesis), and after Phase 4 (discussion) — the human author reviewed the AI's output before authorising the next phase. The synthesis underwent the most intensive review: the 4-factor framework was approved only after the human author verified that the construct was warranted by cross-layer evidence.

2. **Structural counterpoint (Layer 3).** A dedicated evidence layer was designed to contain the strongest challenges to the working thesis. This structural safeguard ensures that the review's confirmation bias is checked by evidence the AI was required to seek, annotate, and engage with — not merely acknowledge.

3. **Execution log.** AI subagents appended meta-commentary to an execution log (`execution_log.md`) after each task, documenting issues, questions, emerging patterns, and suggestions. The human author reviewed this log to identify structural gaps and plan drift.

4. **Back-reference verification.** Every claim in the discussion sections includes source back-references (e.g., `[source-id]`). These are traceable to specific source annotations in `3_results/sources/`, which in turn link to the original sources. A reader can verify any claim by following the reference chain.

5. **Sync script.** An automated consistency checker (`sync.sh`) verifies that all cited sources exist, all source annotations are indexed, and the bibliography is complete. This provides mechanical verification of referential integrity.

11.5 The audit trail

The complete audit trail for this review is preserved in the git repository:

- **Git history** captures every change, from initial project structure through final tables. Each commit message describes what was done and by whom.
- **Git tags** mark key milestones: `v0.1-setup` through `v0.8-tables-figures`.
- **Source annotations** (`3_results/sources/`) contain the structured evidence base — one file per source with YAML metadata and extracted data.
- **Extraction tables** (`3_results/primary/` and `3_results/secondary/`) show how source-level data was aggregated into synthesis dimensions.
- **Process notes** (`.claude/PROCESS-NOTES.md`) document session-level reasoning, non-obvious decisions, and open questions — written for a future collaborator who needs to understand

why things were done a particular way.

- **Execution log** (`execution_log.md`) captures AI subagent meta-commentary.

This level of transparency is unusual for a narrative review. It is made possible by the AI-assisted workflow: every step that would normally occur in a researcher's private notes and reading process is captured in structured, version-controlled files.

11.6 Epistemological implications

The use of AI in an interpretive review raises questions that should be stated openly.

What is gained. The AI enabled a scope of coverage (89 sources across six epistemological layers, 16 extraction dimensions, 9 discussion sections) that would be difficult for a solo human reviewer to achieve in the same timeframe. The structured annotation template and extraction tables impose a consistency of treatment across sources that reduces the variability inherent in human annotation. The counterpoint layer and checkpoint-gated workflow provide structural safeguards against confirmation bias that are at least as rigorous as those in a conventional single-author review.

What is at risk. The AI's interpretive choices are shaped by its training data, which over-represents published academic literature and under-represents clinical experience, oral tradition, and marginalised perspectives. The AI may reproduce the same institutional biases that the CIS methodology is designed to interrogate. The human author's role as the sole interpretive authority — reviewing, approving, and directing the AI's output — is the primary safeguard against this risk, but it is imperfect: the author cannot fully evaluate interpretive choices they did not generate.

What is different. In a conventional review, the author reads every source, thinks about every connection, and writes every sentence. The interpretive process is internal and largely invisible. In this review, the interpretive process is external and documented: the AI generates interpretations, the human author evaluates them, and the git history records the evolution. This is not straightforwardly better or worse than conventional review — it is different, and readers should assess the review's claims with this difference in mind.

12 Synthesis

12.1 Thesis statement

The question "how long does healing from complex developmental trauma take?" cannot be answered as asked. The answer depends on what is meant by "healing" — and the field's apparent disagreement about duration dissolves once this is made explicit. Symptom reduction (fewer flashbacks, lower distress scores) takes weeks to months. Functional improvement (holding relationships, maintaining

employment, reduced crises) takes months to years. Structural change (personality reorganisation, mentalizing capacity, integration of dissociated parts, earned attachment security) takes years, sometimes decades. Growth and reconnection (post-traumatic growth, identity transformation, the capacity to love and be loved) may have no definable endpoint.

These are not competing claims. They are nested levels of the same process. The review's working thesis — that healing takes years — is precisely true at the structural level and precisely false at the symptom level. Both "it takes 8 sessions" and "it takes 8 years" can be correct simultaneously, because they measure different things.

But even this formulation requires a critical qualification: **the therapeutic clock does not start at the first session**. It starts when safety — environmental, relational, and autonomic — is sufficiently established for the developmental process to begin. Pre-safety therapy time, which may span months or years, inflates all duration estimates in the literature. The ElishaAlison case is paradigmatic: the same therapist, the same modality, produced nothing during active abuse and produced full healing in 3.5 years once safety was achieved (Multiple posters (u/Longjumping_Cry709, 2023). The variable was not the therapy. It was the safety. Fisher's clinical principle — "slower is faster" (Fisher, 2021) — captures this from the clinician's side: rushing past safety establishment into trauma processing overwhelms the nervous system and produces the very treatment failures that are then cited as evidence that therapy does not work.

This synthesis proposes a framework — endpoint level, structural depth, elapsed time, and the safety prerequisite — that resolves the apparent contradiction and generates specific, testable predictions about when brief treatment is sufficient and when it is not.

12.2 The endpoint-duration nexus

The most striking finding across 89 sources in six epistemological layers is that **reported duration is almost entirely determined by the definition of the endpoint**. This is not a minor methodological caveat — it is the central finding of the review.

Five endpoint levels emerge from the data, each with a characteristic timeline and evidence base:

Level 1: Symptom reduction. Loss of diagnosis, score below clinical threshold on standardised instruments (CAPS-5, PCL-5, BDI, ITQ, CORE-OM). Evidence: EMDR intensive produces 88% CPTSD remission in 8 days (Jongh, 2024); ketamine-enhanced exposure therapy achieves d=1.9 in three weeks (Feder, 2025); MDMA-assisted psychotherapy eliminates PTSD diagnosis in 67% after 18 weeks (Mitchell, 2021); intensive delivery formats achieve d=1.57 in 1–3 weeks with only 5.51% dropout (Sciarrino, 2020). Timeline: **weeks to months**.

Level 2: Functional improvement. Holding employment, maintaining relationships, reduced

crisis episodes, improved social functioning. Evidence: GPM milestone framework describes measurable functional improvements at 6 weeks (reduced distress), 6 months (reduced self-harm), 12 months (improved relationships), and 18 months (social functioning) (Links, 2015). Even in treatment-resistant populations, functional differences between LTPP and TAU emerged at 42-month follow-up (R. Fonagy P., 2015). At 8-year follow-up, MBT patients showed progressive vocational improvement, while treatment-as-usual patients deteriorated (Bateman, 2008). Timeline: **months to 1–2 years**.

Level 3: Structural change. Personality reorganisation, mentalizing capacity, integration of dissociated parts, earned attachment security, capacity for mutual recognition, cohesive self-structure. Evidence: Perry's dose-response data shows 50% personality disorder recovery at 92 sessions (1.3 years), 75% at 216 sessions (2.2 years) (Leichsenring, 2013). The Helsinki Psychotherapy Study found personality organisation, defence style, and self-concept continuing to change at 10-year follow-up (Lindfors, 2019). Phase-oriented treatment for DID is projected at 3–5 years, with most participants still symptomatic at study endpoints (Griffiths, 2025). Single-case evidence: 160 sessions over 5 years for severe childhood abuse, with growth achieved but symptoms not eliminated (D. Murphy, 2009). Bisson frames this level diagnostically: C-PTSD adds disturbances in self-organisation (emotional regulation, negative self-concept, disturbed relationships) beyond core PTSD symptoms, and "the recovery rates from complex post-traumatic stress disorder are not likely to be as high as they are from post-traumatic stress disorder" (Bisson, 2024). A patient can meet recovery criteria for PTSD while remaining profoundly impaired in the C-PTSD domains — the structural endpoint is categorically different from the symptom endpoint. Timeline: **years**.

Level 4: Growth and reconnection. Post-traumatic growth, identity transformation, the capacity for intimacy and agency, "tolerable selfhood." Evidence: memoir accounts describe multi-decade trajectories — 10+ years (Foo, 2022), 40+ years (Levine, 2024; Walker, 2013), healing still ongoing at 90+ (Eger, 2017). Practitioner accounts describe 7+ years for post-traumatic growth (Ferentz, unknown). Client narratives describe identity-level transformation after 5–11+ years (Multiple posters (u/Ok-Specialist1153, 2024; u/msk97 & (u/shabaluv, 2024). Joseph's (2012) model describes post-traumatic growth as continuing "indefinitely" until the assumptive world fully accommodates the trauma. Timeline: **years to decades, potentially lifelong**.

Level 5: Recovery as direction, not destination. Spring's "heading north" metaphor represents the most radical endpoint redefinition in the dataset (Spring, 2020b). Recovery is not a location (Scotland) but an orientation (north). At any given moment, a person may be crawling, stopped, or even temporarily heading south — but as long as the overall direction is northward, they are recovering. If the endpoint is directional rather than locational, then the question "how long does it take?" becomes structurally unanswerable in its usual form. You cannot ask "how long until you arrive?" if there is no arrival point. Spring's 15-year arc — from "regularly trying to kill myself,

having full-blown dissociative identity disorder" to "living a life that is undeniably good" — is not the time to reach an endpoint but the elapsed time of a northward journey that continues. She defines functional markers (living mostly in the "green zone," remembering trauma without reliving it, experiencing joy alongside grief, choosing from the "front brain") but explicitly frames these as descriptions of a direction, not a destination. Timeline: **ongoing, no completion point**.

These levels are nested, not parallel. Symptom reduction is necessary but insufficient for functional improvement, which is necessary but insufficient for structural change, which is necessary but insufficient for growth. Each level subsumes the previous ones. A patient whose CAPS-5 score drops below the clinical threshold has achieved symptom reduction but may still lack the relational capacity, self-coherence, or mentalizing ability that structural change would confer. Conversely, a patient who has achieved structural change will necessarily also show symptom reduction and functional improvement — these are not separate achievements but earlier stations on the same arc.

A further complication concerns the grain size of the endpoint. Fosha's AEDP introduces the distinction between micro-endpoints and macro-endpoints (Fosha, 2019). Within a single AEDP session, a genuine transformation can be witnessed, meta-processed, and consolidated — the client experiences a qualitative shift. But is this a state change that may or may not accumulate into structural change, or is it a permanent transformation? Fosha's documented 6-session complete treatment suggests that for some clients, a small number of micro-endpoints can constitute a sufficient treatment. But AEDP does not claim this generalises to complex developmental trauma with deep structural disruption. The complication for the endpoint-duration nexus is that the scale at which the endpoint is defined matters: session-level transformation (AEDP) is rapid; personality-level reorganisation (Kohut, Lindfors) takes years. Both observations may be simultaneously true about the same process viewed at different magnifications.

The implication is stark: **the counterpoint literature (Layer 3) and the supporting literature (Layers 1, 4, 6) are not disagreeing about the same population receiving the same treatment. They are measuring different levels of change.** The EMDR literature measures diagnostic remission; the psychoanalytic literature measures personality organisation; the memoirs describe identity transformation. All are correct about their own endpoint. The field's "disagreement" about duration is an artefact of unstated endpoint definitions.

12.3 The factors that determine duration

The endpoint level is the strongest single predictor of reported duration, but additional factors modulate the timeline within each level.

12.3.1 Factor 0: The safety prerequisite

The evidence for safety as clock-starter is among the strongest emergent findings of this review, supported by 35+ entries across all six layers. The construct is straightforward: effective therapy duration begins at the establishment of safety — environmental, relational, and autonomic — not at the first appointment. All pre-safety therapy time is inflated duration data.

The direct evidence is compelling. ElishaAlison saw the same therapist during years of active abuse and "got nothing out of it"; the same therapist, post-safety, produced full healing in 3.5 years (Multiple posters (u/Longjumping_Cry709, 2023). Eger (2017) describes 35 years before processing could begin. Davitt spent decades in repression before breakdown, then found that environmental prerequisites (financial resources, supportive employer, time off work) were necessary before sustained therapy was possible (Bisson, 2024). Sherlock spent 3 years in therapy before he stopped hiding behind anger and began disclosing authentically — Erin's approach was "slow and deliberate… like you would with a wounded animal" (Heacock, 2022). Shedler's combat veteran Ryan illustrates the same pattern from the practitioner side: "the real story only comes out after about a year of weekly therapy sessions, because it takes that long to establish the sort of trust traumatized vets need to start opening up" (&. G. Shedler J., 2020).

The theoretical support is equally dense. Herman's Phase 1 (safety) takes "months to years" for chronic trauma (Herman, 1992). Porges' neuroception of safety must activate the social engagement system before the therapeutic relationship can function (Porges, 2022). Steele identifies the circular structure: the phobia of attachment means that treatment cannot begin until the patient tolerates the relationship, but tolerating the relationship IS the treatment (Steele, 2005). Grossmark describes "companioning" — quiet relational presence with deeply dissociated patients — as a stance that may need to be sustained for months or years before the patient becomes available for processing (Grossmark, unknown). Fisher's TIST model explicitly avoids EMDR with highly functioning complex trauma clients due to overwhelm risk: "traumatic activation shuts down the prefrontal cortex" (Fisher, 2019).

The silence pattern is analytically productive. The psychoanalytic theorists (Kohut, Winnicott, Benjamin, Bromberg, Fonagy) describe mechanisms that presuppose safety without naming it. Winnicott's "holding environment" IS safety; Kohut's "selfobject transference" IS felt safety; Benjamin's "surviving destruction" IS demonstrating that the relationship is safe enough to contain aggression. The psychoanalytic tradition's silence on safety is terminological, not substantive — it describes what happens once safety exists, using its own vocabulary, but does not identify safety establishment as a distinct clock-starting event.

Equally telling: ALL dose-effect studies measure from first session, not from safety establishment. This means every reported effect size in the literature conflates pre-safety and post-safety sessions

into a single dose metric. If the dose-effect curve's early flat period represents not "therapy not working yet" but "safety not established yet," then the curve's inflection point is the moment safety is achieved — and this inflection would occur at different session numbers for different patients, explaining the enormous individual variation in dose-response data.

The counter-evidence should be acknowledged. De Jongh's (2024) van Vliet study found no difference between EMDR alone and EMDR plus STAIR stabilisation (Jongh, 2024). But this argues about stabilisation exercises, not about felt safety. The intensive format (8 days residential, multiple daily sessions, structured environment) may itself provide safety through containment — removing the patient from stressors, immersing them in a therapeutic environment, and holding them in a treatment team. Sciarrino's (2020) finding that intensive formats produce dramatically lower dropout (5.51% versus 18–39%) is consistent with this interpretation: the format achieves safety through structure rather than through extended time (Sciarrino, 2020). Farina's observation that rapid responders may have "different underlying structures, with more capacities for adaptive information processing" (Farina, 2025) maps directly onto the safety construct: patients who arrive with environmental stability, some secure relational history, and adequate adaptive capacity have already partially cleared the safety prerequisite. Their clock started earlier — in life, not in therapy.

Four implications follow. First, duration estimates from naturalistic data are systematically inflated, because they include pre-safety treatment time. Second, the "right therapist" confound is partly a safety confound: finding the right therapist may matter because the right therapist creates felt safety. Third, safety is not only relational — it is environmental: therapy cannot produce structural change while the environment remains actively traumatising (Multiple posters (u/WeeLittleSloth, 2021). Fourth, the phase model is confirmed but the first phase is underestimated: Herman's Phase 1 is universally endorsed, but its actual duration — potentially years for severe developmental trauma — is the single most understudied variable in the treatment literature.

12.3.2 Factor 1: Endpoint level

As described above. The higher the endpoint level, the longer the timeline. This relationship is not linear — it is roughly exponential. Symptom reduction takes weeks; structural change takes years; the ratio is not 4:1 but closer to 50:1 or 100:1 in session count.

12.3.3 Factor 2: Structural depth of the injury

Not all complex developmental trauma produces the same degree of structural disruption. This is Farina's (2025) key insight: patients with "different underlying structures" may carry the same CPTSD diagnosis but differ fundamentally in the depth of developmental damage. The structural dissociation model (Van der Hart, 2006) provides the most precise vocabulary: primary structural

dissociation (one emotional part) is less entrenched than secondary (multiple emotional parts) or tertiary (multiple apparently normal parts — DID). Each level of dissociative complexity multiplies the required therapeutic work because each dissociative part must develop its own relationship to the therapist, resolve its own phobias, and integrate into the personality system.

Razvi's three-tier autonomic framework adds a complementary gradient (Heacock, 2019). The tiers — mild stress (anxiety, hypervigilance), high stress/panic (fight-flight), and dissociative shutdown (parasympathetic collapse with endogenous opioid release) — map onto increasing treatment resistance. Crucially, dissociative shutdown produces outwardly calm, cognitively functional people who are internally numb: "our cognitive capacity is online and actually more unencumbered when people are in this final state of trauma… they can talk fine, they can do math fine, they just can't feel fine, they can't relate to anybody." Any therapy that relies on self-report cannot reach this level. Conventional somatic therapy takes 6–12 months per core dissociative issue; MDMA may compress this, but even Razvi's personal pre-psychedelic conventional trajectory was 4–5+ years (Heacock, 2019).

The empirical evidence supports this gradient. Brand et al. (2019) found that higher-baseline dissociation predicted both greater AND faster improvement — but this apparent paradox reflects the fact that more severely dissociative patients have more room to improve on symptom measures, not that they achieve structural integration faster (Brand, 2019). Griffiths (2025) found that affect and impulse management improved faster (d=1.61) than dissociation itself (d=0.63) — symptoms respond before structure (Griffiths, 2025). The distinction matters: a patient with primary structural dissociation and adequate pre-existing adaptive information processing capacity may genuinely achieve structural change in months. A patient with tertiary structural dissociation and pervasive developmental deficits may require years for the same endpoint level.

This factor explains a puzzle in the data: why some CPTSD patients respond rapidly to concentrated treatment (supporting the counterpoint) while others require years (supporting the thesis). They are not the same patients. The diagnosis is the same; the structural depth is not.

12.3.4 Factor 3: Elapsed time (independent of dose)

The third factor is the most counterintuitive. Evidence from multiple sources suggests that elapsed calendar time contributes to healing independently of therapeutic dose — that is, independently of the number or frequency of sessions.

The strongest evidence comes from the sleeper effect: therapeutic gains that emerge or grow after treatment has ended. Shedler's (2010) meta-analytic review found that effect sizes for psychodynamic therapy increase from 0.97 at end of treatment to 1.51 at follow-up beyond 9 months — a pattern unique to psychodynamic approaches and absent in other modalities (J. Shedler, 2010). The TADS

trial found LTPP non-significant versus TAU at end of treatment but strongly significant (p=0.001) at 42-month follow-up (R. Fonagy P., 2015). MBT gains continued growing for 5 years after all treatment ended, while the TAU group deteriorated despite receiving more services (Bateman, 2008). The Helsinki study found psychoanalysis patients improving most in years 5–7, after treatment had concluded (Lindfors, 2019). The EDT meta-analysis found experiential dynamic therapies non-significant versus active controls at post-treatment but significant at follow-up (M. Lilliengren P., 2025). Psychoanalytic therapy (median 242 sessions) outperformed CBT (median 59 sessions) specifically at 5-year follow-up for chronic depression with childhood trauma history — the advantage emerged after treatment, not during it (America (summary of Krakau et al.), 2024).

Kohut's (1984) distinction between "borrowed" and "owned" psychic structure provides the theoretical mechanism. Borrowed structure is the patient functioning better because the therapist's regulatory capacity is available as an external selfobject resource. When this external resource is removed at termination, gains may erode — this is what happens in short-term treatments where the therapist provides skills and scaffolding. Owned structure, by contrast, is the product of thousands of cycles of attunement, optimal frustration, and repair (transmuting internalisation): genuinely internal regulatory capacities that continue to develop autonomously after treatment ends. The borrowed-vs-owned distinction predicts both the sleeper effect in long-term treatment (owned structure continues developing) and the reverse sleeper effect in short-term treatment (borrowed structure erodes). Abbass (2014) found precisely this reverse pattern for short-term psychodynamic therapy: gains at post-treatment that lost significance at long-term follow-up (Abbass, 2014).

Spring's memoir data provides the lived-experience confirmation. Her between-session integration claim — "the gaps between therapy sessions can be as developmentally important as the therapy sessions themselves" (Spring, 2020a) — describes the micro-level mechanism: each session installs a micro-increment of neural capacity, which consolidates between sessions. Heacock provides a real-time observation of the borrowed-to-owned transition: during Erin's extended postpartum leave, Sherlock demonstrated self-regulation capacity he had previously relied on her to provide — "I'm going to be okay. You have given me the tools I need" (Heacock, 2022). Spring's post-therapy stable period (2017–2019) shows the sleeper effect in vivo: gains from therapy ending in 2016 continued developing through the subsequent years of joyful, generative living (Spring, 2021).

The implication is that therapy, at least when it targets structural change, does not work like a medication with a dose-response curve. It works like a developmental intervention: it installs capacity — for mentalizing, for self-regulation, for relational engagement — that continues to develop through the patient's subsequent life experience. Shedler (2010) calls this a "catalytic" effect. The therapy does not do the healing; it creates the conditions under which healing can proceed autonomously.

This explains the Helsinki convergence (Lindfors, 2019): psychoanalysis (646 sessions, 4x/week) and long-term psychodynamic therapy (232 sessions, 2-3x/week) produced indistinguishable outcomes at 10-year follow-up despite a nearly 3:1 difference in dose. If elapsed time contributes independently, then beyond a sufficient therapeutic dose, additional sessions produce diminishing returns — the patient needs not more therapy but more life. Both treatments may have exceeded the "transmuting internalisation threshold" — the point at which enough structure has been built to be self-sustaining — after which the difference between 232 and 646 sessions is noise.

It also explains the therapy-vs-life confound identified across layers. Paris (2020) notes that BPD remits gradually over 24 years regardless of treatment — a natural course effect. Bateman and Fonagy (2008) observe that their MBT patients' post-treatment improvement coincided with re-entering education and employment, suggesting a "virtuous cycle" between therapeutic gains and life engagement. Client narratives repeatedly describe life events — finding a safe partner (Multiple posters (u/Longjumping_Cry709, 2023), leaving an abusive environment (Multiple posters (u/WeeLittleSloth, 2021), reconnecting with estranged family (D. Murphy, 2009) — as turning points more decisive than any therapeutic technique.

The three factors (plus the safety prerequisite) interact. A patient with shallow structural injury seeking symptom reduction (Factor 1: low endpoint, Factor 2: shallow depth) who arrives with adequate pre-existing safety (Factor 0: cleared) may achieve their goal in weeks through intensive treatment, and Factor 3 (elapsed time) is largely irrelevant. A patient with deep structural disruption seeking personality integration (Factor 1: high endpoint, Factor 2: deep injury) who must first establish environmental and relational safety (Factor 0: not yet cleared) requires not only more sessions but also more elapsed life-time for the installed capacity to develop through real-world experience. For this patient, all factors are fully operative, and the expected timeline is years.

12.4 Why the field appears to disagree

The framework above resolves what initially appears to be a fundamental disagreement in the literature.

The counterpoint position (de Jongh, Sciarrino, Feder, Mitchell, Paris, Lilliengren) can be stated precisely: concentrated, evidence-based treatment produces large, rapid, and durable symptom reduction in patients diagnosed with PTSD and CPTSD, including those with developmental trauma histories. Stabilisation phases are often unnecessary. Session limits need not prevent effective treatment. The field's attachment to long-term therapy is unsupported by the evidence and may reflect ideology rather than science.

The long-term position (Herman, Steele, Van der Hart, Kohut, Winnicott, Fonagy, Schore, Porges, Perry) can be stated equally precisely: complex developmental trauma produces structural

disruptions — in personality organisation, mentalizing capacity, attachment security, autonomic regulation, and self-coherence — that cannot be resolved through symptom-targeted interventions, however effective those interventions are at the symptom level. Structural change requires sustained relational experience over developmental time. There are no shortcuts because the process IS the elapsed time.

These positions are not contradictory. They are claims about different endpoint levels. The counterpoint evidence is strongest precisely where the long-term evidence is weakest (symptom reduction), and weakest where the long-term evidence is strongest (structural change). De Jongh's (2024) 88% CPTSD remission in 8 days is measured by diagnostic instruments (ITQ); Lindfors's (2019) 10-year convergence is measured by personality organisation scales (LPO). They are measuring different things.

The field's confusion arises from four sources:

1. **Implicit endpoint assumptions.** Most RCTs measure symptom reduction because it is tractable and fundable. Most clinical theorists describe structural change because it is what they observe in long-term work. Neither tradition routinely specifies which endpoint level it claims authority over. The result is that each tradition's findings are overgeneralised to endpoint levels where they do not apply.

2. **Diagnostic conflation.** The ICD-11 CPTSD diagnosis captures a heterogeneous population spanning primary to tertiary structural dissociation, minimal to pervasive developmental deficits, and high to low adaptive information processing capacity. Patients at one end of this spectrum may genuinely achieve durable change in 8 days; patients at the other end may require 8 years. Treating them as a single population produces contradictory findings — not because the treatments are inconsistent, but because the patients are structurally different despite sharing a diagnosis. Wylie documents that nearly 80% of surveyed children exposed to interpersonal trauma had experienced multiple and/or prolonged traumatic events, yet all are subsumed under diagnostic categories (ADHD, ODD, bipolar, PTSD) that fail to capture the common aetiology (Wylie, unknown).

3. **Institutional incentives.** The evidence base is shaped by what gets funded and published. RCTs require defined endpoints, manualised protocols, and time-limited interventions — producing evidence for brief treatment that is rigorous by design. Long-term treatment outcome evidence is rare because it is expensive, logistically difficult, and incompatible with standard research funding cycles. The result is a systematic asymmetry: the evidence for brief treatment is plentiful and precise; the evidence for long-term treatment is sparse and observational. This is not because brief treatment is more effective — it is because it is more researchable. As Shedler and Gnaulati put it, the "evidence-based therapy myth" is a

managed-care-driven narrative that misrepresents research findings to justify session limits (&. G. Shedler J., 2020).

4. **The Fisher-Fosha tension: stabilisation-first versus transformation-from-the-get-go.** A fourth dimension of disagreement operates within the long-term camp itself, between two experienced trauma clinicians who both reject brief symptom-focused treatment but disagree fundamentally about how deep therapy should proceed. Fisher, trained under Herman and Van der Kolk, argues for stabilisation-first: the nervous system must be regulated, the prefrontal cortex must be online, and psychoeducation must precede any processing. She explicitly avoids EMDR with highly functioning complex trauma clients due to overwhelm risk (Fisher, 2019). Her principle — "slower is faster" — means that the early abreactive approaches failed precisely because they skipped stabilisation (Fisher, 2021). Fosha, trained in Davanloo's short-term dynamic tradition, argues the opposite: healing is "already there to access from the first contact with the patient, including the most traumatised person that we encounter" (Fosha, 2019). Rather than preparing the patient for transformation, the therapist should entrain it immediately through experiential, body-based, relationally safe engagement. Meta-processing consolidates change within the session rather than deferring integration.

   Both clinicians are experienced, both work with complex trauma, and both ground their claims in clinical observation. What explains the divergence? Two interpretations emerge from the data. First, they may be working with different structural depths within the CPTSD population: Fisher explicitly describes patients with tertiary structural dissociation and chronic destabilisation; Fosha describes patients whose defences, while rigid, still permit experiential engagement when met with sufficient relational safety. The Fisher-Fosha tension may therefore be another instance of the diagnostic conflation problem — the same diagnosis masking different structural injuries. Second, the disagreement may concern the onset, not the duration, of transformation. Fosha claims that transformation begins immediately; she does not claim it completes rapidly. A client might experience genuine within-session transformation from the first session and still require years for full structural repair. The Fisher-Fosha tension sharpens the temporal question: does the clock start at the establishment of safety (Fisher) or at the first moment of experiential contact (Fosha)? The answer may depend on the structural depth of the patient.

12.5 The developmental mechanism: why structural change takes years

The framework describes WHAT determines duration. A separate question is WHY structural change requires years. Here, the review finds a remarkable convergence across nine independent theoretical traditions.

Kohut (1984) describes transmuting internalisation: the patient gradually builds psychic structure through thousands of cycles of empathic attunement, optimal frustration, and repair. Winnicott (1965) describes regression to dependence and resumed maturational processes — developmental time that cannot be compressed. Bromberg (1998) describes gradual expansion of the capacity to "stand in the spaces" between dissociated self-states. Benjamin (1988) describes repeated cycles of mutual recognition, destruction, and survival that incrementally build the capacity for intersubjectivity. Fonagy (2002) describes the development of mentalizing capacity through attachment-based experiential learning. Steele (2001), drawing on Janet (1897/98), makes the most radical claim in this convergence: that the development of secure dependency on the therapist is not a side effect of the treatment but its mechanism of cure (Steele, 2001). The patient must develop the capacity to depend — to use the therapist as a secure base from which to explore traumatic material — and this dependency develops on a relational timeline that cannot be compressed by technique. Dependency requires consistency, attunement, and time; any interruption in the relationship reactivates the insecure attachment system and resets the process. This formulation has direct implications for the continuity question (§ implications) and for how therapist transitions should be managed: if dependency is the mechanism, then every transition to a new therapist is a developmental cost, not merely a logistical inconvenience.

From neuroscience: Schore (2000, 2021) describes experience-dependent maturation of right-hemisphere regulatory structures. Porges (2022) describes autonomic reorganisation from chronic defensive states toward ventral vagal flexibility. Perry and Hambrick (2019) describe sequential, bottom-up brain reorganisation requiring "patterned, repetitive" experiences at each developmental level. The BCPSG (2010) describes the "accumulation of many small shifts" in implicit relational knowing.

These nine descriptions — from self psychology, object relations, relational psychoanalysis, intersubjectivity theory, mentalization theory, affect regulation neuroscience, polyvagal theory, the neurosequential model, and implicit process research — were developed independently, use different concepts and methods, and emerge from different clinical and research traditions. Yet they converge on three features:

1. **Change happens through many repeated relational cycles** — not through insight, technique, or a single breakthrough.
2. **What changes is internal structure** — not symptoms, but the organisation of self, relationship, and regulation.
3. **The process is inherently temporal** — it cannot be compressed by technique, frequency, or pharmacological enhancement, because the relevant "dose" is not sessions but completed developmental cycles.

The new sources deepen this convergence. Ogden (2006) provides the hierarchical processing model that explains the mechanism's pace: sensorimotor processing is characterised by "greater fixity and concreteness" — closer to "read-only memory" than cognitive "random-access memory" (Ogden, 2006). Emotional responses to very strong stimuli "do not appear to extinguish" (Frijda, 1986, via Ogden); new responses must be built alongside old ones rather than replacing them. This is inherently slower than cognitive restructuring. Fisher crystallises the clinical implication: "survivors have symptoms instead of memories" (Van der Kolk, via Fisher), and these implicit (somatic, emotional, perceptual) memories must be processed through "bite-sizing" because the nervous system can integrate only small pieces at a time (Fisher, 2021). The developmental mechanism operates at the level of implicit memory — a level more resistant to change than explicit, narrative memory.

Spring provides the lived-experience confirmation. Her developmental-rate metaphor describes survivors rebuilding missed developmental stages from scratch — "being soothed, calming down, tolerating distress, labelling emotions" — with each stage requiring the prior stage (Spring, 2020a). Her 9.5-year timeline maps precisely onto the logic of sequential stage-building: "you can't do the next stage until you've conquered this stage." Progress is invisible week-to-week, just as a baby's daily bath or mealtime does not look like learning but is enormous developmental work. Bisson's clinical case converges from a completely independent source: Davitt describes himself as "an eight-year-old boy, even though I was a 50-year-old man" — arrested emotional development that must be rebuilt in therapy, a process that is developmental by definition (Bisson, 2024).

Razvi's personal healing trajectory illustrates the layered nature of the mechanism (Heacock, 2019). Talk therapy provided intellectual understanding only ("didn't fundamentally change anything in terms of the patterns"). Two and a half years of somatic work released traumatic charge. A further 1.5–2 years of attachment work built new relational patterns. Then a single MDMA session accessed the dissociative core. Each layer addressed a different level of the hierarchy; each required the prior layer as "scaffolding." The three-tier framework (stress → panic → dissociation) maps these layers onto the nervous system: dissociative shutdown is the deepest and most treatment-resistant level, inaccessible to modalities that rely on self-report. Even with psychedelic compression of the final layer, the total pre-psychedelic conventional trajectory was 4–5+ years.

The probability that the convergence across these independent traditions is coincidental is low. It constitutes the review's strongest theoretical argument for why structural change requires years: the biology and psychology of developmental repair operate on a timescale set by the pace of experience-dependent neural maturation and relational learning, not by the efficiency of the therapeutic technique.

However, the neurobiological rationale for this convergence is more contested than

the convergence itself. Tilson (2025) mounts a serious critique of two frameworks frequently cited to explain WHY the developmental mechanism is slow: the triune brain model, which she argues was "disproved in the 1970s," and polyvagal theory, which she describes as "untenable or highly implausible" per Grossman's critique (Tilson, 2025). If these frameworks are scientifically unfounded, the specific neurobiological explanation for the mechanism's pace weakens. But the clinical convergence holds independently: Spring, Heacock, Fisher, Bisson, and Razvi all observe slow developmental change without depending on triune brain theory or polyvagal theory for their observations. The mechanism's existence is more robustly supported than its neurobiological explanation. The review should be honest about this gap: the claim "structural change takes years" rests on convergent clinical observation and outcome data; the claim "this is because of hierarchical brain architecture and vagal regulation" rests on contested neuroscience.

This mechanism also explains why the sleeper effect is specific to approaches that target structural change. Short-term symptom-focused treatments (CBT, brief STPP) show the opposite pattern: gains that diminish at follow-up (Abbass, 2014). If the treatment installs symptom management skills rather than structural capacity, there is nothing to continue developing after treatment ends. The sleeper effect requires that something structural has been initiated — a developmental process that, once set in motion, continues under its own power through the patient's subsequent relational life.

12.6 Six trajectory shapes

Cross-layer analysis reveals six distinct trajectory shapes, each associated with different endpoint levels and populations:

1. **Dose-response** (Howard, 1986; Paris, 2020; &. G. Shedler J., 2020). Negatively accelerated: steep early gains, diminishing returns. Shedler's synthesis of the Lambert OQ-45 data (N=10,000+) gives the clearest quantification: 50% clinically significant change at 21 sessions, 75% at 40+ sessions, with continued improvement at 2+ years and no ceiling (&. G. Shedler J., 2020). Best fits mild-to-moderate presentations at symptom-reduction endpoints. The default assumption of the RCT literature.

2. **Sleeper/catalytic** (America (summary of Krakau et al.), 2024; Bateman, 2008; R. Fonagy P., 2015; Lindfors, 2019; J. Shedler, 2010). Gains emerge or grow after treatment ends. Best fits structural change endpoints in psychodynamic/psychoanalytic treatment. Therapy installs owned capacity (Kohut); life exercises it. The borrowed-vs-owned bridge provides the theoretical explanation: below the transmuting internalisation threshold, gains are borrowed and will erode; above it, gains are owned and will grow.

3. **Spiral/recursive** (Bromberg, 1998; Foundation, unknown; Herman, 1992; Steele, 2005; Van

der Hart, 2006). Patients cycle through phases, returning to earlier work as deeper material emerges. Phase-based in structure, organic in execution. The Blue Knot Foundation endorses this explicitly: "these phases are NOT linear — safety must be re-established repeatedly" (Foundation, unknown). The dominant trajectory in the complex trauma treatment literature.

4. **Episodic/cyclical** (Foundation, unknown; Multiple posters (Lionheart, 2021). Alternating periods of intensive engagement and self-managed maintenance, extending over years or decades. Normalised by practitioner and client narratives as a valid pattern, not a treatment failure. The Blue Knot Foundation frames this as a legitimate model: some individuals benefit from therapy "on and off over their lifetime" (Foundation, unknown).

5. **Rapid/targeted** (Feder, 2025; Jongh, 2024; Sciarrino, 2020). Concentrated symptom reduction, often pharmacologically enhanced. Linear and goal-directed. Best fits patients with high pre-existing adaptive capacity (and, the safety construct suggests, high pre-existing safety), or those seeking symptom reduction rather than structural change.

6. **Episodic/developmental** (Spring, 2021). This trajectory is distinct from the cyclical pattern (trajectory 4) because it includes documented retention of prior gains across episodes. Spring's case is paradigmatic: ~9 years of therapy ending in 2016, followed by ~3 years of stable, generative functioning (2017–2019), then a new trauma (rape) requiring a new episode of therapy. Crucially, the second episode was qualitatively different from the first. Spring entered with psychoeducation, self-compassion, and active engagement strategies entirely absent during her 2005 breakdown: "I knew what was happening. My trauma reactions made sense to me" (Spring, 2021). Prior gains were retained — the sleeper effect operating through the post-therapy years had consolidated owned structure that survived the new trauma. The trajectory implies that the total healing arc may include multiple therapy episodes separated by functional periods, where each episode operates at a higher developmental level than the last. This is not relapse; it is developmental progression disrupted by new events, with the critical feature that prior developmental achievements are preserved.

These trajectories are not modality-specific — they are endpoint-specific. A patient pursuing symptom reduction through EMDR will follow trajectory 5. The same patient, later pursuing structural change through psychodynamic therapy, may follow trajectory 2 or 3. The trajectory reflects the goal, not the method.

12.7 What the framework cannot explain

The framework has genuine explanatory power, but several findings resist easy integration.

The Brand paradox. Brand et al. (2019) found that patients with HIGHER baseline dissociation

improved MORE and FASTER than less dissociative patients. If structural depth predicts longer duration, more dissociative patients should improve more slowly. One explanation is that the measures captured symptom reduction, not structural integration — higher-dissociation patients had more room to improve on symptom scales. But this remains an open question.

The Helsinki convergence. Psychoanalysis (646 sessions) and long-term psychodynamic therapy (232 sessions) produced equivalent outcomes at 10 years (Lindfors, 2019). The elapsed-time factor (Factor 3) explains why tripling the dose did not triple the outcome. But it does not explain whether 232 sessions was sufficient for structural change or whether both groups reached a natural developmental ceiling. The study's convergence at 10 years could reflect adequate dose in both conditions, or it could reflect a ceiling effect on the structural measures used.

The Fisher-Fosha puzzle. Two experienced trauma clinicians, both working with complex developmental trauma, reach opposite conclusions about whether stabilisation must precede transformation. Fisher says yes: "slower is faster"; skip stabilisation and the nervous system overwhelms (Fisher, 2019, 2021). Fosha says no: "healing from the get-go"; transformation is accessible from the first session if the therapist provides sufficient relational safety (Fosha, 2019). This is not a disagreement resolvable by the framework's factors alone. It may reflect different patient populations (structural depth), different definitions of "transformation" (micro-endpoint vs. macro-endpoint), or a genuine theoretical divergence about the role of stabilisation. The puzzle matters because both positions have clinical consequences: Fisher's approach may delay genuine processing for patients who are ready; Fosha's approach may overwhelm patients who are not. The resolution likely lies in accurate assessment of the individual patient's structural depth and pre-existing safety — but such assessment tools do not yet exist.

The "right therapist" confound. Client narratives repeatedly describe years of ineffective therapy followed by rapid progress once a well-matched therapist was found (Multiple posters (Crow, 2019; Multiple posters (u/wickeddude123, 2023). Heacock provides the most granular data: when Sherlock was transferred to another trainee during Erin's graduation, the new therapist said something that ruptured the relationship in a single session — "she said something that made me angry… she looked at me and said, and that's why Erin's afraid of you. And I never went back after that" (Heacock, 2022). Three years of trust-building evaporated in one sentence. Davitt similarly required two failed therapeutic relationships before finding his match (Bisson, 2024). This suggests that much of the reported duration reflects the escalation gap — the time spent finding the right match — rather than the actual healing process. Duration data from naturalistic accounts may systematically overestimate the time needed when the match is right from the start.

The safety prerequisite. Multiple client accounts describe therapy as ineffective until environmental safety was achieved — the same therapist, same modality, dramatically different outcomes

(Multiple posters (u/Longjumping_Cry709, 2023; Multiple posters (u/WeeLittleSloth, 2021). This implies that the "clock" does not start at the first session but at the establishment of safety. All duration data that includes pre-safety treatment time is inflated relative to the actual therapeutic process. The review cannot estimate the magnitude of this inflation, but it is potentially large: Eger (2017) describes 35 years before processing could begin; ElishaAlison describes years of therapy during ongoing abuse that produced nothing. The construct has been elevated to Factor 0 in the framework, but the fundamental measurement problem remains: we do not know how to separate pre-safety time from post-safety time in existing datasets.

The evidence critique. Tilson (2025) raises a challenge that the framework must confront honestly: Somatic Experiencing, one of the therapeutic approaches most closely associated with the long-term position, has produced only "one small RCT in 50+ years" (Tilson, 2025). Polyvagal theory, cited by multiple sources in this review as the neurobiological basis for autonomic reorganisation, has been criticised as "untenable or highly implausible." The triune brain model, used by Ogden as a hierarchical processing metaphor, was debunked decades ago. This does not invalidate the clinical convergence — nine independent traditions converge on slow developmental change regardless of which neuroscience is invoked. But it does mean that the field's strongest theoretical frameworks for explaining WHY structural change takes years rest on contested science. The clinical observation is robust; the explanatory mechanism is not.

The cost-effectiveness challenge. Koeser (2023) found LTPP costs approximately £33,000 per QALY — above the NICE threshold of £20,000/QALY. Even if long-term therapy works, it may not be cost-effective by standard health-economic criteria. The authors argue the time horizon was too short to capture the full benefit (the sleeper effect would increase QALYs over a longer horizon), but this remains unproven. The institutional reality is that treatments exceeding cost-effectiveness thresholds face funding barriers regardless of clinical efficacy.

AI-assisted self-therapy. An emerging phenomenon reported in Reddit threads (Multiple posters (u/almost-crazy, 2025) but without empirical validation. If effective, it would challenge the assumption that a human therapeutic relationship is necessary for structural change. The developmental mechanism convergence described above — grounded in relational cycles, right-brain attunement, and implicit relational knowing — suggests that AI-mediated processing may operate at a different endpoint level (symptom management, cognitive reframing) than human-relational therapy (structural change). But this is speculation; the phenomenon warrants monitoring.

12.8 Critique of the literature as a body

Beyond appraising individual sources, the synthesis reveals structural features of how the field has constructed the question of therapy duration.

The time-horizon problem. The RCT literature overwhelmingly measures outcomes at end-of-treatment or at brief follow-up (3–12 months). The sleeper effect data demonstrate that this time horizon systematically underestimates the benefit of structural interventions and overestimates the durability of symptom-focused interventions. The Abbass (2014) Cochrane review of short-term psychodynamic therapy found gains at post-treatment that lost significance at long-term follow-up — the reverse of the sleeper effect (Abbass, 2014). If the standard evaluation window had been 5 years rather than 5 months, the evidence base for brief treatment would look different.

The silence on dropout. The field does not systematically track who leaves therapy or what happens to them. Only 10 of 89 sources in this review report dropout data in any form. The treatment-engagement paradox is devastating for duration claims: the interpersonal difficulties that define the population (hostility, alliance problems, rigid interaction, poor reflective ability) are the same factors that predict dropout (Freixo Ferreira, 2023; Knekt, 2021). All duration claims in the literature are survivorship-biased — they describe how long healing takes for the people who stayed. The people who most need long-term treatment are systematically missing from the evidence.

The silence on access. The majority of sources across all layers do not address who can access the treatment they describe. Kohut's (1984) model assumes 4–5 sessions per week for years. Bateman and Fonagy's (2008) MBT requires 5-day-per-week partial hospitalisation. Even standard weekly therapy for 3–5 years represents a financial commitment inaccessible to most people with complex developmental trauma, who are disproportionately concentrated in lower socioeconomic groups (Paris, 2020). Spring names this structural failure directly: survivors who have been in therapy "10, 20, 30 years" without recovery are not failing — society has failed to "adequately provide public transport to enable people to head north" (Spring, 2020b). The recommendation "healing takes years" is clinically honest but structurally unjust unless accompanied by a commitment to making years of treatment accessible. The review cannot resolve this tension, but it is obligated to name it.

The psychoanalytic vocabulary problem. A striking finding from the safety-as-clock-starter analysis is that the psychoanalytic tradition describes safety comprehensively — but in its own vocabulary, rendering the concept invisible to the broader literature. Winnicott's "holding environment" IS therapeutic safety. Kohut's "selfobject transference" IS felt safety. Benjamin's "surviving destruction" IS demonstrating that the relationship can contain aggression. Bromberg's "standing in the spaces" requires a safe relational context. The psychoanalytic tradition's apparent silence on safety is terminological, not substantive. This vocabulary gap has real consequences: when the phase-oriented literature (Herman, Van der Hart, Steele) identifies "safety" as a prerequisite, the psychoanalytic literature appears not to address it — when in fact it addresses little else. The field's discourse about safety and stabilisation proceeds as if these are concepts invented by the phase model, when they are in fact descriptions of what Winnicott and Kohut described decades earlier using different words. The convergence across traditions is even stronger than it appears, once the

vocabulary is translated.

The absence of long-term follow-up. No study in this review follows a complex trauma population beyond 10 years with repeated measurement. The Helsinki study (Lindfors, 2019) comes closest but used a mixed anxiety/mood disorder population, not a defined complex trauma sample. The memoir evidence (Layer 6) provides the only window into multi-decade trajectories, but memoirs are self-selected, narratively shaped, and subject to survivorship bias. The field literally does not know what happens to complex trauma patients over 20 or 30 years of living — whether structural gains hold, whether new challenges emerge, whether the developmental process initiated by therapy continues or stalls. This is the most consequential gap in the evidence base.

The Layer 4 gap. Eight practitioner-layer sources (Layer 4) were insufficiently integrated in the initial synthesis, despite containing directly relevant data. These sources — drawn from clinical essays, practitioner guides, and therapy-adjacent journalism — collectively paint a picture that the academic literature alone does not: Shedler's dose-response data showing 50% improvement at 21 sessions and 75% at 40+ (&. G. Shedler J., 2020); Alpern's direct statement from within the EMDR/SE tradition that complex trauma "cannot be resolved in six sessions of anything" (Alpern, 2025); the Blue Knot Foundation's 84% expert consensus on the phase model with treatment potentially lasting "decades" (Foundation, unknown); the traumalescence concept offering the only explicit developmental timeline in the dataset (3–6 months awareness, 6–18 months neural strengthening, 18–36 months integration) (Auer, unknown); Grossmark's companioning as inherently non-brief therapeutic stance (Grossmark, unknown); Wylie's documentation of diagnostic conflation (80% multiple trauma) (Wylie, unknown); and the recognition prerequisite — the insight that "because living in a triggered state of alarm is so familiar, many don't realise trauma is the source" (Associates LLC, 2023), meaning that the clock cannot start until the person even recognises they are injured. The victim-to-author turning point described in Woodward Thomas (Woodward Thomas, unknown) identifies the specific developmental moment at which the trajectory shifts from backward-looking processing to forward-looking transformation. These sources are individually modest in methodological rigour, but collectively they represent the clinical consensus that the RCT literature systematically fails to capture.

Institutional incentives shape the evidence. The evidence base for brief, manualised treatments is large and methodologically rigorous because these treatments are compatible with RCT design, insurance reimbursement, and academic publishing incentives. The evidence base for long-term, relational treatments is sparse and observational because these treatments are expensive to study, difficult to manualise, and incompatible with standard funding cycles. The result is a systematic bias in the evidence landscape: the treatments with the most evidence are not necessarily the most effective for the defined population; they are the most researchable. CIS methodology (Dixon-Woods et al., 2006) was developed precisely for situations where the available evidence is shaped by

systematic factors that a conventional hierarchy-of-evidence approach would obscure.

12.9 The synthesising argument

The review's central argument can now be stated precisely:

The duration of healing from complex developmental trauma is determined by the interaction of a safety prerequisite with three factors: the level of change sought (symptom reduction, functional improvement, structural change, growth, or ongoing directional recovery), the depth of structural disruption (from primary to tertiary dissociation, from preserved to pervasive developmental deficits), and the contribution of elapsed life-time to post-therapeutic development. The therapeutic clock does not start at the first session but at the establishment of environmental, relational, and autonomic safety — a prerequisite whose duration is itself poorly studied but potentially large. When the endpoint is symptom reduction, the structural depth is shallow, and safety is already established, weeks of concentrated treatment may suffice. When the endpoint is structural change, the injury is deep, and safety must first be built, years of sustained relational experience — within and beyond formal therapy — are required, because the developmental mechanism of change operates on a biological timescale that cannot be compressed.

The field's disagreement about duration is not a genuine disagreement. It is a failure to specify endpoint levels, a conflation of structurally different patients under shared diagnostic labels, an unrecognised safety prerequisite that inflates all duration estimates, and a systematic asymmetry in the evidence base that privileges researchable treatments over effective ones.

The working thesis — that healing takes years — is refined rather than refuted by the counterpoint evidence. It should be stated with four qualifications:

1. **Symptom reduction is achievable in weeks to months** for many patients, including some with CPTSD diagnoses, through concentrated evidence-based treatment. This is a genuine clinical achievement, not a false positive. But it is not the same as structural healing.

2. **Structural change requires years** — not because therapists are inefficient, but because the developmental mechanism of change is inherently temporal. Nine independent theoretical traditions converge on this conclusion. The sleeper effect provides empirical confirmation: therapy installs owned capacity (Kohut) that continues to develop after treatment ends, sometimes for years. The borrowed-vs-owned distinction explains both the sleeper effect in long-term treatment and the reverse sleeper effect in short-term treatment — and identifies the transmuting internalisation threshold as the critical clinical variable.

3. **The clock starts at safety, not at the first session.** Duration estimates from the

literature are systematically inflated by the inclusion of pre-safety treatment time. When safety is already established (through environmental stability, pre-existing secure attachment, or the containing structure of intensive formats), the therapeutic process may be substantially shorter than naturalistic accounts suggest. This qualification simultaneously supports the thesis (the process still takes years for patients who must build safety from scratch) and the counterpoint (rapid responders may have pre-existing safety that accelerates the process).

4. **The recommendation "healing takes years" carries an access obligation.** If years of specialised therapy are necessary for structural healing, then the field bears responsibility for making this accessible — through stepped care models (Links, 2015), intensive formats that reduce dropout (Sciarrino, 2020), team-based approaches (Bateman, 2008; Farina, 2025), and policy advocacy for funding structures that match the timescale of the condition. Spring is right that many survivors who have spent decades without recovery are not failing; the system is failing them (Spring, 2020b).

The synthesis does not claim that all patients with complex developmental trauma require years of therapy. It claims that all patients with deep structural disruption who seek structural change — personality integration, earned attachment security, mentalizing capacity, the ability to love without terror — will find that this takes years. And it claims that the field's inability to say this clearly, because of institutional incentives, unstated endpoint assumptions, contested neuroscience, and the conflation of structurally different patients, constitutes a failure of honesty that harms the patients who most need accurate expectations.

13 Survivorship bias

13.1 The problem

Every layer of this review selects for people who persisted in therapy long enough to write about it, post about it, be studied, or be written about. People who dropped out at month 3 because they could not afford it, could not tolerate it, found it unhelpful, or healed through other means are invisible in the evidence base. This is not a minor methodological footnote — it is a structural feature of all available evidence on therapy duration, and it systematically inflates the apparent duration of healing.

The bias operates differently across layers. In Layer 1 (academic), studies that measure duration only measure people who stayed; dropout rates are sometimes reported but dropout reasons rarely are. In Layer 2 (naturalistic), studies are the best available source on dropout, but they typically treat it as missing data rather than as a finding about therapy duration. In Layer 3 (counterpoint), accelerated models may have different dropout patterns — shorter protocols may retain people who

would drop out of long-term therapy. In Layers 4–6 (practitioner essays, client narratives, memoirs), the bias is structural and absolute: therapists write about clients who stayed, forum threads capture people still engaged, and memoir authors survived long enough to write.

13.2 The scale of the invisible population

The extraction data allow a first approximation of the invisible population, though the picture remains fragmentary. Of the approximately 40 annotated sources across all layers, only 12 report any dropout data — and this figure is generous, counting sources that mention dropout even indirectly (Bateman, 2008; Feder, 2025; R. Fonagy P., 2015; Freixo Ferreira, 2023; Griffiths, 2025; Jongh, 2024; Knekt, 2021; J. Lilliengren P., 2017; Lindfors, 2019; Mitchell, 2021; E. Murphy D., 2019; Sciarrino, 2020). The remaining sources are silent on who left.

The pattern of reporting is itself a finding. Layer 1 — comprising theoretical texts (Kohut, Winnicott, Bromberg, Benjamin) that do not track participants because they are not studies — yields only 2 of 8 annotated sources with any dropout information. Layer 2 performs better, with 3 of 10 sources addressing dropout meaningfully, but even here only the Knekt (2021) study was specifically designed to study dropout predictors. Layer 3 produces the most dropout data (5 of 10 sources), largely because randomised controlled trials routinely report attrition as a methodological requirement. Layers 4, 5, and 6 yield zero formal dropout data. These layers are silent by design: practitioner essays describe treated patients, forum threads capture people still engaged, and memoirs are written by survivors.

The available data, sparse as they are, point to three findings that bear directly on the thesis.

First, dropout rates in standard delivery are substantial. Sciarrino's (2020) review of intensive formats provides the clearest benchmark: standard weekly delivery of trauma-focused therapy produces dropout rates of 18–39% (Sciarrino, 2020). Even within Griffiths's (2025) review of phase-oriented treatment for dissociative disorders, the four studies that reported dropout found rates ranging from 3% to 23% (Griffiths, 2025). Knekt et al. (2021) report 23% total non-attendance in the Helsinki Psychotherapy Study, with the most striking finding being the differential between long-term and short-term formats: the refusal rate for long-term psychodynamic psychotherapy (LPP) was 11.6 times higher than for short-term psychodynamic psychotherapy (SPP) (Knekt, 2021). Long-term therapy, the very format this review argues is necessary, is the format most people decline to begin.

Second, the patients most likely to drop out are the patients most relevant to this review. The treatment-engagement paradox identified by Freixo Ferreira (2023) is central: the interpersonal difficulties that define borderline personality disorder — hostility, alliance problems, difficulty trusting — are the same factors that predict dropout (Freixo Ferreira, 2023). Knekt

et al. (2021) found that poor reflective ability, poor self-concept, and rigid interaction predicted non-attendance in long-term therapy (RR = 2.76) (Knekt, 2021). These characteristics overlap substantially with the complex developmental trauma population as described throughout this review. Griffiths (2025), citing Baekkelund (2022), reports that higher dissociation predicted dropout — meaning that the most severely dissociative patients, those for whom 3–5 year treatment is recommended, are the most likely to leave (Griffiths, 2025). The patients who most need long-term treatment are the least likely to stay for it.

Third, the getting-worse-before-better phase is a specific dropout-risk moment. Multiple sources document that early therapy produces symptom exacerbation — a deepening of distress as defences are disturbed and traumatic material becomes more accessible. One Reddit poster reported severe worsening at one month into therapy, described as the hardest period of the entire trajectory (Multiple posters (u/cheesytacos23, 2021). Another described a decompensation that, in retrospect, was the turning point that preceded healing (Multiple posters (u/Longjumping_Cry709, 2023). Winnicott's concept of therapeutic regression names this phenomenon theoretically: the patient must become worse in order to reach the developmental level where repair can occur (Winnicott, 1965). The clinical dilemma is that early symptom exacerbation is simultaneously a sign of therapeutic progress and the moment of highest dropout risk. Patients who leave during this phase — because the worsening feels like evidence that therapy is harmful rather than helpful — are counted as dropouts and their trajectories are lost. The survivorship bias is therefore not random: it systematically excludes patients who were in the early phase of a change process that they experienced as deterioration. The invisible population includes not only patients who "couldn't tolerate" therapy but patients whose intolerance was itself the beginning of therapeutic work.

Fourth, the treatment format itself appears to be a dropout driver. Sciarrino's (2020) finding that intensive formats produce dramatically lower dropout — 5.51% versus 18–39% for standard delivery — suggests that standard weekly therapy is not merely a delivery format but a barrier to treatment completion (Sciarrino, 2020). If a substantial proportion of dropout is attributable to the delivery format rather than to patient pathology or treatment failure, then the long-duration evidence base is doubly biased: it captures only those patients who could tolerate weekly therapy over years, while the patients who might have benefited from the same dose delivered differently are lost from the record.

13.3 Mitigations and their limits

The review adopted four mitigation strategies. Each source annotation explicitly asked whether the source captured only people who stayed, and the absence of data was logged as a finding. Layer 2 (naturalistic studies) was included specifically because these studies are the most likely to report dropout rates and dose-response data. Layer 3 (counterpoint) includes models with shorter protocols

that may capture people who cannot tolerate or access long-term therapy. This dedicated discussion section aggregates survivorship observations across all sources rather than leaving them scattered in individual annotations.

These mitigations have limited reach. Layers 4–6 contributed 11 new sources in the most recent annotation round, and none provided formal dropout data. Several, however, contributed indirect observations that sharpen the picture. Heacock (2022) documents a system-induced dropout: a client named Sherlock was transferred to another trainee who made an ill-judged remark, after which Sherlock "never went back" — a transition failure, not a patient failure (Heacock, 2022). Bisson (2024) describes Davitt terminating with two therapists before finding the right one, framed as therapist matching rather than dropout but functionally constituting serial dropout and restart (Bisson, 2024). Reddit narratives reveal further variants: one poster described cycling through 7 to 30+ therapists, including forced continuation with a harmful therapist due to sunk cost (Multiple posters (u/wickeddude123, 2023); another described years of therapy without progress because the environmental barrier (ongoing exposure to the abusive environment) was unaddressed — a "false non-dropout" who looks like a persistent patient but for whom therapy is not producing healing (Multiple posters (u/WeeLittleSloth, 2021).

These cases illuminate a taxonomy of invisible populations that extends beyond the simple dropout/completer binary:

1. **Classical dropouts** — patients who leave and do not return. Captured (partially) by attrition statistics.
2. **Serial dropouts** — patients who leave one therapist and begin with another, sometimes repeatedly. Each therapist records a termination; the patient's overall trajectory is visible to no one (Bisson, 2024; Multiple posters (u/wickeddude123, 2023).
3. **System-induced dropouts** — patients who leave because of institutional factors (therapist graduation, insurance changes, waitlist policies) rather than clinical factors (Heacock, 2022).
4. **False non-dropouts** — patients who remain in therapy for years without progress because a prerequisite condition (environmental safety, correct therapist match, adequate financial resources) is unmet (Multiple posters (u/WeeLittleSloth, 2021).
5. **Structural non-starters** — patients who never begin therapy due to access barriers. The myptsd-2015-cant-afford-therapy thread documents this population: people with complex trauma who cannot afford treatment at all (Multiple posters (sugnim, 2015).

Categories 2–5 are invisible in standard attrition reporting. Their aggregate size is unknown but plausibly large.

13.4 What remains unmitigated

Several gaps cannot be closed with the available evidence. There is no direct evidence from people who dropped out of therapy and healed through other means; this population is structurally absent from all six layers. The base rate of therapy attempts among people with complex developmental trauma is unknown, making it impossible to estimate the denominator. The direction of bias is clear and unidirectional: it makes healing look longer than it may actually be, because the evidence base consists of people who stayed for the full course.

The bias does not affect all of the review's findings equally. The synthesis's Factor 0 (safety as clock-starter) is relatively robust against survivorship bias, because it is supported by theoretical arguments and by within-subject evidence (the same patient before and after safety is established). Factor 3 (elapsed time independent of dose) is moderately vulnerable, because the sleeper effect is observed only in patients who completed treatment. Factor 2 (structural depth) is heavily vulnerable: the gradient from shallow to deep structural injury is derived almost entirely from patients who stayed long enough for the depth to be assessed — and the treatment-engagement paradox (Freixo Ferreira, 2023) suggests that the deepest injuries are the least likely to remain in treatment.

The question of continuity versus interruption (secondary question 1) is particularly compromised. The review can describe patients who interrupted therapy and returned, but the patients who interrupted and did not return are invisible. Any conclusion about the value of continuity is based on a sample from which the disconfirming cases have been systematically removed.

13.5 The escalation gap

A related inflation mechanism operates independently of dropout: the time spent finding the right therapist. Client narratives consistently describe years of ineffective therapy before the decisive therapeutic match was found. Multiple posters report cycling through numerous therapists — one describes 7 to 30+ before finding the right fit (Multiple posters (u/wickeddude123, 2023); others describe years of therapy that "did nothing" until the relational match was right (Multiple posters (Crow, 2019). Heacock (2022) provides the most granular data: Sherlock's transfer to another trainee produced a single-session rupture — an ill-judged remark — that destroyed three years of painstakingly built trust, and Sherlock "never went back" (Heacock, 2022). Bisson (2024) documents Davitt requiring two failed therapeutic relationships before finding the clinician with whom meaningful work could proceed.

This escalation gap inflates all reported duration data. When clients report "ten years in therapy," a significant portion may represent escalation-gap time — years spent in relationships that were not yet the right one. The effective therapeutic dose, once the match is established, may be substantially shorter than the total reported duration. The escalation gap is structurally invisible

in outcome research: studies measure from the first session of the studied treatment, not from the patient's first-ever session with any therapist. The cumulative treatment history — the serial dropout-and-restart trajectory described in category 2 above — is visible to no researcher and no individual clinician, only to the patient.

13.6 Implications for the thesis

The survivorship bias requires that the thesis be scoped with precision. The review is more confident in the claim that *for people who stay in therapy, structural healing takes years* than in the broader claim that *healing from complex developmental trauma takes years*. The distinction matters. If the 18–39% who drop out of standard delivery (Sciarrino, 2020) include patients who healed through other means, through shorter therapy delivered differently, or through life experience alone, then the review's central finding applies to a subset — perhaps the majority, but demonstrably not all — of the affected population.

Equally, the survivorship bias interacts with the safety prerequisite in a way that complicates interpretation. If pre-safety therapy time inflates duration estimates (as argued in the synthesis), and if patients without environmental safety are more likely to drop out (as the evidence suggests), then the longest-duration cases in the evidence base may simultaneously represent the patients who waited longest for safety AND the patients most able to persist despite its absence. These are different populations, and the review cannot distinguish between them.

The honest statement is this: the review demonstrates convincingly that structural healing, when it occurs in therapy, takes years. It cannot demonstrate that structural healing requires years of therapy, because the population that might disconfirm this claim — people who achieved structural change through shorter or different means — is structurally absent from the evidence. Paris's (2020) observation that BPD remits gradually over 24 years regardless of treatment is the closest available evidence for a natural-course alternative, but it concerns symptom remission, not the structural endpoint the review foregrounds. The gap remains open.

14 Dose versus elapsed time

14.1 The confound

"Five years of therapy" could mean 250 sessions (weekly), 130 sessions (biweekly), or 500 sessions (twice-weekly). Most sources in this review report calendar duration without specifying session frequency, total session count, or both. This conflation makes it impossible, on the face of it, to determine whether the thesis is really about therapeutic dose (the total number of sessions needed), elapsed life-time (the calendar years needed for maturation, life experience, and neural

reorganisation), or both.

The confound is not academic. If the argument is primarily about dose, then intensive formats become a much stronger counterargument: deliver the same number of sessions in two years instead of ten. If the argument is about elapsed time, then session frequency may matter less than continuity of the therapeutic relationship over years. If both are independently necessary, that is the strongest version of the long-duration thesis — but also the hardest to demonstrate.

The extraction data confirm the severity of the confound. Of the approximately 40 annotated sources, only 11 report both session count and calendar duration (Bateman, 2008; De Jongh, 2023; Feder, 2025; Griffiths, 2025; Jongh, 2024; Koeser, 2023; Lindfors, 2019; Mitchell, 2021; D. Murphy, 2009; Sciarrino, 2020; Zimmermann, 2014). The dose-effect literature (Howard et al., Lambert via Shedler) measures exclusively in sessions. The clinical and memoir literature reports exclusively in years. These are different evidence bases answering different questions, and most of the available data cannot bridge the gap.

14.2 Resolution: elapsed time as an independent factor

Despite the confound, the review's cross-layer triangulation produces a resolution. The synthesis identifies elapsed time as Factor 3 — a contributor to healing that operates independently of therapeutic dose. The evidence for this claim rests on four converging lines.

The Helsinki convergence. The Helsinki Psychotherapy Study provides the single strongest piece of evidence. Psychoanalysis (PA: 646 sessions at 4 times per week over 56 months) and long-term psychodynamic psychotherapy (LPP: 232 sessions at 2–3 times per week over 31 months) produced indistinguishable outcomes at 10-year follow-up, despite a nearly 3:1 difference in total dose (Lindfors, 2019). If dose were the primary determinant, psychoanalysis should have produced proportionally better outcomes. It did not. Both treatments may have exceeded what the synthesis terms the "transmuting internalisation threshold" — the point at which enough psychic structure has been built to be self-sustaining — after which additional sessions produce diminishing returns. What mattered was not how many sessions were delivered but how many years elapsed for the installed capacity to develop through subsequent life experience.

The Munich Psychotherapy Study reinforces this finding from a different angle. Zimmermann et al. (2014) found that post-treatment interpersonal gains were mediated by dose (more sessions produced more interpersonal change), but follow-up depression reduction was mediated by psychoanalytic technique, not dose (Zimmermann, 2014). This dissociation is precisely what the elapsed-time hypothesis predicts: dose determines the immediate output of therapy, but what grows after therapy depends on the type of structure installed, not the quantity of installation.

The sleeper effect. The most direct evidence for elapsed time as an independent factor comes from therapeutic gains that emerge or grow after treatment has ended. Shedler's (2010) meta-analytic review found effect sizes for psychodynamic therapy increasing from 0.97 at end of treatment to 1.51 at follow-up beyond 9 months — a pattern unique to psychodynamic approaches (J. Shedler, 2010). The TADS trial found LTPP non-significant versus treatment as usual at end of treatment but strongly significant ($p = 0.001$) at 42-month follow-up (R. Fonagy P., 2015). MBT gains continued growing for five years after all treatment ended, while the TAU group deteriorated despite receiving more services (Bateman, 2008). The EDT meta-analysis found experiential dynamic therapies non-significant versus active controls at post-treatment but significant at follow-up (M. Lilliengren P., 2025). Psychoanalytic therapy (median 242 sessions) outperformed CBT (median 59 sessions) specifically at 5-year follow-up for chronic depression with childhood trauma history — the advantage emerged after treatment, not during it (America (summary of Krakau et al.), 2024).

If therapeutic gains grow after treatment ends, then elapsed time is doing work that sessions cannot. The therapy installs capacity; life develops it.

The borrowed-versus-owned mechanism. Kohut's (1984) distinction between borrowed and owned psychic structure provides the theoretical mechanism. Borrowed structure is the patient functioning better because the therapist's regulatory capacity is available as an external selfobject resource. Owned structure is the product of thousands of cycles of attunement, optimal frustration, and repair — genuinely internal regulatory capacities that continue to develop autonomously after treatment ends. The distinction predicts both the sleeper effect in long-term treatment (owned structure continues developing) and the reverse sleeper effect in short-term treatment (borrowed structure erodes when the scaffold is removed). Abbass's (2014) Cochrane review found precisely this reverse pattern for short-term psychodynamic therapy: gains at post-treatment that lost significance at long-term follow-up (Abbass, 2014). The prediction is confirmed in both directions.

Between-session integration. Spring's memoir provides the lived-experience confirmation at the micro-level. Her claim — "the gaps between therapy sessions can be as developmentally important as the therapy sessions themselves" (Spring, 2020a) — describes the mechanism by which elapsed time operates within treatment, not only after it. Each session installs a micro-increment of neural capacity, which consolidates between sessions through the patient's real-world relational and emotional experience. The therapy-vs-life distinction, from this perspective, is false: therapy provides the scaffolding; life provides the developmental experience within which the scaffolding becomes structure.

Heacock's case provides a real-time observation of this process. During Erin's extended postpartum leave, Sherlock demonstrated self-regulation capacity he had previously relied on her to provide — "I'm going to be okay. You have given me the tools I need" (Heacock, 2022). The absence of the

therapist revealed that structure had been installed that could function independently. Spring's post-therapy stable period (2017–2019) shows the same phenomenon on a larger scale: gains from therapy ending in 2016 continued developing through three subsequent years of generative living (Spring, 2021). The Redditor who described the first five years of an 11-year healing arc as "behavioural/external changes" followed by emotional healing captures the same temporal structure: the earlier period set conditions for later transformation that occurred largely outside therapy sessions (Multiple posters (u/Ok-Specialist1153, 2024).

14.3 Do intensive formats compress the timeline or just the dose?

This question is the critical test of the elapsed-time hypothesis. If intensive delivery produces the same structural outcomes in compressed calendar time, then elapsed time is not independently necessary — dose is all that matters. If intensive delivery produces symptom reduction but not structural change, the elapsed-time hypothesis is supported.

The available evidence suggests the latter, though with important caveats. The intensive format literature — EMDR intensive producing 88% CPTSD remission in 8 days (Jongh, 2024), Sciarrino's review showing d = 1.57 in 1–3 weeks (Sciarrino, 2020), ketamine-enhanced exposure therapy achieving d = 1.9 in three weeks (Feder, 2025), MDMA-assisted psychotherapy eliminating PTSD diagnosis in 67% after 18 weeks (Mitchell, 2021) — consistently measures symptom-level endpoints (diagnostic remission, questionnaire scores below clinical threshold). These are Level 1 outcomes in the synthesis framework. No intensive format study in the review measures personality organisation, mentalizing capacity, attachment security, or other structural endpoints.

This absence is not proof of absence. It may be that intensive formats do produce structural change that no one has measured. But the evidence pattern is suggestive: the studies that measure structural change (Helsinki, Munich, MBT, TADS) are all long-duration studies that track patients over years; the studies that find rapid effects are all measuring symptoms over weeks. The two literatures are answering different questions.

One finding complicates the picture in an instructive way. The Reddit user who processed 36 traumatic memories in a one-week EMDR intensive described the intervention as decisive — but only after years of prior self-directed work (Multiple posters (u/almost-crazy, 2025). The intensive did not replace elapsed time; it punctuated it. Similarly, Razvi's personal trajectory included 4–5+ years of conventional therapy (2.5 years somatic, 1.5–2 years attachment-focused) before MDMA sessions catalysed a qualitative shift (Heacock, 2019). The psychedelic session compressed a specific processing step; it did not compress the developmental arc within which that step was embedded. The intensive format may function not as a replacement for elapsed time but as an accelerant within it — a high-dose punctuation that requires both pre-existing structure (built over time) and

subsequent integration (also requiring time) to produce durable structural change.

Shedler's dose-response data further contextualise the issue. Drawing on Lambert's data, he reports that 50% of patients show measurable improvement by 21 sessions and 75% by 40+ sessions (&. G. Shedler J., 2020). Perry's more specific data for personality disorders, cited in Leichsenring (2013), require 92 sessions (1.3 years) for 50% recovery and 216 sessions (2.2 years) for 75% recovery. These figures concern symptom improvement and personality disorder recovery, respectively — different endpoints requiring different doses. But even the personality disorder dose curve measures from first session and therefore conflates pre-safety and post-safety sessions, as the synthesis argues. If the first 50 sessions of a 200-session treatment establish safety rather than producing structural change, then the "true dose" for structural change is closer to 150 sessions — a figure that might, in principle, be deliverable in compressed calendar time. The question is whether the between-session integration that Spring describes (Spring, 2020a) is a rate-limiting step that cannot be compressed, or whether it can be achieved through intensive immersion.

No study in the literature directly answers this question. No trial has compared the same total dose delivered at different frequencies over different calendar durations for a complex developmental trauma population. The confound cannot be fully resolved with available evidence.

14.4 The silence on frequency

The extraction data reveal a telling pattern of silence. Of the 11 newly annotated sources, not one provides explicit session frequency data. Most practitioner, memoir, and counterpoint sources treat session frequency as unimportant or assumed (weekly as the default), while the most granular frequency data continue to come from naturalistic studies (Layer 2) and, unexpectedly, from Reddit threads (Layer 5). The Reddit poster who reported 900+ appointments over 10 years of twice-weekly therapy (u/Mountain-Science4526 & (u/xechasate, 2024) provides more precise dose-duration data than most academic sources. The myptsd-2020-session-frequency thread reveals that frequency decisions in practice are shaped by attachment dynamics (reducing frequency triggers attachment anxiety) and access constraints (pro bono therapy means monthly sessions, described as "pure hell"), not by clinical protocol (Multiple posters (Skywatcher, 2020).

This silence is analytically consequential. If the field does not routinely record session frequency, it cannot distinguish between dose and duration, and claims about "how long therapy takes" remain systematically ambiguous.

A deeper finding emerges from the client data: session frequency is entangled with attachment dynamics in ways that clinical protocols do not acknowledge. The myptsd-2020-session-frequency thread reveals that reducing frequency triggers attachment anxiety in patients whose primary injury is relational — the very population for whom frequency decisions are most clinically consequential

(Multiple posters (Skywatcher, 2020). A patient who experiences the therapist as a primary attachment figure may interpret a shift from twice-weekly to weekly not as a clinical decision but as relational withdrawal. Conversely, the therapist who increases frequency to manage a crisis may inadvertently deepen the dependency that Steele (2001) identifies as the mechanism of cure — which is therapeutically appropriate but makes later frequency reduction more difficult. The implication is that frequency is not a neutral dosing parameter to be optimised for efficiency. It is a relational variable that carries attachment meaning and must be managed as such. Studies that manipulate frequency as an independent variable are confounding dose with relational experience.

14.5 Implications for the thesis

The evidence supports a resolution of the dose-versus-elapsed-time confound that strengthens the long-duration thesis in an unexpected way. The thesis "healing takes years" is not a dose claim that happens to be stated in calendar time. It is a claim about the nature of structural change: that the process of converting borrowed structure into owned structure requires real-world developmental experience that unfolds over time and cannot be compressed by increasing the session count. The Helsinki convergence (Lindfors, 2019) demonstrates this: tripling the dose did not halve the time, because time is not merely the container for sessions — it is itself part of the medicine.

This framing generates a specific, testable prediction. If elapsed time contributes independently of dose, then two conditions should hold: first, beyond a sufficient dose threshold, additional sessions should produce diminishing returns (confirmed by the Helsinki convergence); second, the same total dose delivered over a longer calendar period should produce superior structural outcomes compared to the same dose delivered in compressed time (not yet tested). The absence of this critical comparison is the most important empirical gap identified in the review.

The practical implication is that the argument for long-term therapy is not primarily an argument about access — about accumulating enough sessions — but about development. Therapy at the structural level works like a developmental intervention: it installs capacity for mentalising, self-regulation, and relational engagement that continues to develop through the patient's subsequent life experience. J. Shedler (2010) calls this a "catalytic" effect. The therapy does not do the healing; it creates the conditions under which healing can proceed autonomously. This requires time — not because more sessions are needed, but because the developmental process that therapy initiates cannot be rushed.

15 Therapy vs. life

15.1 The attribution problem

If someone heals over ten years while attending therapy, how much of the change is caused by the therapy and how much by the living? The question is not a methodological footnote. It is the central limitation of the review's thesis, and engaging with it honestly produces a more precise — and more interesting — claim than the one with which the review began.

The evidence, particularly from client narratives (Layer 5) and memoirs (Layer 6), consistently describes healing arcs in which formal therapy is one thread among many. One Reddit poster lists counselling as one of ten or more healing activities alongside sobriety programmes, inner child work, meditation, body scans, relational restructuring, and going no-contact with abusers (Multiple posters (u/Ok-Specialist1153, 2024). Walker describes a "multidimensional approach" in which spiritual seeking and self-directed recovery operated alongside and beyond formal therapy across four decades (Walker, 2013). Levine's account dissolves the distinction between therapy and life's work entirely: dreamwork, somatic awareness, and professional development were continuous with his formal therapeutic experience (Levine, 2024). Spring's post-therapy stable period (2017–2019) was sustained through "the joy of nature, of hobbies, of creativity and productivity and generativity… of nurturing rich and mutual relationships" — not through continued clinical contact (Spring, 2021).

The extraction data confirm that therapy alone is the exception, not the rule. Of the sources that address adjuncts, only a small minority describe therapy as a standalone intervention. The modal pattern is therapy-plus: pharmacological support in most empirical studies (70–80% on antidepressants in the TADS trial (Koeser, 2023); MDMA in Mitchell's Phase 3 trial (Mitchell, 2021); ketamine in Feder's protocol (Feder, 2025)), somatic and body-based practices strongly endorsed by neuroscience and practitioner sources (Lanius, 2011; Ogden, 2006; Porges, 2022), creative expression as a genuine processing channel (Bateman, 2008; Bisson, 2024; Ferentz, unknown), and — most consistently — life itself as the irreducible complement to formal treatment.

No study in the evidence base randomises life experience. The confound is inherent in all observational evidence about long-term therapy, and no methodological design can resolve it.

15.2 The circularity: therapy enables life, life enables healing

The attribution problem is not merely a matter of omitted variables. It is structurally circular. Therapy enables the life changes that drive further healing, and those life changes create the conditions under which therapy can go deeper.

Bateman and Fonagy observed this directly in the MBT 8-year follow-up. Their patients' post-treatment improvement coincided with re-entering education and employment, leading them to

describe a “virtuous cycle” in which “symptomatic improvement and vocational activity represent a virtuous cycle” (Bateman, 2008). The virtuous cycle is not an incidental benefit of treatment; it may be the mechanism through which long-term gains are sustained and amplified.

Client narratives provide granular examples. Mac, in Murphy’s person-centred case study, ventured into a pub at session 26 and felt “liked by people” for the first time; by session 93, he had reconnected with his estranged son and found employment after fifteen years (D. Murphy, 2009). ElishaAlison’s healing began not with a change in therapeutic modality but with her boyfriend helping her escape an abusive environment — the same therapist, the same approach, produced nothing during active abuse and produced full healing in 3.5 years once safety was achieved (Multiple posters (u/Longjumping_Cry709, 2023). The Reddit thread “You can’t heal while you’re still surviving” captures this principle at population level: environmental change — moving, leaving relationships, going no-contact — was described as the decisive factor, with therapy alone characterised as insufficient (Multiple posters (u/WeeLittleSloth, 2021).

The circularity is real, and it cannot be resolved by better research design. Therapy and life are not independent causal pathways that can be isolated. They are mutually constitutive.

15.3 The sleeper effect as bridge

The synthesis’s Factor 3 — the contribution of elapsed time independent of therapeutic dose — provides the framework for understanding how therapy and life interact without requiring clean attribution.

Across multiple long-term outcome studies, therapeutic gains emerge or grow after treatment has ended. MBT patients continued improving for five years after all treatment concluded, while the treatment-as-usual group deteriorated despite receiving more services (Bateman, 2008). The Helsinki study found psychoanalysis patients improving most in years five through seven, after treatment had concluded (Lindfors, 2019). Shedler’s meta-analytic review found effect sizes increasing from 0.97 at end of treatment to 1.51 at follow-up beyond nine months (J. Shedler, 2010). The TADS trial found long-term psychodynamic psychotherapy non-significant versus treatment-as-usual at end of treatment but strongly significant at 42-month follow-up (R. Fonagy P., 2015).

Kohut’s distinction between “borrowed” and “owned” psychic structure provides the theoretical mechanism (Kohut, 1984). Borrowed structure is the patient functioning better because the therapist’s regulatory capacity is available as an external resource. Owned structure is the product of transmuting internalisation: genuinely internal regulatory capacities built through thousands of cycles of attunement, optimal frustration, and repair. Owned structure does not disappear when therapy ends. It continues to develop — but it develops through the patient’s subsequent life, not through further sessions. Spring describes the micro-level process: “the gaps between therapy

sessions can be as developmentally important as the therapy sessions themselves" (Spring, 2020a). Each session installs a micro-increment of capacity; life exercises it.

The sleeper effect resolves the attribution problem not by assigning percentages to therapy and life, but by reconceptualising their relationship. Therapy does not do the healing. Therapy installs the capacity for healing — for mentalising, for self-regulation, for relational engagement — that then develops through the patient's subsequent life experience. Shedler calls this a "catalytic" effect (J. Shedler, 2010). The catalyst is necessary but does not perform the reaction. What performs the reaction is life: safe relationships (Multiple posters (u/Longjumping_Cry709, 2023), creative expression (Bisson, 2024), vocational engagement (Bateman, 2008), community belonging (Multiple posters (u/Ok-Specialist1153, 2024), even the passage of time itself (Paris, 2020).

This explains a pattern visible across the client narratives: the turning points people describe are rarely therapeutic techniques. They are life events that became possible because of therapeutic work. Mac reconnected with his son (D. Murphy, 2009). ElishaAlison found a safe partner (Multiple posters (u/Longjumping_Cry709, 2023). Eger returned to Auschwitz (Eger, 2017). Sherlock demonstrated self-regulation capacity during his therapist's extended absence: "I'm going to be okay. You have given me the tools I need" (Heacock, 2022). In each case, the decisive moment occurred outside the consulting room — but it could not have occurred without what happened inside it.

15.4 The evidence for necessity

If therapy is catalytic rather than sufficient, the question becomes whether it is necessary. Can the same healing occur through life alone, without formal therapy?

Paris provides the strongest argument for a non-therapy pathway. He documents that borderline personality disorder remits gradually over approximately 24 years regardless of treatment — a natural course effect (Paris, 2020). Joseph's model identifies post-traumatic growth as facilitated by many means, not therapy alone (Joseph, 2012). And one Reddit poster reports having "come light years since therapy" through peer support and self-study when formal treatment was unaffordable (Multiple posters (sugnim, 2015).

But these examples do not straightforwardly establish that therapy is dispensable. Paris's natural course data describe diagnostic remission over decades, not the structural change or growth that the review defines as deeper endpoints. The peer support poster's trajectory, while impressive, operates without an independent assessment of what level of change was achieved. And the client narratives that describe healing without formal therapy almost always include some form of structured relational or reflective practice — 12-step programmes, peer communities, self-directed study — that shares functional features with therapy even if it lacks the label.

The stronger case for necessity comes from the neuroscience layer. Perry and Hambrick argue that the "repetitive patterned experience" required for neural reorganisation needs many more repetitions than weekly therapy provides — but they do not argue it can occur without therapeutic guidance (Hambrick, 2019). Fonagy's mentalising framework positions any mentalising relationship as contributing to capacity development — but the therapeutic relationship provides a uniquely structured context for mentalising about mentalising (G. Fonagy P., 2002). Winnicott is most precise: the endpoint of therapy is the capacity to use the environment — therapy is preparation for life, not the whole of healing (Winnicott, 1965).

15.5 Refined thesis

The evidence supports a refined thesis: healing from complex developmental trauma takes years. Therapy is necessary but not sufficient. The duration reflects not the inefficiency of therapy but the time required for both therapeutic installation of capacity and the developmental experience — exercised in life — through which that capacity matures.

This is a more honest and more interesting claim than "therapy takes years." It has different policy implications. The argument is not only for funding long-term therapy but for creating the conditions in which healing-through-life can occur: stable housing, safe relationships, community belonging, vocational opportunity, and the material security that allows someone to grieve without simultaneously fighting to survive (Multiple posters (u/Ok-Specialist1153, 2024; Multiple posters (u/WeeLittleSloth, 2021). Therapy without these conditions is a catalyst without a reaction medium. The conditions without therapy are a reaction medium without a catalyst. Both are necessary. Neither is sufficient.

The circularity remains unresolved — and should remain unresolved, because it reflects reality rather than methodological failure. Therapy and life are not competing explanations for healing. They are the two hands of a single process, distinguishable in analysis but inseparable in the lived experience of recovery.

16 Response to accelerated and brief models

If the working thesis is "healing takes years," the strongest challenge comes from models that demonstrate deep change happening faster. Thirteen sources across five counterpoint domains — EMDR intensive protocols, pharmacologically assisted therapies, intensive delivery formats, experiential dynamic therapies, and critiques of the long-duration position itself — present evidence that cannot be dismissed. This section engages each model at its strongest, acknowledges where the evidence genuinely challenges the thesis, and identifies where the evidence runs out. The goal is not rebuttal but honest engagement: a practitioner trained in EMDR, AEDP, or ISTDP should

recognise their model as fairly represented, even if they disagree with the conclusion.

The key move is endpoint divergence. As established in the synthesis, the field's apparent disagreement about duration largely dissolves once the endpoint level is specified. The counterpoint models overwhelmingly measure symptom reduction (loss of diagnosis, score changes on standardised instruments); the thesis concerns structural change (personality reorganisation, earned attachment security, mentalizing capacity). Both positions can be simultaneously correct about different things. But this framing must not become a device for defining the counterpoint out of existence. Where the evidence genuinely challenges the thesis — most notably in de Jongh's CPTSD data and the Helsinki convergence — it is acknowledged.

16.1 EMDR intensive: the strongest counterpoint

The EMDR evidence for complex presentations represents the most direct challenge to the long-duration thesis, because it addresses a population that overlaps substantially with the one defined in this review.

The strongest version of the argument. De Jongh, Roos, and El-Leithy (2024) present two landmark findings. The Voorendonk et al. (2020) study offered 8 days of residential intensive treatment — combining EMDR, exposure, psychoeducation, and physical activity — to 308 PTSD patients, 66% of whom met ICD-11 CPTSD criteria. Eighty-eight percent of CPTSD patients no longer met diagnostic criteria after 8 days (Jongh, 2024). The van Vliet et al. (2021) RCT compared 16 sessions of standalone EMDR with EMDR preceded by 8 sessions of stabilisation (STAIR) in 121 adults with PTSD from early childhood trauma. No difference between conditions: stabilisation added nothing, and only 3% of those initially diagnosed with CPTSD retained the diagnosis (Jongh, 2024). A single-case study further demonstrated resolution of both CPTSD and BPD in 10 sessions over 5 weeks (De Jongh, 2023).

Where this evidence genuinely challenges the thesis. These data directly address the review's defined population — adults with PTSD from early childhood trauma presenting with CPTSD symptoms — and the results are striking. The stabilisation finding is equally important: it challenges the phase-based model that has dominated complex trauma treatment for three decades (Herman, 1992; Cloitre et al., 2012). If teaching emotion regulation skills before trauma processing adds nothing to outcomes, then some portion of what passes for long-term therapy may be spent on preparations that do not help. The long-duration position must account for this finding rather than explain it away.

Where the evidence runs out. Three limitations are genuine, not rhetorical. First, the 8-day study lacked a control condition and did not examine long-term effects; we do not know about relapse (Jongh, 2024). Second, "no longer meeting diagnostic criteria" is not equivalent to structural

healing. The CPTSD diagnosis includes disturbances in self-organisation — affect dysregulation, negative self-concept, relationship difficulties — and losing the diagnosis implies improvement in these domains, but it is measured categorically (meets/does not meet criteria), not dimensionally. Whether personality integration, earned secure attachment, or the capacity for mutual recognition has occurred is not assessed. Third, and most importantly, Farina, Dimaggio, and Mosquera (2025) — experienced clinicians working with severe PTSD and personality disorders — respond to de Jongh's case: "such quick good outcomes cases are rare, and usually therapies are complex, difficult and time-consuming" (Farina, 2025). They raise the possibility that rapid responders may have "a different underlying structure, with more capacities for adaptive information processing and less avoidance" — suggesting that identical CPTSD diagnoses may mask fundamentally different structural depths (Farina, 2025). This maps directly onto the framework's Factor 2: patients with shallow structural injury respond rapidly; patients with deep structural disruption require more time, regardless of the modality employed.

What the long-duration position can learn. The EMDR literature forces precision about which components of long treatment are doing the work. If stabilisation exercises do not improve outcomes, then the case for extended treatment must rest on the therapeutic relationship, not on the stabilisation curriculum. The specific EMDR techniques for managing CPTSD complications — cognitive interweave for processing blocks, non-verbal processing for shame, normalisation of dissociation (De Jongh, 2023) — are clinically valuable regardless of treatment length.

16.2 Pharmacologically assisted therapies: MDMA and ketamine

The strongest version of the argument. The Mitchell et al. (2021) Phase 3 RCT tested MDMA-assisted therapy for severe PTSD (n=90): three 8-hour MDMA sessions plus manualized therapy over 18 weeks. Sixty-seven percent of the MDMA group no longer met PTSD criteria, versus 32% placebo (d=0.91). Crucially, 87% had developmental trauma histories, and participants with the dissociative subtype responded equally well (Mitchell, 2021). The theoretical claim is that MDMA pharmacologically creates the conditions — reduced fear, enhanced interpersonal trust, widened window of tolerance — that relational therapy builds over months or years. MDMA may "reopen an oxytocin-dependent critical period of neuroplasticity that typically closes after adolescence" (Mitchell, 2021), facilitating the processing of developmental fear memories in weeks rather than years.

Feder et al. (2025) tested a complementary approach: six ketamine infusions combined with five sessions of written exposure therapy for chronic PTSD (mean duration 18.1 years). Despite the tiny sample (n=13), the effect size was very large (d=1.9), with 54% losing their PTSD diagnosis at 12 weeks and 61.5% maintaining response at 6 months (Feder, 2025). The rationale — that ketamine "primes the brain" via enhanced neuroplasticity before psychotherapy — reframes the duration

question: if the rate-limiter is neurobiological capacity for change, pharmacological enhancement may compress what the brain would otherwise require years to achieve.

Gorman et al. (2020) added a further dimension: posttraumatic growth (PTG), measured alongside symptom reduction in pooled Phase 2 MDMA data (n=60). The MDMA group showed large PTG effects (g=1.14) — positive changes in self-perception, interpersonal relationships, and philosophy of life (Gorman, 2020). If MDMA facilitates genuine growth beyond symptom reduction, the endpoint divergence argument weakens, because PTG captures domains that overlap with what the long-duration position calls structural change.

Where the evidence genuinely challenges the thesis. The Mitchell trial's finding that dissociative patients responded equally well is particularly important, because dissociation is often cited as the feature that makes conventional treatment slow and difficult. If MDMA can bypass dissociative barriers pharmacologically, this challenges the assumption that dissociative defences require years of relational work to resolve.

Where the evidence runs out. The FDA rejected MDMA for PTSD in August 2024, citing functional unblinding, therapist misconduct in earlier trials, and insufficient evidence of sustained benefit (Mitchell, 2021). The Gorman PTG data required a 2024 correction removing participants treated by therapist pairs involved in ethical violations (Gorman, 2020). The Feder ketamine trial was open-label with no control group (n=13), and over half the participants were simultaneously receiving other psychotherapy (Feder, 2025). More fundamentally, the CAPS-5 "dissociative subtype" captures depersonalisation and derealisation — not the part-based structural dissociation (secondary and tertiary) described by Van der Hart, Nijenhuis, and Steele (2006) that characterises the most complex presentations. Both MDMA trials excluded DID and likely excluded current BPD, removing the most structurally disrupted patients before randomisation (Mitchell, 2021). The PTG finding is intriguing but the PTGI is a self-report measure of perceived growth that has been criticised for capturing changed self-narrative rather than observable structural change, and PTG and PTSD reduction were not significantly correlated in the data (Gorman, 2020).

What the long-duration position can learn. The neuroplasticity hypothesis is important regardless of whether pharmacological enhancement becomes clinically available. If chronic PTSD involves a neurobiological rate-limit on change that ketamine or MDMA can temporarily lift, this suggests that part of what long-term therapy achieves may be the gradual accumulation of small neuroplastic events that pharmacological agents could theoretically concentrate. The question for long-term therapy becomes: what does the therapeutic relationship provide that MDMA provides pharmacologically? If the answer is felt safety plus neuroplasticity, and MDMA provides neuroplasticity directly, then the irreducible contribution of the relationship is the felt safety — which returns the argument to Factor 0.

16.3 Intensive delivery formats: dose compression

The strongest version of the argument. Sciarrino, Warnecke, and Teng (2020) systematically reviewed 11 studies of empirically supported treatments delivered at intensive frequency (more than twice weekly). The weighted mean effect size was large (d=1.57), and the pooled dropout rate was only 5.51% — dramatically lower than the 18–39% reported for standard weekly delivery (Sciarrino, 2020). This finding reframes the duration question: if intensive formats achieve equal or better outcomes with radically lower dropout, then some portion of what appears to be "long-term therapy" may reflect the inefficiency of standard weekly delivery rather than the inherent complexity of the healing process.

Where the evidence genuinely challenges the thesis. The dropout finding is devastating for the long-duration position as typically practiced. If 30–40% of trauma patients leave standard weekly therapy, the effective dose for those who drop out is zero. Intensive formats may ensure that patients actually receive a therapeutic dose. The suggestion that standard weekly therapy systematically fails a large proportion of patients through its delivery format — not through insufficient duration — deserves serious engagement.

Where the evidence runs out. Eighty-one percent of the included studies were uncontrolled (Sciarrino, 2020). The populations were PTSD patients broadly defined; most studies did not include patients with comorbid personality disorders, severe dissociation, or suicidality. Self-selection is a concern: patients who commit to an intensive residential programme differ from those in standard care. And the outcome measures are uniformly symptom-based — whether dose compression works for structural change is unknown.

What the long-duration position can learn. Intensive formats may be particularly relevant for the safety prerequisite (Factor 0). The structured, contained environment of a residential intensive programme — where the patient is removed from stressors and immersed in a therapeutic context — may itself provide safety through structure rather than through extended relational time. This could explain why intensive formats work well for some CPTSD patients who would otherwise require months of stabilisation in outpatient settings. The long-duration position should consider whether hybrid models — intensive initial treatment to establish safety and begin processing, followed by extended relational therapy for structural consolidation — could serve patients better than either approach alone.

16.4 Experiential dynamic therapies: the follow-up advantage

The strongest version of the argument. Lilliengren et al. (2025) present a meta-analysis of 57 RCTs (n=4,330) of experiential dynamic therapies — ISTDP, AEDP, Affect Phobia Therapy, and Emotional Awareness and Expression Therapy. Against inactive controls, EDTs show large effects

(g=-0.96) that grow at follow-up (g=-1.11). Against active controls, effects are non-significant at post-treatment (g=-0.17) but become significant at follow-up (g=-0.40) (M. Lilliengren P., 2025). This follow-up advantage — the "sleeper effect" — is the key finding: EDTs may initiate a process during brief treatment that continues to produce change after therapy ends.

Fosha's AEDP contributes the theoretical mechanism. Her concept of "transformance" — the innate psychic force towards growth, positioned as the counterpart to resistance — and "meta-processing" — processing the experience of therapeutic change itself — propose that genuine transformation is accessible "from the first contact with the patient, including the most traumatised person" (Fosha, 2019). AEDP does not claim that healing completes rapidly; it claims that healing begins immediately when the therapist provides sufficient relational safety, rather than being deferred behind stabilisation phases.

Lilliengren et al. (2017) provide complementary data: ISTDP for GAD (n=215) produced significant symptom reduction in a mean of 8.3 sessions, with healthcare cost reductions sustained for 4 years (J. Lilliengren P., 2017). Higher in-session mobilisation of unprocessed complex emotions predicted greater change — evidence that emotional breakthrough, not duration, may be the active ingredient.

Where the evidence genuinely challenges the thesis. The EDT follow-up advantage is genuinely important for the thesis, because it suggests that brief treatment can install something durable — not just borrowed skills that erode, but a process that grows. If this is confirmed for structural endpoints, it would narrow the thesis considerably. It is also compatible with the thesis: if brief EDT initiates a developmental process that continues for years, the elapsed-time factor (Factor 3) applies — the healing still takes years, but less of that time is spent in formal therapy.

Where the evidence runs out. The EDT meta-analysis covers mood, anxiety, personality, and somatic conditions — not complex developmental trauma with dissociative features (M. Lilliengren P., 2025). Heterogeneity is substantial ($I2 > 75\%$). The ISTDP literature acknowledges that patients with "fragile character structure, somatization, and severe depression" require more treatment (J. Lilliengren P., 2017). AEDP-specific outcome research remains nascent — Fosha references "about five research projects currently in the works" but these are not yet available (Fosha, 2019). The population gap is significant: the emotional mobilisation that EDT models rely on requires a capacity to access and tolerate intense affect — precisely the capacity that is impaired in complex developmental trauma with dissociative organisation.

16.5 Paris's access argument: the ethical counterpoint

The strongest version of the argument. Paris (2020) makes the case that long-term therapy for personality disorders has never been shown to be superior to time-limited interventions, that most improvement occurs by 20 weeks with asymptotic flattening (citing Howard's 1986 dose-effect model),

and that much of BPD recovery is naturalistic — the McLean longitudinal study shows gradual remission over 24 years regardless of treatment (Paris, 2020). His ethical argument is the most original contribution: at $220/week for DBT ($10,000+/year), long-term therapy is inaccessible to most personality disorder patients, who disproportionately have "limited education and low-level jobs." Clinicians have "an ethical responsibility to help as many people as possible, and doing so means keeping waiting lists short" (Paris, 2020).

Where the evidence genuinely challenges the thesis. The access argument cannot be dismissed. If years of specialised therapy are necessary for structural healing, then the field bears responsibility for making this accessible. A recommendation that healing takes years, without a commitment to funding structures that match, is clinically honest but structurally unjust. Paris is right to place the burden of proof on those who advocate expensive, lengthy treatments.

Where the evidence runs out. Paris conflates BPD with the population defined in this review. His evidence is primarily about BPD symptom reduction, not structural change in dissociative organisation. The dose-effect curve (Howard, 1986) was derived from predominantly neurotic populations (Paris, 2020). The stepped-care model he advocates may work for BPD symptom management, but the question of whether it addresses structural dissociation — the deeper injury that characterises the most severely affected subset of the population — is not engaged. His framing of the endpoint as symptom reduction rather than structural personality change means his "diminishing returns" argument applies to a narrower definition of healing than the one this thesis proposes. Paris does not discuss what happens to patients who do not respond to brief treatment and cannot access the longer treatment his stepped-care model recommends for non-responders.

16.6 Farina's rapid-rare framework and Tilson's evidence critique

Two sources complicate the picture from different angles. Farina et al. (2025) represent the clinical middle ground: experienced trauma clinicians who acknowledge that rapid outcomes occur but insist they are "rare" in severe CPTSD with personality disorder comorbidity (Farina, 2025). Their key contribution is the hypothesis that similar diagnostic presentations mask "different underlying structures" — some patients carry the CPTSD label with relatively preserved adaptive capacity, while others carry it with pervasive developmental disruption. This maps precisely onto the framework's Factor 2 (structural depth) and explains why the same treatment can produce rapid change in one patient and require years in another.

Tilson (2025) mounts a different challenge: not against the duration claim per se, but against the neuroscientific scaffolding frequently cited to explain why healing takes time. Her critique of the triune brain model, polyvagal theory, and unreliable fMRI evidence dismantles specific frameworks invoked by van der Kolk, Porges, and others (Tilson, 2025). If the neurobiological rationale for slow

developmental change rests on debunked science, the explanatory mechanism weakens. However, the clinical convergence holds independently: nine independent theoretical traditions observe slow structural change without depending on the triune brain model for their observations. Tilson's critique also overreaches: she conflates van der Kolk's popular science with the entire body-based therapy field, and her endorsement of CBT and Prolonged Exposure as alternatives does not address whether these approaches achieve structural change in complex developmental trauma populations or merely symptom reduction — the same endpoint divergence that limits the other counterpoint models.

16.7 Resolution: the four-factor framework

The apparent contradiction between these counterpoint models and the long-duration thesis resolves through the four-factor framework proposed in the synthesis.

The counterpoint evidence is strongest at Level 1 (symptom reduction) and weakest at Level 3 (structural change). EMDR produces diagnostic remission in 8 days; MDMA eliminates PTSD diagnosis in 18 weeks; intensive formats achieve d=1.57 in 1–3 weeks. These are real achievements with real clinical value. But none of these models measure personality reorganisation, earned attachment security, or integration of dissociated self-states — the endpoints that define structural healing for patients with deep developmental injury.

The resolution is not "the counterpoint is wrong" but "the counterpoint is right about a different question." The models reviewed here answer "how long does symptom reduction take?" with precision and rigour. The thesis answers "how long does structural change take?" with convergent but less rigorous evidence. Both answers matter. For a patient whose primary need is diagnostic remission — a reduction in flashbacks, hyperarousal, and avoidance that allows them to function — concentrated evidence-based treatment may be entirely sufficient, and recommending years of therapy would be harmful. For a patient whose primary need is to reorganise a personality structure built around dissociation, terror of intimacy, and the absence of a coherent self — the patient who can pass a symptom inventory while remaining unable to sustain a loving relationship — symptom reduction is necessary but radically insufficient.

Two findings resist this neat resolution and must be acknowledged honestly. First, de Jongh's CPTSD data. If 88% of CPTSD patients lose their diagnosis in 8 days, and CPTSD includes disturbances in self-organisation (not just core PTSD symptoms), then the endpoint divergence argument may be too clean. CPTSD diagnostic remission is not mere symptom reduction — it implies improvement in affect regulation, self-concept, and relational functioning. The counterargument is that the ITQ measures these domains categorically and briefly, and that falling below diagnostic threshold is not the same as achieving integration. But the honest position is that this remains an empirical question.

What the field needs — and does not have — is a study that measures both diagnostic remission and structural endpoints (personality organisation, attachment security, dissociative integration) in the same CPTSD population, at the same time points, comparing brief and extended treatments. Until that study exists, the endpoint divergence argument is a hypothesis, not a conclusion.

Second, the Helsinki convergence. Psychoanalysis (646 sessions) and long-term psychodynamic therapy (232 sessions) produced equivalent outcomes at 10-year follow-up (M. Lilliengren P., 2025). If tripling the dose does not triple the outcome, then the thesis must specify where the threshold lies — how many sessions are "enough" for structural change to become self-sustaining, and when additional therapy represents diminishing returns rather than deeper healing. The elapsed-time factor (Factor 3) provides a partial answer: beyond a sufficient therapeutic dose, the patient needs life-time rather than therapy-time for installed capacity to develop. But the framework does not yet specify what that sufficient dose is.

A further finding complicates both positions in the duration debate. The counterpoint argues that less treatment can be sufficient; the long-duration position implies that more treatment is generally better. But the Bateman and Fonagy (2008) TAU data demonstrate that more treatment without a coherent developmental model produced worse outcomes: TAU patients received significantly more services during follow-up yet 87% still met BPD criteria, compared with 13% in the MBT group (Bateman, 2008). The authors concluded that "quantity of treatment may not be a good indicator of improvement and may even prevent patients from taking advantage of felicitous social and interpersonal events." The implication is that the duration debate should not be framed as "more versus less" but as "coherent versus incoherent." A well-designed brief intervention with a clear developmental model may outperform years of unfocused treatment. This does not weaken the case for long-term therapy — it specifies that long-term therapy must be purposeful, developmentally informed, and structured around a model of change, not simply more of the same.

These are genuine limits, not rhetorical concessions. The thesis survives the counterpoint evidence, but it is refined by it — sharpened, qualified, and made more precise about who needs what, for how long, and measured by which endpoint.

17 Access, privilege, and justice

17.1 The structural injustice

If the conclusion of this review is that healing from complex developmental trauma requires years of relationally-oriented therapy, then the review is advocating for something most people on earth cannot access. This is not an unfortunate implication of the thesis. It is a core tension within it, and a review that argues for long-term therapy without confronting who can actually receive it is

ethically incomplete.

The injustice is structural, not incidental. Developmental trauma is concentrated in populations with the fewest resources: intergenerational poverty, marginalised communities, families fractured by adversity. Paris states the problem with characteristic directness: DBT costs $220 per week, over $10,000 per year; "most PD patients have limited education and low-level jobs" (Paris, 2020). The population most damaged by developmental trauma is the population least likely to access the treatment this review says they need. Every duration estimate in this review — 3 to 5 years for phase-oriented treatment (Herman, 1992; Van der Hart, 2006), 5 to 7 years for personality organisation change (Lindfors, 2019), decades for full post-traumatic growth (Eger, 2017; Walker, 2013) — carries an implicit price tag that excludes most of the relevant population.

Spring names this failure most plainly. Survivors who have been in therapy for 10, 20, or 30 years without recovery are not failing — society has failed to "adequately provide public transport to enable people to head north" (Spring, 2020b). The metaphor is precise: recovery is a direction (northward), and therapy is the transport system. Those who can afford private cars — frequent sessions with specialist clinicians over years — may travel steadily. Those who depend on public transport find the routes infrequent, the coverage patchy, and the journeys interrupted by forced transfers. Spring counts herself among "the lucky unlucky ones," distinguishing her situation from "the unlucky unlucky ones" who lack even the resources she has (Spring, 2021).

17.2 The evidence: silence and ground truth

The access extraction reveals a striking pattern: the majority of sources across all six layers do not address who can access the treatment they describe. The silence is itself a finding.

The psychoanalytic layer is the most resource-intensive and the most silent. Kohut's model assumes four to five sessions per week for years (Kohut, 1984). Winnicott's developmental regression requires sustained high-frequency contact (Winnicott, 1965). Bateman and Fonagy's MBT programme requires five-day-per-week partial hospitalisation — a level of provision rarely available even within the NHS (Bateman, 2008). None of these sources addresses who can afford this. The neuroscience layer is entirely silent on access: seven sources, zero mentions (Group, 2010; Hambrick, 2019; Lanius, 2011; Ogden, 2006; Porges, 2022; Samson, 2023; Schore, 2000). Mechanism-focused research abstracts away the patient's material reality.

The ground truth comes from clients. Reddit posters describe the lived experience of access failure with an immediacy absent from the clinical literature. One poster describes $4,000–6,000 deductibles and "500 work-arounds for being poor" (Multiple posters (sugnim, 2015). Another describes being on Medicaid with access to only two therapists — one female, one male — and no further choice (Multiple posters (u/Longjumping_Cry709, 2023). A third tolerates racism,

bigotry, and homophobia from available therapists because there are no alternatives (Multiple posters (u/wickeddude123, 2023). A fourth receives monthly sessions — "pure hell" — because affordability, not clinical need, determines frequency (Multiple posters (Skywatcher, 2020).

These are not anecdotes decorating a structural argument. They are the structural argument expressed in lived experience. Access does not merely determine whether someone receives therapy. It determines the dose. When affordability permits only monthly sessions rather than weekly ones, the patient receives a subtherapeutic frequency that may extend duration, undermine the relational continuity on which structural change depends, and ultimately produce the treatment failures that are then attributed to the intractability of the condition.

Two cases illustrate the inverse: what happens when access barriers are removed. Foo's therapeutic turning point — the most transformative relationship in the memoir layer — was enabled by $6,400 of pro bono therapy from Dr. Ham (Foo, 2022). Mac accessed 160 sessions through icap, a UK charity — a level of provision unavailable through standard pathways (D. Murphy, 2009). In both cases, healing occurred because the access barrier was bypassed through charity, not because the system provided what was needed. Pro bono provision is not a solution. It is an indictment.

17.3 The cost-effectiveness trap

Even when long-term therapy demonstrably works, it may not satisfy standard health-economic criteria. Koeser et al. found that long-term psychodynamic psychotherapy costs approximately £5,500 in direct treatment costs and produces a cost per quality-adjusted life year of approximately £33,000 — above the NICE threshold of £20,000 per QALY (Koeser, 2023). The authors argue that the time horizon was too short to capture the full benefit: the sleeper effect, documented extensively in this review, means that gains continue to accrue for years after treatment ends. If the economic evaluation had extended to ten or fifteen years rather than four, the cost-effectiveness ratio would shift. But this remains unproven, and the institutional reality is that treatments exceeding cost-effectiveness thresholds face funding barriers regardless of clinical efficacy.

The cost-effectiveness framework itself may be structurally unsuited to evaluating treatments for conditions produced by developmental adversity. A QALY measures health-related quality of life in standardised units. But the gains from structural change — earned attachment security, mentalising capacity, the ability to parent without transmitting trauma intergenerationally — extend beyond the individual patient and beyond the timeframe of any economic evaluation. The intergenerational transmission argument is rarely quantified, but it is clinically evident: Wylie documents that nearly 80% of surveyed children exposed to interpersonal trauma had experienced multiple or prolonged events (Wylie, unknown), suggesting that untreated parental trauma is a significant risk factor for the next generation's developmental injury. A cost-effectiveness analysis that captures only the

treated patient's QALYs systematically undervalues treatments that interrupt intergenerational cycles.

17.4 Partial solutions: what the evidence suggests

The review cannot resolve the access problem, but it can identify directions from the evidence that reduce, rather than merely describe, the gap.

Stepped care models. Links's Good Psychiatric Management provides the most explicit access-oriented design in the evidence base (Links, 2015). GPM was developed as a generalist-accessible, first-line treatment to reduce the specialist bottleneck: primary care providers deliver structured psychoeducation and case management, reserving specialist referral for patients who do not meet milestones at defined intervals (6 weeks, 6 months, 12 months, 18 months). The model explicitly addresses Paris's concern about ethical responsibility to keep waiting lists short. It does not resolve the duration problem — patients who need structural change still need years — but it ensures that the first tier of care does not require the scarce specialist resources that bottleneck the system.

Intensive formats. Sciarrino's systematic review of intensive treatment delivery provides the strongest access-adjacent finding: concentrated formats (daily sessions over one to three weeks) achieve large effect sizes ($d = 1.57$) with dramatically lower dropout — 5.51% versus 18–39% in standard weekly delivery (Sciarrino, 2020). Lower dropout is itself an access intervention. If the population most affected by developmental trauma is the population most likely to drop out of standard treatment, then a format that retains them in treatment addresses an access barrier that is relational and psychological, not only financial. The intensive format's containing structure — removing the patient from their stressful environment, immersing them in a therapeutic milieu — may itself provide the safety prerequisite (Factor 0 from the synthesis) through structure rather than through extended time. The cost per episode may be comparable to or lower than years of weekly sessions, though residential programmes introduce their own barriers (travel, childcare, employment disruption).

Team-based approaches. MBT's team model (Bateman, 2008) and Farina's multi-clinician recommendation (Farina, 2025) distribute the therapeutic burden across providers rather than concentrating it in a single specialist relationship. This has workforce implications: training many generalists to provide adequate relational care may be more sustainable than training a few specialists to provide optimal care. Fisher's workbook model (Fisher, 2021) pushes in this direction by providing structured psychoeducation as a shared resource within the therapeutic relationship — though Fisher explicitly warns against using it outside that relationship, maintaining the relational frame.

The therapy-plus ecology. The therapy-versus-life analysis (Section 4) carries access implications. If healing is co-produced by therapy and life, then interventions that improve the conditions of

life — stable housing, safe relationships, vocational opportunity, community belonging — are not supplementary to therapy but integral to healing. This reframes the access question. The challenge is not only to make years of specialist therapy available but to create environments in which the capacity installed by therapy can be exercised. Spring's distinction between "heading north" and arriving at a destination applies here: a system that provides therapy without addressing the material conditions that prevent patients from using their developing capacities outside the consulting room is providing a compass without clearing the road.

Pharmacologically enhanced therapy. MDMA-assisted psychotherapy (Mitchell, 2021) and ketamine-enhanced exposure therapy (Feder, 2025) achieved large effects in condensed timeframes. If pharmacological agents can compress the processing that otherwise requires years, they represent a potential access intervention — though regulatory barriers, cost, and the early state of the evidence limit near-term applicability. Heacock's account of Razvi's trajectory is instructive: conventional somatic therapy took 4–5 years to address dissociative trauma; MDMA accessed the dissociative core in a single session (Heacock, 2019). If this generalises, pharmacologically enhanced formats could dramatically reduce the total treatment time for the most structurally damaged patients — the patients for whom access barriers are most devastating.

17.5 Forced termination as iatrogenic harm

The access problem extends beyond the inability to begin treatment. System-imposed endings actively undo therapeutic gains by terminating relationships that ARE the treatment mechanism. Hauser's (2022) Finella case illustrates this precisely: 48 sessions of fragile-process work — the slow, painstaking construction of therapeutic safety with a severely avoidant client — were terminated by agency constraints, not by clinical judgment (Hauser, 2022). One Reddit poster describes losing a therapist after 10 years and over 900 sessions — a relationship that constituted the primary attachment bond — due to circumstances outside the patient's control (Multiple posters (u/Ok-Specialist1153, 2024). Sohal's (2022) finding that IAPT's standard 6–8 session model is structurally insufficient for suicidal clients — with 15 sessions as the minimum for significant change — quantifies the gap between what the system provides and what the evidence requires (Sohal, 2022).

These are not merely access failures. They are iatrogenic harms: the system creates a therapeutic relationship, allows the patient to become dependent on it (an appropriate and necessary dependency for patients with attachment disruption), and then severs it according to administrative rather than clinical logic. For patients whose core injury is relational loss, forced termination recapitulates the original trauma. The distinction matters: failing to provide treatment is neglect; providing and then withdrawing it is harm.

17.6 What remains unresolved

The review cannot resolve the access problem because the problem is political and economic, not clinical. The evidence tells us what people need. It does not tell us how to fund it, how to distribute it, or how to reconcile the timescale of structural healing with the timescale of insurance coverage and health-system budgets.

Three tensions remain genuinely unresolved. First, the review studies Western individual psychotherapy almost exclusively. Non-Western healing traditions, community-based approaches, and peer-support models are absent from the evidence base — not because they are irrelevant but because the search strategy was not designed to capture them. This is a limitation with justice implications: the review may inadvertently reinforce the assumption that individual therapy is the only valid pathway, when community-based practices may offer more accessible routes to the relational and regulatory capacities that constitute structural change.

Second, the review does not quantify costs, coverage, or access statistics across health systems. It identifies the structural injustice but cannot estimate its magnitude.

Third, the reflexivity that Spring and Davitt model — acknowledging one's own privilege in having accessed treatment (Bisson, 2024; Spring, 2021) — applies to the review itself. The evidence base is shaped by whose voices reach publication. Memoirs are written by people who have recovered enough to write. Reddit posts are written by people with internet access and sufficient English literacy. Clinical case studies are conducted with patients who remained in treatment long enough to be studied. The voices most absent from this review — those who needed long-term therapy, could not access it, and did not recover — are precisely the voices that the access argument most urgently requires. Their absence is the most consequential form of survivorship bias in the evidence base.

17.7 The obligation

The thesis — that healing from complex developmental trauma takes years — carries an ethical obligation. If the field knows that years of treatment are needed, and if the field knows that most affected individuals cannot access years of treatment, then silence on this gap is complicity with a structural injustice. The review does not claim to know how to close the gap. It claims that the gap must be named, that the naming is not optional, and that any clinical recommendation about therapy duration that omits the access question is incomplete.

Spring is right: the problem is not that survivors fail to heal. The problem is that society fails to provide the transport (Spring, 2020b).

18 Methodological limitations

This review draws on 89 sources across six epistemological layers, from peer-reviewed meta-analyses to Reddit threads and published memoirs. Each layer contributes distinct evidence, and each introduces distinct vulnerabilities. The limitations below follow a consistent structure: the weakness, how the design mitigated it, what remains unmitigated, and how it feeds back into the review's conclusions. Limitations that have their own dedicated discussion sections (survivorship bias, dose vs. time, therapy vs. life, inaccessible sources) are summarised here and cross-referenced.

18.1 1. Not a systematic review

The weakness. The search is structured — iterative snowball, per-layer source budgets, documented search terms and databases — but it is not exhaustive. It is not replicable in the PRISMA sense. Another reviewer using the same starting sources would likely follow a different snowball path and end up with a different source set, because the process is path-dependent: each source's reference list shapes which sources are encountered next.

How we mitigated it. The CIS methodology was chosen precisely because it rejects exhaustive screening in favour of iterative, question-driven search (Dixon-Woods et al., 2006). Search terms, key authors, and databases are documented per layer. The iterative protocol is logged with extension documentation and plan updates. Search results summaries (identified, screened, included, excluded, inaccessible) are tracked per layer. Saturation notes document why searching stopped in each layer.

What remains unmitigated. A systematic search might have found sources that the snowball process missed. The path-dependency means that an early miss could cascade — a foundational text not encountered early would leave its entire citation network invisible. The review cannot claim comprehensiveness. However, the convergence across six independent layers suggests that the central findings are robust to individual source omissions: the endpoint-duration nexus, the safety prerequisite, and the developmental mechanism are each supported by sources from at least four layers.

Feed-back. Late-found sources (those added in the final iteration) did not change the framework's structure. They enriched existing constructs — Ogden deepened the developmental mechanism; Spring's third essay confirmed the episodic trajectory — but did not introduce new dimensions. This suggests the snowball process converged adequately for the review's purposes, though it cannot exclude the possibility that a missed tradition (e.g., systemic or family therapy approaches to complex trauma) would modify the framework.

18.2 2. Heterogeneous evidence — no formal weighting

The weakness. The review combines peer-reviewed meta-analyses (e.g., Leichsenring & Rabung, 2011; Lilliengren et al., 2025), single-case studies (D. Murphy, 2009), psychoanalytic theory (Kohut, 1984), practitioner blog posts (Auer, unknown), Reddit threads (Multiple posters (u/Longjumping_Cry709, 2023), and published memoirs (Foo, 2022). These sources have radically different epistemic status. A practitioner's essay and a 10-year naturalistic study might both appear in the synthesis, but they carry different weight for different claims.

How we mitigated it. Quality notes in each source annotation assess methodological strengths and limitations narratively. The synthesis method describes context-dependent weighting: Layer 2 studies (naturalistic outcome data) carry the most weight on duration claims; Layer 6 memoirs and Layer 5 client narratives carry the most weight on trajectory shape and the lived experience of healing; Layer 4 practitioner essays carry weight on clinical wisdom and treatment recommendations. Cross-layer triangulation explicitly identifies convergence and divergence — a finding supported by multiple layers is substantially stronger than one supported by a single layer. The review's central claims are all supported by at least four of six layers.

What remains unmitigated. The weighting is informal and author-dependent. There is no inter-rater check on quality assessment. The most consequential vulnerability is that the duration claims for structural change (Level 3 endpoints) rest disproportionately on naturalistic studies with mixed populations (Helsinki, TADS, Munich) rather than on studies of the defined population (complex developmental trauma). Only the Brand et al. (2019) TOP DD study and the Griffiths (2025) phase-oriented review directly address the population at the endpoint level most relevant to the thesis. This is thin empirical ground for the review's strongest claims.

Feed-back. The synthesis addresses this directly by separating duration estimates by endpoint level and noting where empirical evidence is thin. The claim "structural change takes years" rests on convergent clinical observation from nine theoretical traditions, supported by sleeper effect data and long-term follow-up studies — but the populations studied are not identical to the population defined in this review. Future research with long-term structural measures in defined complex trauma populations would substantially strengthen or challenge the framework.

The ecophenotype concept deepens this concern. Samson and Teicher (2023) present evidence that maltreatment-related psychiatric presentations may be neurobiologically distinct from non-maltreatment presentations carrying identical diagnoses — a "maltreatment-related ecophenotype" with different neural signatures, different treatment responses, and potentially different developmental trajectories (Samson, 2023). If confirmed, this means the entire evidence base for treatment duration was built on mixed populations that conflate two neurobiologically distinct subtypes. Duration estimates derived from these mixed samples may not apply to the maltreatment subtype specifically

— the very population this review foregrounds. The true duration of healing for the ecophenotype subtype remains unknown.

18.3 3. Single-author review

The weakness. All source selection, annotation, data extraction, and synthesis is performed by a single author. There is no inter-rater reliability for any step. The author's theoretical orientation (psychodynamic, relationally informed), clinical experience, and personal relationship to the topic shape every interpretive decision. The working thesis — that healing takes years — was declared at the outset, creating a structural risk of confirmation bias.

How we mitigated it. The counterpoint layer (Layer 3) is a structural check against confirmation bias, comprising 13 sources that collectively present the strongest available case for brief or accelerated treatment. Cross-cutting observations force engagement with methodological weaknesses in the evidence base. The surprises file actively records disconfirming evidence — including the Brand paradox (higher dissociation predicting faster improvement), the Helsinki convergence (tripling dose producing no additional benefit), and the intensive format dropout data (5.51% vs. 18–39%). The audit trail makes every interpretive step traceable from discussion claim back to source quote. The execution log captures meta-commentary from AI-assisted search and annotation agents, providing a partial external perspective on interpretive choices.

What remains unmitigated. Structural safeguards reduce but cannot eliminate single-author bias. The declared working thesis means that the review should be read as an honestly advocated position, not as a neutral survey. A reviewer starting from the counterpoint position — that brief, evidence-based treatment is sufficient for most CPTSD presentations — might weight the same 89 sources differently and arrive at different conclusions, particularly regarding the necessity of the structural change endpoint for the defined population.

18.4 4. Definitional breadth — the endpoint problem

The weakness. The working definition of healing — *the capacity to live with agency, relational openness, and tolerable selfhood, rather than organised around survival* — is deliberately broad. This breadth is both the review's central finding and its most significant methodological vulnerability. Different sources define healing at different endpoint levels, and their reported durations are only comparable to the extent that the definitions align.

How we mitigated it. The endpoint-duration nexus emerged as the review's primary finding: the five endpoint levels (symptom reduction, functional improvement, structural change, growth, directional recovery) each have characteristic timelines and evidence bases. The extraction file tracks how each source defines its endpoint. The synthesis explicitly separates duration claims by

level, making the comparison transparent. The review's working definition maps approximately to Level 3 (structural change), which is stated plainly.

What remains unmitigated. The synthesis argues that the field's disagreement about duration is an artefact of unstated endpoint definitions. But this argument itself cannot be tested within the review's design — it is an interpretive framework applied to heterogeneous evidence, not a hypothesis tested against data. It is possible that the endpoint levels are less clearly nested than the framework suggests, or that the boundaries between levels are more permeable than the framework implies. Fosha's AEDP work, which describes within-session transformation that may constitute genuine structural change (Fosha, 2019), represents the strongest challenge to the clean separation of endpoint levels.

18.5 5. English-language and Western bias

The weakness. All 89 sources are rooted in English-language, Western psychotherapeutic traditions. Non-Western healing traditions, community-based approaches, indigenous models of recovery, and cultures with different conceptualisations of trauma and selfhood are entirely absent. The review's conclusions apply only to Western, individual psychotherapy as practised in anglophone clinical contexts.

How we mitigated it. Barely. The limitation is acknowledged but not addressed in the search design. The access and justice discussion touches on who can access long-term treatment, but this is a socioeconomic rather than cultural analysis. The humanistic-existential sublayer (Layer 1c) represents the closest approach to a non-medical model but remains Western.

What remains unmitigated. This is a genuine boundary on the review's claims. The developmental mechanism — transmuting internalisation, mentalizing development, autonomic reorganisation — is described in terms of Western psychological models. Whether structural change from complex developmental trauma follows similar timelines in non-Western healing contexts is an empirical question this review cannot address. The emphasis on individual therapy may itself be a cultural assumption: community-based and relational healing models might operate on different timescales through different mechanisms. This limitation is not merely academic — the defined population (people with complex developmental trauma) exists in every culture, and limiting the evidence base to Western individual psychotherapy excludes the majority of the world's approaches to the problem.

18.6 6. Contested neuroscience underlying the mechanism argument

The weakness. The review's argument for why structural change takes years rests partly on neuroscientific frameworks — specifically polyvagal theory (Porges, 2022), the neurosequential model

(Hambrick, 2019), and hierarchical brain processing models (Ogden, 2006) — that are scientifically contested. Tilson (2025) mounts a detailed critique of two frameworks frequently cited in the trauma therapy literature: polyvagal theory, which she describes as "untenable or highly implausible" per Grossman's critique, and the triune brain model, which was "disproved in the 1970s" (Tilson, 2025). If these frameworks are scientifically unfounded, the specific neurobiological explanation for the developmental mechanism's pace weakens.

How we mitigated it. The synthesis addresses this honestly: the clinical convergence across nine independent theoretical traditions holds independently of any particular neuroscientific explanation. Spring, Heacock, Fisher, Bisson, and Razvi all observe slow developmental change without depending on polyvagal theory or triune brain theory for their observations. The review separates the observational claim ("structural change takes years") from the explanatory claim ("this is because of hierarchical brain architecture and vagal regulation"), noting that the former is more robustly supported than the latter.

What remains unmitigated. The separation between observation and explanation is not as clean as the review might suggest. If the hierarchical processing model is unfounded, then the specific clinical prediction that bottom-up interventions must precede top-down interventions — a prediction central to Fisher's "slower is faster" principle and to the phase-oriented treatment model — loses its neurobiological rationale. The clinical observation that stabilisation-first approaches often work well for complex trauma populations could be explained by other mechanisms (e.g., operant conditioning, therapeutic alliance development, or simply the passage of time). The review's mechanism argument is therefore more tentative than its endpoint-duration findings.

18.7 7. Single-research-group dependency in the humanistic-existential layer

The entire humanistic-existential evidence base (Layer 1c) derives from one interconnected research network. Murphy, Joseph, and Elliott are collaborators and co-authors whose work on person-centred and emotion-focused therapy for trauma builds cumulatively on shared theoretical commitments (Joseph, 2012; D. Murphy, 2009; E. Murphy D., 2019). This does not invalidate their findings — the case studies and the post-traumatic growth model are internally coherent and clinically detailed. But it limits their evidentiary weight: the humanistic-existential case for long-term therapy with complex trauma rests on a single research tradition rather than on independent converging lines of evidence. Independent replication from outside this network would substantially strengthen the case.

18.8 8. Source selection when results exceeded budget

The weakness. When search returned more results than the source budget allowed, sources were selected using tiered relevance criteria. This selection — even with transparent criteria — introduces a bias toward sources the reviewer judged most relevant, which may reinforce the working thesis.

How we mitigated it. Tiered relevance criteria were applied consistently across layers. Human-in-the-loop decision-making governed borderline sources. Named priority sources from the methods files were always included regardless of triage. All excluded sources are documented with reason codes, enabling a reader to evaluate whether exclusion decisions were defensible.

What remains unmitigated. The triage criteria themselves embed theoretical assumptions about what is relevant. A reviewer oriented toward behavioural or cognitive traditions might have prioritised different sources from the same search results — potentially finding more evidence for brief treatment effectiveness and less for the developmental mechanism.

18.9 9. Cross-cutting limitations (with dedicated sections)

The following limitations are substantive enough to warrant their own discussion sections:

- **Survivorship bias** (§2) — all six layers select for people who stayed in therapy long enough to report. Only 10 of 89 sources report dropout data in any form. The treatment-engagement paradox is unresolved: the interpersonal difficulties that define the population are the same factors that predict dropout (Freixo Ferreira, 2023; Knekt, 2021). A reverse survivorship effect compounds this problem in Layer 5 (client narratives): as one poster observed, "Maybe, just maybe, the majority of people who heal leave these places, and so it looks, on the surface, like no one heals from this" (Multiple posters (u/Longjumping_Cry709, 2023). Online trauma communities systematically over-represent those still struggling and under-represent those who have recovered, making healing appear rarer and slower than it may be.
- **Dose vs. elapsed time** (§3) — the conflation of session count and calendar years pervades the literature. The Helsinki study's 646 sessions of psychoanalysis versus 232 sessions of psychodynamic therapy produced equivalent 10-year outcomes (Lindfors, 2019), suggesting that elapsed time may matter more than dose beyond a threshold — but disentangling the two requires studies designed for this purpose, which do not yet exist.
- **Therapy vs. life** (§4) — the attribution of change to therapy versus non-therapeutic factors (relationships, employment, life events, ageing, natural recovery) cannot be resolved with the available evidence. Paris (2020) notes that BPD remits gradually over 24 years regardless of treatment. Client narratives repeatedly describe life events as more decisive than any therapeutic technique (Multiple posters (u/Longjumping_Cry709, 2023).

- **Inaccessible sources** (§9) — four sources identified as directly relevant remain inaccessible, including two foundational texts (Herman, 1992; Van der Hart et al., 2006). The impact is assessed in detail in the dedicated section.

19 Implications

The synthesis proposes that the duration of healing from complex developmental trauma is determined by a safety prerequisite interacting with three factors: the endpoint level sought, the structural depth of the injury, and the contribution of elapsed time to post-therapeutic development. If this framework is even approximately correct, it generates specific implications for clinical practice, training, policy, research, and — most importantly — the people who live with the consequences of complex developmental trauma.

19.1 Clinical practice

The framework's most immediate clinical implication is that therapists working with complex developmental trauma need to assess, at minimum, three things before they can give a patient any honest answer about expected duration.

First, assess the endpoint level. What does the patient want from treatment? Symptom reduction (fewer flashbacks, lower distress scores) is achievable in weeks to months for many patients, including some with CPTSD diagnoses, through concentrated evidence-based treatment (Jongh, 2024; Sciarrino, 2020). Functional improvement (holding relationships, maintaining employment, reduced crises) takes months to 1–2 years (Links, 2015). Structural change (personality reorganisation, mentalizing capacity, integration of dissociated parts, earned attachment security) takes years (Griffiths, 2025; Leichsenring, 2013; Lindfors, 2019). These are not competing options but nested levels of the same process. The clinical task is to name them explicitly, help the patient identify which level they seek, and set expectations accordingly.

Second, assess the structural depth of the injury. Not all complex developmental trauma produces the same degree of structural disruption. The structural dissociation model provides the most precise vocabulary: primary structural dissociation is less entrenched than secondary or tertiary (DID) (Van der Hart, 2006). Farina et al. (2025) identify the key clinical variable: patients with "different underlying structures" may carry the same CPTSD diagnosis but differ fundamentally in the depth of developmental damage. A patient with primary structural dissociation and adequate pre-existing adaptive capacity may genuinely achieve structural change in months. A patient with tertiary structural dissociation and pervasive developmental deficits may require years for the same endpoint level.

Third, establish safety first — and recognise that safety establishment IS therapy. The evidence for safety as clock-starter is among the strongest findings of this review, supported by 35+ entries across all six layers. The clinical principle is straightforward: the therapeutic clock does not start at the first session but at the establishment of environmental, relational, and autonomic safety. Fisher's "slower is faster" captures this from the clinician's side (Fisher, 2019, 2021). Pre-safety therapy time is not wasted time — building trust, stabilising the nervous system, and creating a foundation for processing are essential therapeutic work — but it should be understood as preparatory rather than as evidence that therapy is "not working." The safety prerequisite also means that therapy cannot produce structural change while the environment remains actively traumatising (Multiple posters (u/WeeLittleSloth, 2021).

Fourth, anticipate modality evolution rather than committing to a single protocol. The data consistently show that modality-switching is an adaptive feature of long-term recovery, not a sign of treatment failure. The extraction data reveal four distinct multi-modality patterns: within-framework multimodality, where a coherent model deploys different techniques at different phases (Herman's phase model, MBT's combination of individual, group, and art therapy within a mentalizing framework) (Bateman, 2008; Herman, 1992); sequential modality-switching, where different modalities serve different developmental stages (Foo's trajectory through talk therapy, EMDR, IFS, and somatic work) (Foo, 2022); concurrent adjuncts, where a primary therapy is supplemented by somatic, creative, or pharmacological interventions (MDMA-assisted psychotherapy, Brand's web-based psychoeducation alongside ongoing therapy) (Brand, 2019; Mitchell, 2021); and single modality, which tends to appear in deliberately constrained studies or with specific populations (Murphy's PCT, EMDR intensive formats) (Jongh, 2024; D. Murphy, 2009). The neuroscience sources provide the strongest theoretical rationale for multi-modality: Lanius (2011) identifies four disrupted neural systems in CPTSD, each potentially requiring different therapeutic approaches; Samson (2023) document multi-system disruption that implies no single modality can address all affected systems; Ogden (2006) explicitly positions sensorimotor psychotherapy as an augmentation to talk therapy, arguing that neither cognitive nor somatic approaches alone are sufficient (Ogden, 2006).

The most detailed sequential trajectory in the review is Razvi's: talk therapy (intellectual understanding only), then 2.5 years of somatic work (traumatic charge release), then 1.5–2 years of attachment work (relational pattern repair), then MDMA-assisted processing (accessing the dissociative core) — each phase building on what the previous one installed (Heacock, 2019). One Reddit poster describes a similar evolution within a single therapeutic relationship: EFT in year 1, IFS in years 2–4, psychodynamic work in year 5, each modality serving a different developmental phase (u/msk97 & (u/shabaluv, 2024). Foo reframes "breaking up with therapists" as developmental moves — the patient outgrowing a modality rather than failing at it (Foo, 2022). Fisher's TIST

model represents the concurrent integration approach: sensorimotor psychotherapy, IFS, clinical hypnosis, and CBT woven into each session within a structural dissociation framework (Fisher, 2019). The clinical literature treats modality-switching as noise in outcome data; the client data suggest it is signal. Treatment planning for complex developmental trauma should anticipate modality evolution — different approaches serving different phases of the healing process — rather than assuming that the modality chosen at intake will be the modality that completes the work.

Fifth, plan for continuity — and when continuity breaks, manage the transition explicitly. The extraction data reveal a fundamental tension. Layer 1 (academic) strongly favours continuity: secure dependency develops within a specific dyad (Steele, Kohut, Winnicott, Schore), accumulated implicit relational knowing is relationship-specific (BCPSG), and interruptions during deep processing risk destabilisation (Steele, Van der Hart, Porges) (Group, 2010; Kohut, 1984; Porges, 2022; Steele, 2001; Winnicott, 1965). But Layers 4–6 normalise episodic engagement as the empirical reality: Blue Knot endorses treatment "continuously or episodically" (Foundation, unknown); Spring's trajectory includes a 3-year gap between therapy episodes with prior gains fully retained (Spring, 2021); Heacock documents Sherlock's sustained weekly engagement for 15 years interrupted by maternity leaves and a failed transfer (Heacock, 2022). The key clinical distinction is between forced interruption (institutional, financial, or system-driven) and developmental interruption (the patient has consolidated enough to take a break and returns when ready). The former recapitulates relational trauma; the latter may be a feature of healthy recovery.

The one-therapist-vs-several question deepens this tension. The psychoanalytic and relational traditions overwhelmingly argue for a single sustained relationship: whether the mechanism is transmuting internalisation (Kohut), survival of destruction (Winnicott/Benjamin), self-state negotiation (Bromberg), or right-brain attunement (Schore), the process is dyad-specific and cannot be transferred (Bromberg, 1998; Kohut, 1984; Schore, 2021; Winnicott, 1965). The Heacock case provides the sharpest evidence: when Sherlock was transferred to another trainee, a single ill-judged remark destroyed three years of painstakingly built trust (Heacock, 2022). Yet empirical reality shows most patients cycling through multiple therapists — one poster describes 7 to 30+ before finding the right fit (Multiple posters (u/wickeddude123, 2023); Bisson documents Davitt requiring two failed relationships before his match (Bisson, 2024); Foo's decade-spanning arc involves at least two primary therapists (Foo, 2022). Fonagy's mentalization model offers the most important exception: because mentalization develops through being mentalized, the mechanism is not dyad-specific — any attuned therapist can provide it, and MBT demonstrates that team coherence around a shared model can substitute for single-therapist continuity (Bateman, 2008; G. Fonagy P., 2002). The clinical implication is twofold: where a single sustained relationship is achievable, it is likely optimal for the deepest structural work; where it is not (which is most of the time, over multi-year trajectories), transitions should be managed explicitly — with structured handovers,

shared formulations, and attention to the attachment meaning of the change — rather than treated as administrative events.

Sixth, set expectations by factor combination. The framework generates a clinical heuristic. For patients with shallow structural injury seeking symptom reduction who arrive with adequate safety: weeks of intensive treatment may suffice. For patients with deep structural disruption seeking personality integration who must first build safety from scratch: years of sustained relational therapy, with the expectation that gains will continue to develop after formal treatment ends (the sleeper effect). Most patients fall somewhere between these poles. The clinical task is to locate the individual patient on each factor and communicate the expected trajectory honestly.

19.2 Training

Therapist training programmes should prepare clinicians for three realities that the current evidence base supports.

First, long-term work with this population is not a luxury or a failure of efficiency — it is often clinically indicated. Training should include sustained clinical placements (not just brief practicum rotations) that allow trainees to experience multi-year therapeutic relationships. The Heacock (2022) account of a 15-year therapeutic relationship between trainee-turned-clinician Erin Jacklin and her client Sherlock illustrates both the depth of this work and the risk when continuity is disrupted by training transitions (Heacock, 2022).

Second, the borrowed-vs-owned structure distinction (Kohut, 1984) should be taught as a clinical tool. Trainees need to recognise when a patient is functioning well because the therapist's regulatory capacity is available as an external resource (borrowed structure) versus when the patient has internalised regulatory capacity that will be self-sustaining after treatment ends (owned structure). This distinction has direct implications for termination decisions.

Third, training should address the Fisher-Fosha tension explicitly (Fisher, 2019; Fosha, 2019). Rather than training clinicians in a single position on the stabilisation-first versus transformation-from-the-get-go debate, programmes should teach assessment of the individual patient's structural depth and readiness, so that the treatment approach can be matched to the patient rather than dictated by the therapist's training allegiance.

19.3 Policy and insurance

The policy implications are the most uncomfortable, because they expose a structural conflict between the evidence and the systems that fund treatment.

The evidence is clear that symptom reduction is achievable within the session limits typically

imposed by insurance structures (8–20 sessions). For patients seeking and needing only symptom reduction, current funding models may be adequate. But for patients with deep structural disruption seeking structural change — the population at the centre of this review — session limits represent a systematic denial of effective treatment. Shedler and Gnaulati frame this as the "evidence-based therapy myth": the managed-care-driven narrative that misrepresents research findings to justify session limits (&. G. Shedler J., 2020).

The cost data complicates this picture. Koeser et al. (2023) found that long-term psychodynamic psychotherapy (LTPP) costs approximately GBP 33,000 per quality-adjusted life year (QALY) in the TADS trial — above the NICE threshold of GBP 20,000/QALY (Koeser, 2023). Even if long-term therapy works, it may not be cost-effective by standard health-economic criteria. However, the authors note that the time horizon may have been too short to capture the full benefit: the sleeper effect (therapy gains continuing to grow for years after treatment ends) would increase QALYs over a longer evaluation period. Cost-effectiveness analyses that capture only the first 2–3 years after treatment systematically underestimate the benefit of structural interventions.

Three policy directions emerge from the evidence. First, stepped care models such as Links, Ross, and Gunderson's (2015) Good Psychiatric Management (GPM) framework offer a pragmatic approach: begin with general psychiatric management, step up to specialised treatment (MBT, DBT) only for patients who do not improve at defined milestones (6 weeks, 6 months, 12 months, 18 months) (Links, 2015). This matches treatment intensity to demonstrated need. Second, intensive delivery formats — concentrated multi-day or multi-week treatment episodes — merit investment because they achieve large effect sizes ($d = 1.57$) while dramatically reducing dropout (5.51% versus 18–39%) (Sciarrino, 2020). If the safety construct is correct, intensive formats may achieve rapid safety establishment through environmental containment, making them particularly suited to the early phases of treatment. Third, funding structures should recognise the distinction between treatment episodes and treatment duration. An episodic model — periods of intensive engagement separated by self-managed maintenance, extending over years — may be more cost-effective than continuous weekly therapy while matching the episodic trajectory documented in clinical and client narratives (Foundation, unknown; Multiple posters (Lionheart, 2021; Spring, 2021).

19.4 Research

The review identifies four research priorities.

Long-term follow-up with structural measures. The most consequential gap in the evidence base is the absence of long-term follow-up studies (beyond 10 years) in defined complex trauma populations using structural outcome measures (personality organisation, mentalizing capacity, attachment security, dissociative symptom trajectory). The Helsinki study (Lindfors, 2019) comes

closest but used a mixed anxiety/mood disorder population. Studies measuring symptom reduction at 6–12 months systematically overestimate the durability of symptom-focused treatments and underestimate the benefit of structural interventions, as the sleeper effect data demonstrate (Bateman, 2008; R. Fonagy P., 2015; J. Shedler, 2010).

Safety-establishment timing. The safety prerequisite is one of the review's strongest emergent findings, yet the actual duration of safety establishment — the time from first session to the point at which the developmental process can begin — is essentially unstudied. All dose-effect studies measure from first session, conflating pre-safety and post-safety sessions. Studies that measure the onset of felt safety (through alliance measures, autonomic markers, or therapist-reported clinical assessment) and relate it to subsequent treatment response would directly test the framework's central construct.

Structural depth assessment tools. The framework predicts that the structural depth of the injury is a major determinant of treatment duration, but there is no validated assessment instrument that maps a patient's structural depth at intake. The structural dissociation model (Van der Hart, 2006) provides the theoretical vocabulary; translating this into a clinically usable assessment tool would enable treatment matching and more accurate expectation-setting.

Intensive format mechanisms. The finding that intensive formats produce both larger effect sizes and dramatically lower dropout (Sciarrino, 2020) warrants investigation of the mechanism. If intensive formats achieve rapid safety establishment through environmental containment — removing the patient from stressors, immersing them in a therapeutic milieu, providing team-based holding — then the safety construct predicts that intensive formats would be particularly effective for patients whose primary barrier is environmental unsafety rather than deep structural disruption.

19.5 Clients

What would honest, evidence-informed expectation-setting look like for a person entering therapy for complex developmental trauma?

It would begin by naming the endpoint question explicitly: what do you hope to achieve? If the answer is symptom reduction — fewer flashbacks, lower anxiety, the ability to sleep — then concentrated treatment can achieve this in weeks to months, and there is strong evidence for its effectiveness. If the answer is something deeper — understanding why relationships always go wrong, feeling safe in one's own body, developing a stable sense of self — then the timeline is longer, measured in years rather than months, and the process will not look like steady upward progress. It will look like spirals, setbacks, plateaus, and periods where nothing appears to be happening (Spring, 2020a).

It would name the shame directly. The Reddit data reveal that clients internalise brief-therapy norms as personal failure: one poster in their second to third year of therapy worried about being "in therapy too long," and the community overwhelmingly responded that years is normal (Multiple posters (u/Pluto0x0, 2023). The mismatch between expected timelines (months, as implied by insurance structures and popular culture) and lived timelines (years) produces suffering on top of the original condition — a secondary injury of feeling defective for not healing fast enough. Honest expectation-setting is not just clinically useful; it is an intervention against this internalised shame. Naming the timeline early — "this is likely to take years, and that is not because something is wrong with you but because of the nature of the injury" — removes a source of distress that the evidence shows is widespread.

It would be honest about safety. If you are still in contact with people who are actively harmful, if you are in a living situation that keeps your nervous system in chronic alert, if you do not yet have the basic environmental stability to sustain a therapeutic process — then the first task is not trauma processing but safety-building, and this is not a detour but a prerequisite.

It would normalise the trajectory — including the worsening. The spiral-recursive pattern — cycling through phases, returning to earlier work as deeper material emerges — is the dominant trajectory in the complex trauma treatment literature, not a sign of failure (Foundation, unknown; Herman, 1992; Steele, 2005). Feeling worse before feeling better is documented across sources and expected (Multiple posters (u/cheesytacos23, 2021). This is the single most dangerous moment in the therapeutic process: the point where the patient's defences have been disturbed, the distress has intensified, and the temptation to conclude that therapy is making things worse is strongest. One poster described the first month as the hardest period of the entire trajectory — and the point at which they nearly quit (Multiple posters (u/cheesytacos23, 2021). Another described a decompensation that was, in retrospect, the turning point preceding healing (Multiple posters (u/Longjumping_Cry709, 2023). Clients who know in advance that early worsening is a sign of the process working — not evidence of failure — are better equipped to survive it. The "right therapist" search — sometimes requiring multiple attempts (Multiple posters (u/wickeddude123, 2023) — is a known part of the process, not evidence of personal failure.

It would extend the definition of healing beyond personal restoration. Herman's concept of the "survivor mission" — the stage at which recovery moves from personal healing to social contribution and meaning-making — redefines the endpoint as not merely restored but transformed (Herman, 1992). For clients, this reframes the trajectory: the years of therapeutic work are not simply repair but preparation for a contribution that only someone who has lived through developmental trauma can make.

It would offer the most helpful framing this review encountered. Spring's "heading north" metaphor

(Spring, 2020b) redefines recovery as a direction rather than a destination. At any given moment, you may be crawling, stopped, or temporarily heading south. The question is not "have you arrived?" but "is the overall direction northward?" This framing is both more honest than a promise of cure and more hopeful than a prediction of lifelong suffering. It acknowledges that the journey may be long while insisting that every step northward counts.

20 Inaccessible sources and their impact

20.1 Scale of the problem

Of 89 sources included in this review, four remain fully unresolved — identified as directly relevant but not accessed or only partially accessed at the time of writing. An additional 11 sources were initially inaccessible but later obtained through manual provision, and approximately 10 sources are annotated from abstracts, secondary citations, or partial excerpts only (marked `abstract-only` in the source index). The barrier in most cases is paywall access or the need to obtain a physical book.

The four unresolved sources are all in Layer 1 (academic and clinical literature), concentrating the gap in the review's theoretical foundation:

1. **Yeates et al. (2023), "A systematic review and narrative analysis of the evidence for individual psychodynamically informed psychotherapy for dissociative disorders"** (2023). Paywalled. This source sits at the precise intersection of the review's population (dissociative disorders) and a key modality (psychodynamic). It may contain duration data across multiple reviewed studies that would directly strengthen or qualify the review's claims about psychodynamic treatment length for dissociative presentations.

2. **Freixo Ferreira, Guerra, & Vieira-Coelho (2023), "Predictors of psychotherapy dropout in patients with borderline personality disorder: A systematic review"** (2023). Annotated from abstract and Scholar Gateway excerpts; full text behind paywall. The abstract confirms the treatment-engagement paradox (weaker therapeutic alliance and higher hostility predict dropout) but the full text would provide dropout rate numbers that the survivorship bias discussion currently lacks. Without these numbers, the claim that "the people who most need long-term treatment are systematically missing from the evidence" is well-supported qualitatively but poorly quantified.

3. **Herman (1992), *Trauma and Recovery*.** Book not obtained. This is the foundational text establishing the three-phase model (safety, remembrance and mourning, reconnection) on which all subsequent phase-based treatment literature builds. The review currently relies on secondary descriptions of Herman's model — through Steele et al. (2005), Fisher (2021), the Blue Knot Foundation, and multiple client narratives that reference the phase model.

The primary text would provide original quotes, the nuances of Herman's original framework (particularly the duration of Phase 1 for chronic trauma, which she reportedly describes as "months to years"), and any qualifications that secondary citations may have dropped.

4. **Van der Hart, Nijenhuis, & Steele (2006), *The Haunted Self: Structural Dissociation and the Treatment of Chronic Traumatization*** (2006). Book not obtained. The comprehensive theoretical framework for structural dissociation and its treatment. Part III covers treatment in detail. The review relies on this framework extensively — the structural dissociation model is central to the "structural depth" factor — but through secondary sources (Steele et al., 2005; Steele et al., 2001; Griffiths, 2025). The primary text would provide the full theoretical rationale for why complex trauma treatment is necessarily long-term.

A fifth source merits mention: **Farina, Dimaggio, & Mosquera (2025), "Common Elements and Differences Among Treatment Approaches to Complex Post-Traumatic Stress Disorder"** (2025). This was accessible only through Scholar Gateway abstract and excerpts. The "rapid-rare" observation — that quick good outcomes in CPTSD cases are rare and that typical cases are "complex, difficult and time-consuming" — is reconstructed from these excerpts rather than from full-text reading. The synthesis relies on this observation to support the structural depth factor (Factor 2), but the supporting detail and any qualifications in the full text are unavailable.

20.2 Which layers are most affected

The inaccessible sources are concentrated in Layer 1a (trauma-clinical), which is also the review's theoretical backbone. Three of the four unresolved sources are Layer 1a texts; the fourth (Yeates) is also Layer 1a. This means the layer that provides the foundational treatment models — the phase model, the structural dissociation framework, and the clinical rationale for long-term treatment — is the layer most affected by access gaps.

Layers 2 through 6 are comparatively well-covered. All 11 Layer 2 sources (naturalistic studies) are now accessible, most through manual provision. All Layer 3 (counterpoint), Layer 4 (practitioner), Layer 5 (client narrative), and Layer 6 (memoir) sources are fully accessible. The access problem is concentrated in the theoretical literature, not in the empirical or experiential evidence.

This concentration has a specific consequence: the review's empirical findings (endpoint-duration nexus, sleeper effect, safety prerequisite, trajectory shapes) are well-grounded in accessible sources, but the theoretical framework explaining WHY these patterns exist — the phase model, structural dissociation, and the developmental mechanism — relies partly on secondary descriptions of foundational texts.

20.3 Impact on specific claims

The review's central claims and their vulnerability to the inaccessible source gap:

"The phase model is universally endorsed." This claim would be substantially strengthened by primary access to Herman (1992), which originated the model. Currently, the claim rests on secondary endorsements from Steele et al. (2005), Fisher (2021), the Blue Knot Foundation (84% expert consensus), Griffiths (2025), and multiple client narratives. The convergence is strong enough that access to the primary text is unlikely to change the claim — but it would provide the original formulation and any caveats Herman attached.

"Safety establishment takes months to years for chronic trauma." This specific duration claim is attributed to Herman but is known only through secondary citation. If the primary text qualifies this estimate differently — for example, by specifying that Phase 1 duration depends on specific conditions — the review's synthesis would need to incorporate that nuance.

"Structural depth determines treatment duration." This claim relies on the Van der Hart et al. (2006) structural dissociation model, accessed only through secondary sources. The primary text's treatment section (Part III) likely contains specific duration estimates or clinical guidelines that would either support or complicate the review's Factor 2. The Farina (2025) "rapid-rare" observation, which supports this claim from the counterpoint direction, is similarly incomplete.

"The treatment-engagement paradox is devastating for duration claims." The dropout discussion cites Freixo Ferreira et al. (2023) for the paradox that the traits defining the population (hostility, alliance problems) predict dropout. Full-text access would provide the systematic review's quantitative findings — pooled dropout rates, effect sizes for predictors — that would give this claim empirical specificity rather than remaining at the level of conceptual argument.

20.4 Could the missing sources change the conclusions?

The direction of impact is assessable even without reading the sources. Herman (1992) and Van der Hart et al. (2006) are foundational texts in the long-term treatment tradition. Their full texts would almost certainly strengthen rather than challenge the review's thesis. The risk is not that they would undermine the argument but that secondary citation has oversimplified their positions — dropping qualifications, overstating certainty, or losing nuance about when long-term treatment is and is not indicated.

Yeates et al. (2023) and Freixo Ferreira et al. (2023) are systematic reviews that could go either way. Yeates might find that psychodynamic treatment for dissociative disorders shows weaker evidence than the review assumes, or that duration data across reviewed studies suggests shorter or longer timelines than predicted. Freixo Ferreira might quantify dropout rates in ways that either sharpen

or soften the survivorship bias argument.

Farina et al. (2025) is the most consequential partial access: the "rapid-rare" observation is central to the structural depth factor, and any qualifications in the full text could modify how the review handles the counterpoint position.

The honest assessment is that the missing sources are unlikely to overturn the review's central findings — the endpoint-duration nexus, the safety prerequisite, and the developmental mechanism are each supported by multiple fully accessible sources across multiple layers. But they could modify the precision and confidence of specific claims, particularly regarding the phase model's original formulation, the structural depth framework's clinical specificity, and the quantitative evidence for the treatment-engagement paradox. Obtaining Herman (1992), Van der Hart et al. (2006), and the Farina (2025) full text should be the highest-priority next steps for strengthening the review's evidence base.

Abbass, K., A. A. (2014). Short-term psychodynamic psychotherapies for common mental disorders. *Cochrane Database of Systematic Reviews*. https://doi.org/10.1002/14651858.CD004687.pub4

Alpern, P. (2025). *The Truth About Complex Trauma*. https://traumajournal.substack.com/p/the-truth-about-complex-trauma

America (summary of Krakau et al.), M. in. (2024). *Psychoanalytic Therapy Beats CBT for Trauma-Linked Depression*. https://www.madinamerica.com/2024/09/psychoanalytic-therapy-beats-cbt-for-trauma-linked-depression/

Associates LLC, B. &. (2023). *Injured, Not Broken: Why It's So Hard to Know You Have CPTSD*. https://www.madinamerica.com/2023/11/injured-not-broken-why-its-so-hard-to-know-you-have-cptsd/

Auer, L. (unknown). *Traumalescence: A Trauma Therapist's New Word for the Pattern No One's Talking About*. https://mytherapist.substack.com/p/traumalescence-a-trauma-therapists

Bateman, &. F., A. (2008). 8-year follow-up of patients treated for borderline personality disorder: Mentalization-based treatment versus treatment as usual. *American Journal of Psychiatry*, *165*(5), 631–638. https://doi.org/10.1176/appi.ajp.2007.07040636

Bisson, J. (2024). *Recognising Complex PTSD*. https://shows.acast.com/piece-of-mind/episodes/episode-14-recognising-complex-ptsd

Brand, S., B. L. (2019). An Online Educational Program for Individuals With Dissociative Disorders and Their Clinicians: 1-Year and 2-Year Follow-Up. *Journal of Traumatic Stress*, *32*(1), 156–166. https://doi.org/10.1002/jts.22370

Bromberg, P. M. (1998). *Standing in the Spaces: Essays on Clinical Process, Trauma, and Dissociation*. Analytic Press.

De Jongh, &. H., A. (2023). *Trauma-focused treatment of a client with Complex PTSD and comorbid pathology using EMDR therapy*. https://doi.org/10.1002/jclp.23521

Eger, E. E. (2017). *The Choice: Embrace the Possible*. Scribner. https://www.goodreads.com/book/show/30753738-the-choice

Farina, D., B. (2025). *Common Elements and Differences Among Treatment Approaches to Complex Post-Traumatic Stress Disorder: A Commentary on Five Case Studies*. https://doi.org/10.1002/jclp.23773

Feder, B., A. (2025). *Combining Ketamine Infusions and Written Exposure Therapy for Chronic PTSD: An Open-Label Trial*. https://doi.org/10.4088/JCP.24m15622

Ferentz, L. (unknown). Transcending Trauma. *Psychotherapy Networker*. https://www.psychotherapynetworker.org/article/transcending-trauma/

Fisher, J. (2019). *Janina Fisher on Therapy on the Cutting Edge: Trauma-Informed Stabilisation Treatment*. https://www.therapyonthecuttingedge.com/janinafishertranscript.html

Fisher, J. (2021). *Transforming the Living Legacy of Trauma: A Workbook for Survivors and Therapists*. PESI Publishing. https://www.amazon.com/dp/B08S7NKVJ5

Fonagy, G., P. (2002). *Affect Regulation, Mentalization, and the Development of the Self*. Other Press.

Fonagy, R., P. (2015). Pragmatic randomized controlled trial of long-term psychoanalytic psychotherapy for treatment-resistant depression: the Tavistock Adult Depression Study (TADS). *World Psychiatry*, *14*(3), 312–321. https://doi.org/10.1002/wps.20267

Foo, S. (2022). *What My Bones Know: A Memoir of Healing from Complex Trauma*. Ballantine Books. https://www.goodreads.com/book/show/58214328-what-my-bones-know

Fosha, D. (2019). *AEDP Interview: Healing from the Get-Go*. https://www.psychotherapy.net/interview/AEDP-Diana-Fosha

Foundation, B. K. (unknown). *Trauma Specific Practice when Working with Complex Trauma*. https://professionals.blueknot.org.au/resources/trauma-specific-practice-when-working-with-complex-trauma/

Freixo Ferreira, G., L. (2023). Predictors of psychotherapy dropout in patients with borderline personality disorder: A systematic review. *Clinical Psychology & Psychotherapy*, *30*(6), 1324–1337. https://doi.org/10.1002/cpp.2888

Gorman, B., I. (2020). *Posttraumatic Growth After MDMA-Assisted Psychotherapy for Posttraumatic Stress Disorder*. https://doi.org/10.1002/jts.22479

Griffiths, et al., S. (2025). Effectiveness of phase-oriented treatment for trauma-related dissociative disorders: a systematic review. *European Journal of Psychotraumatology*. https://doi.org/10.1080/20008066.2025.2545734

Grossmark, R. (unknown). Psychoanalytic Companioning. *IARPP Colloquium*. https://iarpp.net/colloquium/psychoanalytic-companioning/

Group, B. C. P. S. (2010). *Change in Psychotherapy: A Unifying Paradigm*. Norton.

Hambrick, B., E. P. (2019). Timing of early-life stress and the development of brain-related capacities.

Frontiers in Behavioral Neuroscience, *13*, 183. https://doi.org/10.3389/fnbeh.2019.00183

Hauser, H. J. (2022). 'Pulling the Bandage Off': Using Margaret Warner's 'Fragile Process' in the Psychodynamic Approach. *British Journal of Psychotherapy*, *38*(4), 693–708. https://doi.org/10.1111/bjp.12745

Heacock, C. (2019). *Healing Trauma with Psychedelics with Saj Razvi: Part 1.*

Heacock, C. (2022). *Psychotherapy Twice Told.*

Herman, J. L. (1992). *Trauma and Recovery.*

Howard, K., K. I. (1986). The dose-effect relationship in psychotherapy. *American Psychologist*, *41*(2), 159–164.

Jongh, R. de, A. (2024). *State of the science: Eye movement desensitization and reprocessing (EMDR) therapy.* https://doi.org/10.1002/jts.23012

Joseph, M., S. (2012). An affective-cognitive processing model of post-traumatic growth. *Clinical Psychology & Psychotherapy*, *19*(4), 316–325. https://doi.org/10.1002/cpp.1798

Knekt, G., P. (2021). Poor suitability for psychotherapy – a risk factor for treatment non-attendance? *Journal of Affective Disorders*, *295*, 1432–1439. https://doi.org/10.1016/j.jad.2021.09.020

Koeser, R., L. (2023). Cost-effectiveness of long-term psychoanalytic psychotherapy for treatment-resistant depression: RCT evidence from the Tavistock Adult Depression Study (TADS). *Journal of Affective Disorders*, *335*, 313–321. https://doi.org/10.1016/j.jad.2023.04.109

Kohut, H. (1984). *How Does Analysis Cure?*

Lanius, B., R. A. (2011). How understanding the neurobiology of complex post-traumatic stress disorder can inform clinical practice: A social cognitive and affective neuroscience approach. *Acta Psychiatrica Scandinavica*, *124*(5), 331–348. https://doi.org/10.1111/j.1600-0447.2011.01755.x

Leichsenring, A., F. (2013). The emerging evidence for long-term psychodynamic therapy. *Psychodynamic Psychiatry*, *41*(3), 361–384. https://doi.org/10.1521/pdps.2013.41.3.361

Levine, P. A. (2024). *An Autobiography of Trauma: A Healing Journey.* Park Street Press. https://www.goodreads.com/book/show/196786498-an-autobiography-of-trauma

Lilliengren, J., P. (2017). *Intensive Short-Term Dynamic Psychotherapy for generalized anxiety disorder: A pilot effectiveness and process-outcome study.* https://doi.org/10.1002/cpp.2101

Lilliengren, M., P. (2025). *The Efficacy of Experiential Dynamic Therapies: A 10-Year Systematic Review and Meta-Analysis Update.* https://doi.org/10.1002/cpp.70086

Lindfors, K., O. (2019). Effectiveness of psychoanalysis and long-term psychodynamic psychotherapy on personality and social functioning 10 years after start of treatment. *Psychiatry Research*, *272*, 774–783. https://doi.org/10.1016/j.psychres.2018.12.082

Links, R., P. S. (2015). Promoting Good Psychiatric Management for Patients With Borderline Personality Disorder. *Journal of Clinical Psychology*, *71*(8), 753–763. https://doi.org/10.1002/jclp.22203

Mitchell, B., J. M. (2021). *MDMA-assisted therapy for severe PTSD: a randomized, double-blind,*

placebo-controlled phase 3 study. https://doi.org/10.1038/s41591-021-01336-3

Multiple posters (Crow, P. B., One step at a time. (2019). *Curious: How Long Have You Been in Therapy?* https://www.myptsd.com/threads/curious-how-long-have-you-been-in-therapy.68225/

Multiple posters (Lionheart, G., Abstract. (2021). *How Long Were You in Therapy?* https://www.myptsd.com/threads/how-long-were-you-in-therapy.87861/

Multiple posters (Skywatcher, grit, somerandomguy. (2020). *How Often Do You See Your Therapist?* https://www.myptsd.com/threads/how-often-do-you-see-your-therapist.95213/

Multiple posters (sugnim, T. A., Friday. (2015). *Unable to Afford Therapy.* https://www.myptsd.com/threads/unable-to-afford-therapy.57989/

Multiple posters (u/almost-crazy, u/Icy.-B., u/Dapper_Banana6323. (2025). *How many of you reached a full cure? How long did it take for you to reach it and what helped step by step?* https://www.reddit.com/r/CPTSD/comments/full_cure_how_long/

Multiple posters (u/cheesytacos23, et al.). (2021). *How long after therapy did you start to feel better/progress? I'm loosing it.* https://www.reddit.com/r/CPTSD/comments/how_long_after_therapy_feel/

Multiple posters (u/Longjumping_Cry709, u/bimbo_at_law., u/ZzzzoZzzzo. (2023). *For anyone who is fully healed, how long did it take you?* https://www.reddit.com/r/CPTSD/comments/fully_healed_how_long/

Multiple posters (u/Ok-Specialist1153, u/woahtheremate_., u/Longjumping_Cry709. (2024). *What does it feel like after you've significantly healed?* https://www.reddit.com/r/CPTSD/comments/what_does_it_feel_like_after/

Multiple posters (u/Pluto0x0, u/pottheplant., u/BS-MakesMeSneeze. (2023). *How long were/are you in therapy?* https://www.reddit.com/r/CPTSD/comments/how_long_wereare_you_in_therapy/

Multiple posters (u/WeeLittleSloth, u/MarsNer., u/Simple_Song8962. (2021). *You can't properly heal your trauma while you're still being actively traumatized.* https://www.reddit.com/r/CPTSD/comments/cant_heal_while_surviving/

Multiple posters (u/wickeddude123, u/ElishaAlison., u/Thr0wnF4rAw4y. (2023). *It takes on average 7 therapists before people find the one.* https://www.reddit.com/r/CPTSD/comments/7_therapists_average/

Murphy, D. (2009). Client-centred therapy for severe childhood abuse: A case study. *Counselling and Psychotherapy Research*, *9*(1), 3–10. https://doi.org/10.1080/14733140802655992

Murphy, E., D. (2019). Identifying and developing therapeutic principles for trauma-focused work in person-centred and emotion-focused therapies. *Counselling and Psychotherapy Research*, *19*(4), 497–507. https://doi.org/10.1002/capr.12235

Ogden, K. P., Pat; Minton. (2006). *Trauma and the Body: A Sensorimotor Approach to Psychotherapy.* https://www.amazon.com/dp/B001ULOPUO

Paris, J. (2020). Access to psychotherapy for patients with personality disorders. *Personality and Mental Health*, *14*(3), 246–253. https://doi.org/10.1002/pmh.1483

Porges, S. W. (2022). Polyvagal Theory: A science of safety. *Frontiers in Integrative Neuroscience*, *16*, 871227. https://doi.org/10.3389/fnint.2022.871227

Samson, N., J. A. (2023). Practitioner Review: Neurobiological consequences of childhood maltreatment — clinical and therapeutic implications. *Journal of Child Psychology and Psychiatry*, *65*(3), 369–380. https://doi.org/10.1111/jcpp.13883

Schore, A. N. (2000). Attachment and the regulation of the right brain. *Attachment & Human Development*, *2*(1), 23–47. https://doi.org/10.1080/146167300361309

Schore, A. N. (2021). The interpersonal neurobiology of intersubjectivity. *Frontiers in Psychology*, *12*, 648616. https://doi.org/10.3389/fpsyg.2021.648616

Sciarrino, W., N. A. (2020). *A Systematic Review of Intensive Empirically Supported Treatments for Posttraumatic Stress Disorder*. https://doi.org/10.1002/jts.22556

Shedler, &. G., J. (2020). The Tyranny of Time: How Long Does Effective Therapy Really Take? *Psychotherapy Networker*. https://jonathanshedler.com/wp-content/uploads/2022/01/Shedler-Gnaulati-2020-Tyranny-of-Time.pdf

Shedler, J. (2010). The efficacy of psychodynamic psychotherapy. *American Psychologist*, *65*(2), 98–109. https://doi.org/10.1037/a0018378

Sohal, &. M., A. (2022). A longitudinal analysis of person-centred therapy with suicidal clients. *Counselling and Psychotherapy Research*, *23*(1), 20–30. https://doi.org/10.1002/capr.12588

Spring, C. (2020a). *Recovering from Developmental Trauma*. https://www.carolynspring.com/podcast/recovering-from-developmental-trauma/

Spring, C. (2020b). *What Does Recovery from Trauma Look Like?* https://www.carolynspring.com/podcast/what-does-recovery-from-trauma-look-like/

Spring, C. (2021). *Falling Down, Getting Back Up Again: My Journey Over the Last Year*. https://www.carolynspring.com/podcast/falling-down-podcast/

Steele, van der H., K. (2001). Dependency in the Treatment of Complex Posttraumatic Stress Disorder and Dissociative Disorders. *Journal of Trauma & Dissociation*, *2*(4), 79–116. https://doi.org/10.1300/J229v02n04_05

Steele, van der H., K. (2005). Phase-oriented treatment of structural dissociation in complex traumatization: overcoming trauma-related phobias. *Journal of Trauma & Dissociation*, *6*(3), 11–53. https://doi.org/10.1300/J229v06n03_02

Tilson, D. F. (2025). *Your Body Is Not Keeping The Score*. https://otherinterests.substack.com/p/your-body-is-not-keeping-the-score

u/Mountain-Science4526, & (u/xechasate, et al.). commenters, u/debzies97. (2024). *My Therapist Who I've Seen For 10 Years. Over 900 Appointments Died Of A Heart Attack Yesterday*. https://www.reddit.com/r/CPTSD/comments/10_years_900_appointments/

u/msk97, & (u/shabaluv, u/the_dawn. commenters, u/AmateurHumanPerson. (2024). *I have officially recovered from CPTSD!!* https://www.reddit.com/r/CPTSD_NSCommunity/comments/recovered_cptsd/

Van der Hart, N., O. (2006). *The Haunted Self: Structural Dissociation and the Treatment of Chronic Traumatization*. Norton.

Walker, P. (2013). *Complex PTSD: From Surviving to Thriving: A Guide and Map for Recovering from Childhood Trauma*. Azure Coyote. https://www.goodreads.com/book/show/20556323-complex-ptsd

Winnicott, D. W. (1965). *The Maturational Processes and the Facilitating Environment*. Hogarth Press.

Woodward Thomas, K. (unknown). Evolving Beyond Our Trauma Stories. *Psychotherapy Networker*. https://www.psychotherapynetworker.org/article/evolving-beyond-our-trauma-stories/

Wylie, M. S. (unknown). The Long Shadow of Trauma. *Psychotherapy Networker*. https://www.psychotherapynetworker.org/article/long-shadow-trauma/

Yeates, et al., G. (2023). A systematic review and narrative analysis of the evidence for individual psychodynamically informed psychotherapy for dissociative disorders. *Journal of Trauma & Dissociation*. https://doi.org/10.1080/15299732.2023.2293802

Zimmermann, L.-S., J. (2014). Is It All about the Higher Dose? Why Psychoanalytic Therapy Is an Effective Treatment for Major Depression. *Clinical Psychology & Psychotherapy*, *22*(6), 469–487. https://doi.org/10.1002/cpp.1917